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Introduction

This programme aims to support you to

- Understand what heart failure is and how to get the most out of your treatments
- Learn what **you** can do to help manage the condition
- Build your ability to manage your condition,
- Manage any stress or anxiety that you may have
- Boost the power of your heart and your energy levels by gradually building up your fitness.
- Communicate well with your health care team
- Identify other sources of support (such as your family, friends, partner, support groups, social services) to help you to manage the condition.

This programme has been proven to work in clinical trials.

Family and Friends

If you have anyone who is close to you, they will have their own questions about your condition and may want to know what they can do to support you. Your family member or friend may be interested to read this manual as well as the separate Friends and Family resource we have prepared for them. Talk to your close family and friends about how you can work with the facilitator to manage heart failure together.

We hope that by actively participating in this programme you will achieve a good quality of life, as well as potentially slowing down the progression of your condition.

You are not alone! Your facilitator, this manual and the rest of your health care team are here to help you to manage your situation and achieve the best possible control of your heart failure.

How to use this manual

Firstly, don't worry – you don't need to read this all at once from cover to cover! You should discuss with your facilitator (see next page) which topics to work on first and only start on new topics when you are ready. The process should be spread over several months.

Part 1: Understanding Heart Failure

- This section gives you plain facts about heart failure. You should read this section first and you may want to come back to it several times.
- If you live with someone, it can be helpful for them to read this too.

Part 2: Managing Your Condition

- This part of the manual has several sections dealing with topics that are important for most people with heart failure. You should agree with your facilitator which topics to work on first. This might include some topics from this section and some from Part 3.

Tracking Your Progress

- The separate Progress Tracker booklet provides some tools to help you to keep track of your progress. It has been designed to help you get the most from your health care and your 'self care'. It will help you to learn what things improve your symptoms and to know when to seek help from your health care service.

Part 3: Further Information

- This section includes further information about topics that are important for some people, but not everyone, so you may want to dip into these or leave them depending on your own situation.

Not just a book

This manual provides information, advice and helpful tools designed to help you to gradually build up your skills to live with and manage heart failure. It is designed for you to use in the way that best suits your individual needs.

The manual is designed to be given to you by a health care worker, who has been specifically trained to work with you to get the most out of the manual. We call this person **your facilitator**. Your facilitator will see you or talk to you on the phone several times to

1. Assess your situation to find out what treatments you are using, what concerns you might have, what information you need, and what skills for living with heart failure you want to develop.
2. Agree a plan of action – you don't have to do everything all at once – your facilitator

will help you to work out a sensible order in which to do things. You will progress gradually over two or three months.

3. Check your progress and work with you to find ways to overcome any problems that you might encounter along the way.
4. Build your knowledge and skills further. Once you have got the hang of one aspect of managing heart failure, you may want to move on to new areas.
5. Work with any friends or family who might be able to help or provide support.

Important messages:

If you know someone else who has heart failure, or another heart problem, please do not show them this manual, or the audio or videos. People with heart problems can have very different situations and what suits one person might not suit all. Instead of helping them you may harm them.

None of the advice in this manual is intended to replace any advice that you are given by your health care team. If there seems to be a conflict of advice, please discuss this with your facilitator or your GP.

Understanding Heart Failure

Heart failure is a common condition. In fact, as people are living longer, more and more people are being diagnosed with heart failure.

This means that treating, monitoring and supporting those living with the condition has become a high priority for the National Health Service. People are being diagnosed earlier and are receiving more and more specialist care from health professionals both in hospital and at home. New research into how heart failure can be better treated is taking place all the time – new medications and medical devices are being developed. All of these things are helping people with heart failure to reduce their symptoms, slow down the progression of the condition and live happy and full lives.

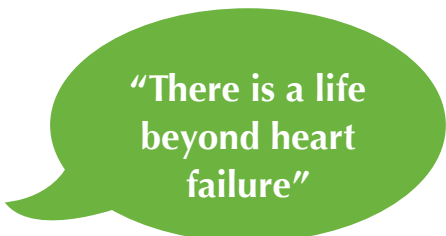
This section provides information to help you understand heart failure and how best to manage it. This includes information about...

- what heart failure is and what causes it
- the signs and symptoms of heart failure
- how heart failure is treated
- what you can do about it

It is important to remember that

- people with heart failure can live happy and fulfilling lives
- understanding your condition can help you take the right action to manage it
- your symptoms may improve with correct treatment and good 'self care'
- you are a key player. Good 'self care' can help you feel more in control of your condition
- family, friends and your health care team can help to support you – you are not alone!

Let's begin by looking at how the heart works ...



**"There is a life
beyond heart
failure"**

How does the heart normally work?

The heart is a large muscle that is responsible for squeezing (pumping) blood around the body. Blood carries oxygen and nutrients to the whole of the body so that everything can work properly. At the same time, blood also carries waste products to organs such as the liver and kidneys so that the waste can be removed. Like any muscle in the body it works best when it is looked after. This includes: staying physically active, eating a healthy diet, taking time out to relax, avoiding smoking and not drinking too much alcohol.

The heart is divided into two halves. The right half receives blood from the body which is low in oxygen (blue side) and pushes it into the lungs to pick up oxygen. The left side of the heart receives blood from the lungs which has had oxygen added (red side) and pushes this around the body. The blood flows to and from the heart through large blood vessels. These blood vessels get smaller and smaller as they pass around the body. The blood vessels feed nutrients and oxygen to your brain, your muscles and all your organs.

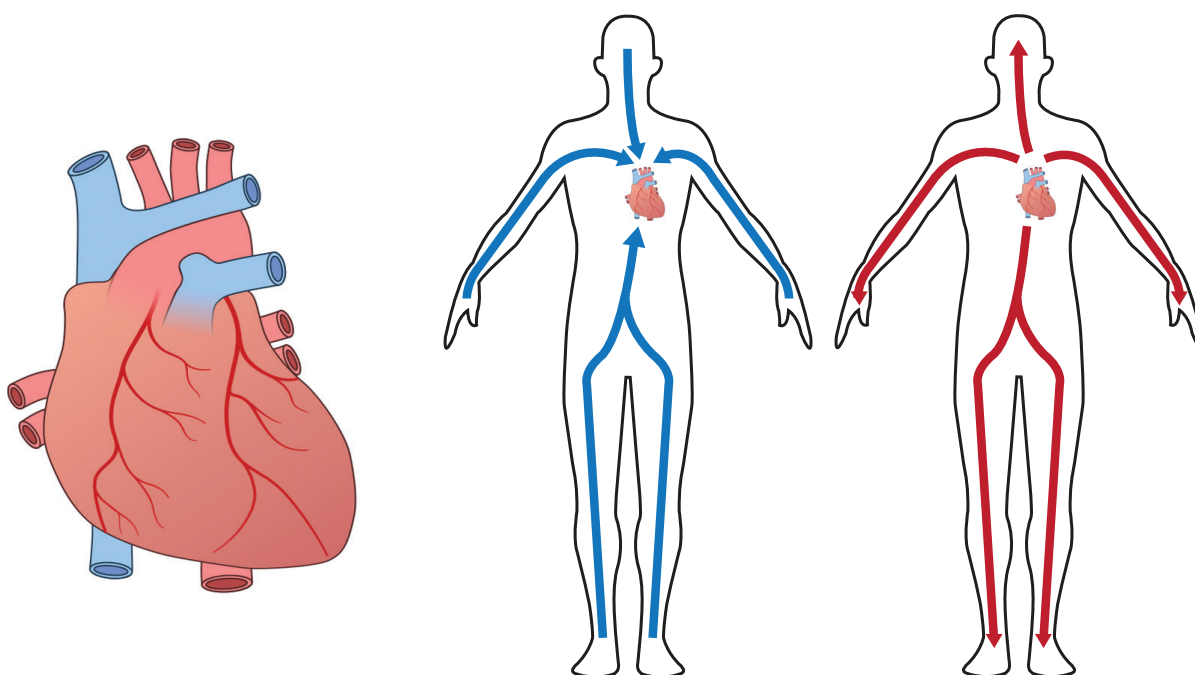


Figure 1: The heart and blood flow

Both the right side and the left side of the heart are also divided in two, making two chambers on each side – that's four chambers in total. The upper chambers are known as 'atria' and the lower chambers are called 'ventricles'. The chambers are kept separate by valves, a bit like doors which separate rooms. These act as 'one way' systems and prevent the blood from flowing backwards. In fact the 'lub-dub' sound that your doctor or nurse hears when they listen to your chest is caused by the heart valves closing.

In order to pump the blood forward, an electrical signal is sent to the heart muscle (Figure 3). This causes the muscle which surrounds each chamber to tighten, forcing the blood from the upper chambers to the lower chambers and from the lower chambers to the lungs and around the body. This is known as a heart beat. At rest, the adult heart normally beats around 60 to 100 times every minute.

When you feel your pulse, this is the beating of your heart. The heart pumps then relaxes, allowing time for it to fill with blood.

Although the heart continues to pump day in day out, it never pumps out all of the blood that fills each chamber.

The normal heart only pushes out 55-70% of the blood it contains.

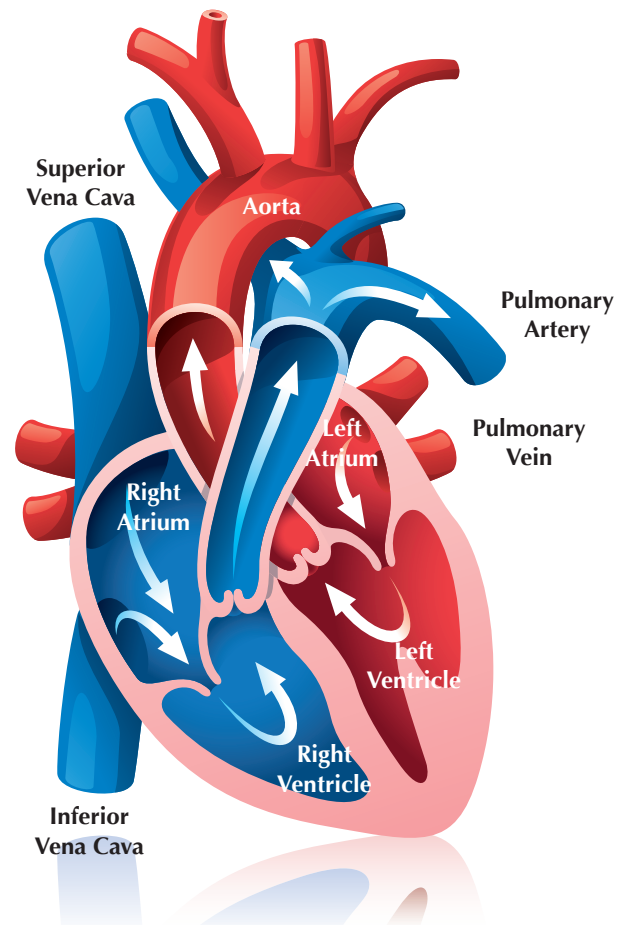


Figure 2: Chambers of the heart and heart valves

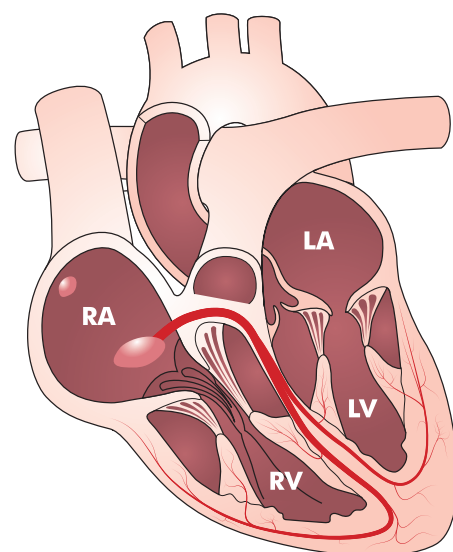


Figure 3: Electrical system of the heart. Electrical pulses tell the muscles when to contract.

Your heart and blood vessels can be thought of like a central heating system. The heart is like the boiler which pumps the warm water (blood) around, the blood vessels are like the pipes that feed all the radiators (flesh and

organs of the body), and the electricity supply ensures the whole system works. If there is a problem with the boiler, the pipes or the electricity supply, it can affect the whole system.

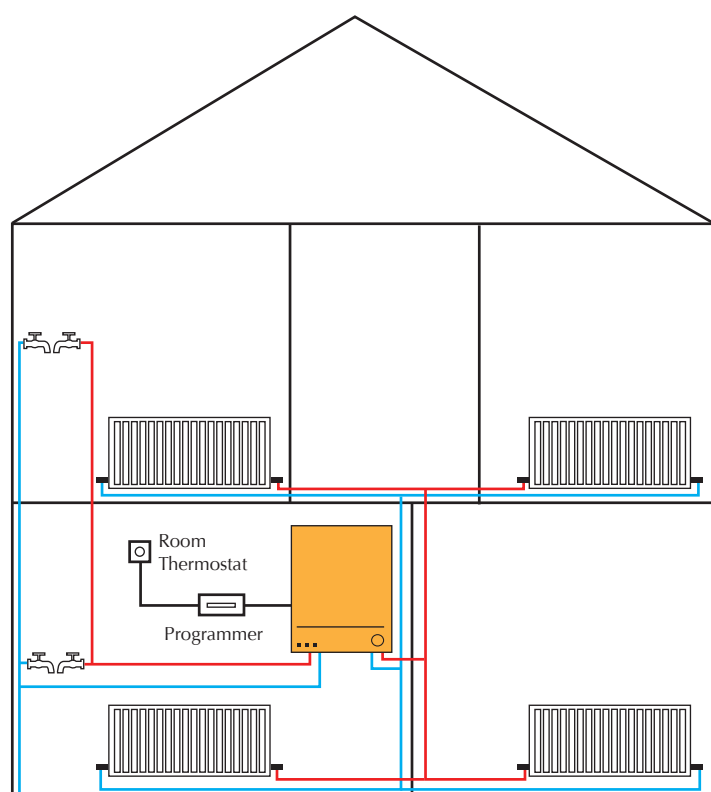
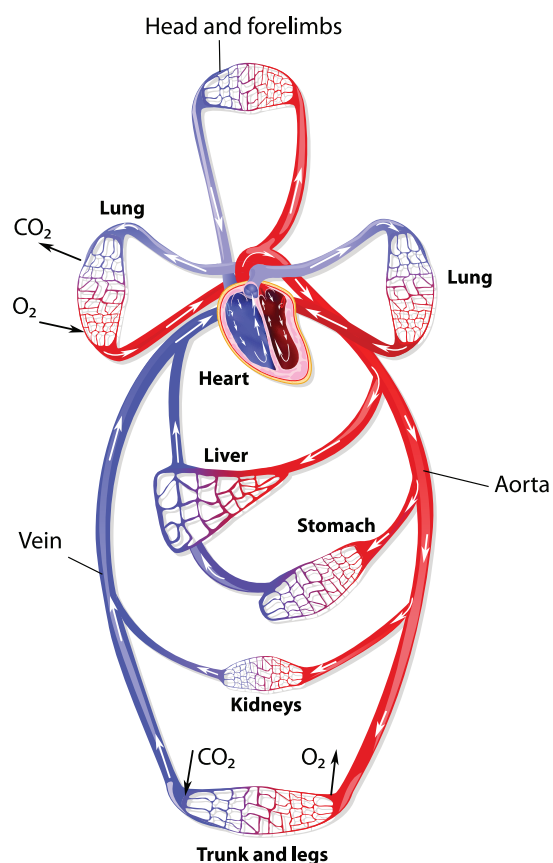


Figure 4: Pumping systems



What is heart failure?

“There is hope”

The term ‘heart failure’ can seem frightening when you first hear it. However, it doesn’t mean your heart has failed and will stop beating. It simply means that something has caused the heart to become less effective at pumping blood. Remember that the normal heart pumps out or ‘ejects’ between 55-70% of the blood it contains with each heartbeat. This ejection fraction is a measure of how well your heart is pumping. If your heart pumps out (ejects) less blood with each heartbeat, this is called ‘heart failure with reduced ejection Fraction’ or HFrEF.

In some cases you can have a normal ejection fraction, but still have heart failure. This is called ‘heart failure with preserved ejection fraction’ or HFpEF. In this type of heart failure, the heart does not relax fully after each heart beat and so it can’t fill back up with as much blood.

In both cases, the heart is not pumping blood as efficiently as it should. Ask your GP or heart failure specialist nurse what type of heart failure you have and what your ejection fraction is.

When the heart is not pumping enough blood, the body reacts in several ways:

- messages are sent to make the heart beat faster and squeeze harder – increasing the work it has to do
- because of this, the heart muscle becomes thicker and bigger
- the body’s small blood vessels become narrower to force more blood to the heart

(and this increases your blood pressure)

- the kidneys reduce the amount of work they do to remove water and salt from the blood, so water and salt can build up in parts of the body.

Unfortunately over time these changes can place more pressure on the heart and reduce its ability to pump even further.

The reduced blood supply to your muscles, kidneys and organs can result in the following signs and symptoms:

- frequent tiredness or fatigue
- swelling (due to a build up of fluid) in the feet, ankles, lower back or abdomen (around the waist)
- breathlessness, coughing or wheezing – due to fluid building up in the lungs
- sudden changes in body weight – getting heavier
- feeling light-headed or confused.

We will take a closer look at the signs and symptoms caused by heart failure in more detail a little later.

Many people develop heart failure gradually over several years. Others develop it suddenly following a heart attack or viral illness. Either way, by taking part in this programme you will learn how you can take action to make the work your heart has to do easier. This will help you to manage your condition and keep the signs and symptoms you have to a minimum.

What causes heart failure?

Heart failure usually happens when an underlying condition has damaged the heart muscle. The two main causes are; having a **heart attack**, and having **high blood pressure** over a long period of time.

Other possible causes include:

- problems with the heart valves
- disease affecting the heart muscle (cardiomyopathy)
- a viral infection which damages the heart muscle
- drinking too much alcohol
- taking recreational or other drugs
- an irregular, uncontrolled heart beat (arrhythmia)
- problems with the heart that you are born with (congenital disorders)
- long term chest (respiratory) conditions
- a shortage of red blood cells (anaemia)
- thyroid disease.

It can be useful to know which of these conditions may have contributed to your heart failure, as this can have an impact on the treatments you have and the way you manage the condition.

Sometimes the cause of heart failure may be unclear and your health care team may not know exactly why you have developed it. However, whatever the cause, the important thing is to focus on what you can do about it. By working through this manual you will learn what actions you can take to minimise your symptoms, reduce any stress or anxiety caused by the condition and improve your quality of life.

If you are finding it hard to come to terms with the diagnosis it is worth speaking to someone about this. Often sharing your thoughts and feelings can help reduce the distress that you are experiencing. There is also a great deal of information in Part 2 of this manual which can help you to cope with the stress and anxiety that living with heart failure can cause.

ACTION

Ask your heart failure specialist or GP about the cause of your heart failure and note down any specific advice you have been given.

The signs and symptoms of heart failure

Earlier in Part 1, we mentioned the common signs and symptoms which are frequently associated with heart failure. These include:

- frequent tiredness or fatigue
- swelling of the feet, ankles, lower back or abdomen
- breathlessness, coughing or wheezing
- sudden changes in body weight – getting heavier very quickly.

The common signs and symptoms associated with heart failure are due to the heart's reduced ability to pump blood around the body. When the heart doesn't work as well as it should, the muscles and organs (such as your kidneys, liver and brain) do not always get enough oxygen and nutrients to carry out their jobs effectively.

Other symptoms may include:

- dizziness or fainting
- a change in your heart beat – either faster, slower, or missing beats
- chest pain
- changes in your appetite or feeling full
- gradual weight loss or weight gain due to a build up of fluids
- difficulty in concentrating or feeling forgetful

- sexual difficulties
- poor sleep

Signs and symptoms vary widely from person to person. People with less severe heart failure tend to have fewer signs or symptoms, while those whose condition is more advanced tend to experience more. However, even here there are differences between people – some people may have only mild symptoms even though their heart has a very low ejection fraction. Others may have a higher ejection fraction but with more severe symptoms.

For many people, the symptoms may vary quite a bit from day to day. This means learning to adapt to having 'good days' and 'bad days'.

We will now take a look at the signs and symptoms in more detail.

REMEMBER

Having a heart which does not work at its maximum efficiency does not necessarily mean that your quality of life will be poor or that your symptoms will stop you from doing all of the things that you enjoy. Many people who stick to their treatment plan and manage the condition well continue to enjoy life.

“There are tablets that can help you, there are things you can do - you can still have a good life”

Tiredness (fatigue)

Feeling tired is a very common symptom experienced by people with heart failure. Some people with heart failure have described their tiredness as feeling like they are carrying a backpack full of rocks around with them all day, or as if they have just completed a marathon even though they have only walked up one flight of stairs. Even concentrating may be difficult and your memory may not be as quick as it used to be.

The sense of exhaustion is caused by your muscles and organs not having enough of the oxygen and nutrients which are carried in the blood. When you are active your muscles need more blood, so walking up a hill or even simple things like hanging up the washing can cause tiredness.

The good news is that this situation can be improved. Firstly, your medications help to manage your symptoms, so getting them right is an important first step. There is also a lot of scientific evidence that, if you have heart failure, building up your fitness can make your heart and muscles work more efficiently. So if you increase your fitness, you will have more energy and feel less tired. Of course, it is harder to exercise if you are tired to start with, but if you build up to it gradually, you will find that the more your fitness builds, the better you feel. In Part 2 of this manual, you

will find a choice of exercise and walking programmes specially designed for people with heart failure. These aim to build up your fitness gradually, through short manageable sessions of exercise.

In part 2, we will look at how to reduce tiredness by getting your medications right, building up your fitness and by pacing your activities.

Fluid retention (congestion) and swelling

Imagine you are in a car heading to the supermarket on a Saturday morning. The car park can hold 200 cars. As you get nearer to the shop the cars in front start to slow down. Eventually you enter the car park. You've never seen it as busy as this before. You are stuck behind at least 20 cars. But you are here now so you might as well keep going. You notice behind you that cars are turning and heading back in the direction they came from. Other cars have started parking on the grassy embankment by the roadside. When traffic gets jammed up like this it is known as 'congestion'.

You may have heard the term 'congestion' used by your doctor or nurse when you have experienced symptoms such as shortness of breath or swelling of your ankles. What is happening is very similar to the busy car park. Your heart continually has to pump blood out. If the heart can't keep up with the

amount of blood coming in, it may struggle with the demands placed on it. In this situation the blood flow becomes sluggish around the body. The kidneys do not receive the normal blood flow. The kidneys normally remove excess water from the blood, but there are two problems – the blood is not flowing well and the kidneys are not working at their best. So, the fluid moves from inside the blood vessels into the tissues around the lungs, feet/ankles, abdomen and lower back. This build-up of fluid is known as congestion. Just as the cars moved from the road on to the grassy embankment to find space, so the fluid within the blood vessels moves into the surrounding tissues to relieve the pressure which is building up in the system.

When fluids build up you may find that your feet and ankles swell, especially if you are standing or sitting for long periods during the day. This is because gravity makes the fluid build up in the lower parts of the legs. This is sometimes called ‘oedema’ or just ‘swelling’. Other areas of the body can also be affected by swelling, such as your lower back or abdomen (around the waist). If fluid builds up around your abdomen it can place pressure on your stomach, making you feel uncomfortable and unable to eat the same amount of food that you are used to. If fluid builds up around your lungs it can cause difficulty in breathing.

When you lie down, the fluid moves back to the centre of the body. This is why you may notice that your feet are swollen at night and back to their normal size by the morning. It

is also why you may become more breathless when lying down.

Take extra care of your skin: When fluid gathers, especially around the feet and ankles, it causes the skin to stretch and this can result in the skin feeling dry and itchy as well as looking shiny. It can also make even the smallest cut or bruise difficult to heal. So remember to look after your skin. Always protect these areas from injury, keep them clean and well moisturised, and report any sores which are slow to heal, especially around your heels. Prevention is better than cure.

Shortness of breath, coughing and wheezing

The sensation of feeling short of breath varies from person to person. Some people may only feel breathless when they are very active or exercising to their maximum, while others may experience shortness of breath when they are at rest. Some people notice symptoms throughout the day, while others may notice them most at night. Some people find they are shorter of breath when they are lying flat and this can make it difficult to sleep. This is because the change of the position of your body may cause fluid to gather in the chest, rather than your lower body. If this happens to you, then using more pillows to prop you up a bit might make you more comfortable.

Doctors and nurses often ask questions about breathlessness to check if the medications prescribed to you are working. These may include: asking about your breathing, checking

if your sleep is becoming more disturbed at night or if you need extra pillows to help to sit more upright up in bed. They are also likely to listen to your chest to assess for signs of fluid in your lungs.

When fluid builds up in the lungs it can cause sounds such as crackling or wheezing. It can also cause irritation, making you cough and produce increased amounts of phlegm (thick, sticky spit). This tends to be white and frothy but can be pink if the cough has caused some bleeding or yellow/green if an infection has developed. **All of these are important signs to tell your doctor or nurse about.**

Feeling breathless is a very common symptom but one which can be very frightening and can cause a great deal of anxiety. If you get breathless, the 'Managing Breathlessness' section later in this manual looks at this in more detail and gives you some practical tips on how to monitor and manage breathlessness and any anxiety it causes.

Sudden weight gain

Checking for a fluid 'build-up' (fluid retention) can be difficult. Some people measure the size of their ankles or waist with a tape measure every day, while others may check if their socks or shoes feel tighter or their belt needs to be secured at a wider notch. Health professionals can check for fluid retention in several ways. These include carrying out blood tests and checking areas of the body which are most frequently affected including your lungs, liver, feet and ankles. When pressed, normal skin springs back quickly while skin that has fluid underneath often remains indented. You may notice your doctor or nurse testing this as part of your medical check-up.

However, one of the best and most accurate ways to monitor if fluid is building up is simply to weigh yourself every day. Everyone gains and loses weight throughout their lives. However, a sudden weight gain of more than 5 pounds (2.3 kilograms) over 3 days is unusual and tends to be due to fluid retention rather than overeating. This level of fluid build-up can be dangerous, but it can be controlled with medications. So, if your fluids build up it is important to take action early to prevent the need for a hospital admission. Early action can make a big difference!

If you experience severe breathing problems or if you are breathless even when resting, contact your GP or Heart Failure Specialist nurse to arrange an urgent review. If you can't wait for a call back, or think it is an emergency, dial 999.

If you experience sudden weight gain of more than 5 pounds (2.3 kilograms) over 3 days, contact your GP or heart failure specialist (or the NHS out of hours service) to arrange an urgent review. If you can't wait for a call back, or think it is an emergency, dial 999.

We will look at how to manage your fluids and other symptoms in more detail in Part 2.

Weight loss

Although less common, sudden weight loss is also important. This may be caused by dehydration, such as if you lose fluid due to a stomach bug, or if you are in a hot environment. Dehydration can lower your blood pressure, causing you to feel light headed and dizzy. If this occurs it is important to discuss it with your doctor or nurse, as your medications may need to be adjusted to help your body recover. Gradual, unplanned weight loss should also be reported. This may be a sign that your body is using up more energy than it should or is struggling to absorb the nutrients it needs. Getting advice from a heart failure specialist or diet specialists may help to put this right.

Heart rate and rhythm

Earlier we described how the heart has its own electrical pathway which passes an electrical charge causing it to contract or pump. It is this pumping action which pushes the blood around the body. In some people with heart failure this electrical pathway can be disrupted, causing changes to the speed and/or

the pattern of the heartbeat. The heart may also beat faster to try to keep up with the body's demand for oxygen. Sometimes the heart can become too fast, or the pattern can become so irregular that it causes symptoms. These symptoms may include:

- feeling the heart beating faster or pounding in the chest, or neck (palpitations)
- sensing an irregular beat or fluttering sensation
- feeling dizzy or lightheaded
- fainting

All of these symptoms are important and should be reported to your doctor as soon as possible. Some require urgent action (such as fainting), particularly if you have not had this investigated and do not know how to manage it.

These symptoms are not always caused by heart failure. Other causes can include: side effects of some medications, low blood pressure, dehydration and feeling anxious.

Having problems with your heartbeat can cause worry and upset for both you and your family. However, many of these problems can be treated. Treatments include medications and medical devices such as pacemakers and implantable cardioverter defibrillators (ICDs) that are implanted into your body to control your heart rate or rhythm. So, if you have this kind of symptom, it is worth discussing with your doctor, or heart failure specialist, or your facilitator.

If your problem is caused or made worse by anxiety, then the specially designed stress management information in the 'Managing Stress' section later in this manual can help you to gain control over the symptoms and even stop them occurring in the future.

More information about cardiac devices can be found in the 'Implanted Devices' section later in this manual.

Chest pain

Chest pain or discomfort can have several causes. These include colds, muscle strain, indigestion and 'angina' (heart pain). Even anxiety can cause chest discomfort. If you develop chest pain it is important that you seek medical advice to ensure you get the correct diagnosis and treatment.

One of the main causes of chest pain is angina. Angina is a pain in the chest that occurs when an area of the heart muscle is not receiving enough oxygen and nutrients. It causes a type of cramp in the heart muscle and just like other types of muscle cramp it should pass with time, normally within 5 minutes. It can occur when the heart needs more blood than it is getting or because the coronary arteries (the heart's own blood supply) are partly blocked.

The most common cause of angina is coronary artery disease (heart disease). Many people with angina find that it occurs most frequently when they are exerting themselves, when they are out and about on a cold or windy day, or after eating a large meal. Feeling anxious or under pressure can also bring on angina. If you are diagnosed with angina you will be given specific treatment to help control it. This may include a GTN (glyceryl trinitrate) spray or tablet. If you want to know more about coronary artery disease or angina, speak to your doctor or heart failure specialist.

Chest pain, tightness or heaviness which persists for over 10 minutes and is not relieved by your GTN spray (if you have angina) or rest is of greatest concern. Particularly if it develops suddenly or is also felt in the arm, shoulder, throat, neck, jaw, back or stomach. Other symptoms to look out for include feeling light-headed, short of breath, sick, cold and clammy or sweaty, or a general sense that something serious is happening. If you feel like this, and it is not eased with your GTN spray or rest, or if the pain is unbearable, contact your emergency services by dialling 999, and tell them that you think you may be having a heart attack.

How might my symptoms change over time?

Heart failure is a condition that usually progresses gradually over time. The symptoms can be stable (with a few ups and downs) for long periods of time. However, problems can sometimes develop which need urgent hospital care – these are often due to the build-up of fluids (congestion), or can be caused by other illnesses such as pneumonia or other heart problems.

The course of the condition is very different from person to person. This is partly due to the severity of the underlying problem (how good your heart is at pumping blood). However, the way you manage the condition and the treatment that your health care team provide can reduce your symptoms and affect the speed with which it progresses. For instance, by getting the medical treatments right and good 'self care' (following the advice in this manual) people often improve their symptoms substantially.

Can heart failure be cured?

This depends on the cause of the heart failure. If the cause of the condition is treatable, for example in the case of valve disease where surgery to replace the damaged valve is an option, it may be possible for the symptoms to be reduced and sometimes people can return to a normal or near normal life. However in many cases, the damage which has been caused to the heart muscle by the underlying condition cannot be reversed. This means that for many people heart failure is a long-term condition which requires careful management and monitoring. Although heart failure often cannot be cured, many people learn to manage it well and can live happy and fulfilling lives.



**"Your heart may not
recover its full capacity, but YOU
can recover"**

What can I do about it?

Part 2 of this manual focuses on the main things that you can do to reduce your symptoms, prevent the need for hospital admissions and reduce the impact of heart failure on your life. These include:

- taking your medications and working with your health care team to get the medications right. This might include making some changes to get your fluid balance right (to avoid congestion), or making changes to reduce any side effects
- building and maintaining a basic level of fitness (making your heart and muscles more efficient so that your heart doesn't have to work too hard leaving you with more energy and be less breathless)
- pacing yourself and having a 'Plan B' for days when you are over-tired
- managing any stress or anxiety and keeping your spirits up
- monitoring your signs and symptoms so that you can take early action if problems start to develop
- getting support and help from other people
- it is important to maintain your new routine, once the meetings with your facilitator have ended. You can continue to use the manual and build on what it has taught you by dipping into it regularly.

It is worth noting that, with the availability of newer treatments and ideas on how to manage the condition, the outlook for people with heart failure is generally much better than it was only 10 years ago. And things are improving all the time as research identifies new ways to improve the quality of life and reduce symptoms of people with heart failure.

Remember you are not alone – your facilitator and health care team are there to support you.

"I have accepted it, but it isn't going to control my life. I might set out earlier to go to town but I pace myself and I just get out and enjoy it"

A quick quiz

There is a lot of information in Part 1. Why not try this quick quiz to see how much you know about your condition?

Quick Quiz	True	False
1. Blood carries oxygen and nutrients to the whole of the body.		
2. Heart failure means that something has caused the heart to become less effective at pumping blood.		
3. People with heart failure should avoid exercise.		
4. Heart failure can cause many signs and symptoms.		
5. Chest pain is nothing to worry about.		
6. It is common to find it difficult to sleep while lying flat.		
7. Health care professionals are the only people who should manage my heart failure.		
8. Heart failure is a progressive and long-term condition. Nothing can be done.		

Answers

1. Blood carries oxygen and nutrients to the whole of the body.

True – *To do this, a normal heart beats 60 to 100 times per minute, pushing out between 55 and 70 per cent of the blood it contains.*

2. Heart failure means that something has caused the heart to become less effective at pumping blood.

True – *The two main causes are: having a heart attack and long-term high blood pressure although there are other causes. If you have heart failure, the heart pushes out less of the blood it contains in each stroke.*

3. People with heart failure should avoid exercise.

False – *This myth may seem to make sense, especially if you think that exercise would be too much for you or too tiring. However, building up your fitness (as much as is possible / within reason) is a very important way of managing your symptoms and reducing your risk of further deterioration. Even a small increase in fitness can make a difference to your energy levels and symptoms. It is important that you don't overdo it and that any activity is paced at the correct level for you. Your facilitator and the 'Keeping Active' section later in this manual will help you to decide what you can do.*

4. Heart failure can cause many signs and symptoms.

True – *These may include*

- *frequent tiredness*
- *swelling due to fluid build-up*
- *breathlessness, coughing or wheezing*

5. Chest pain is nothing to worry about.

False – *Chest pain, tightness or heaviness which is unbearable, or is not relieved by normal treatments or rest after 10 minutes, is of great concern. If you feel like this, contact your emergency services by dialling 999.*

6. It is common to find it difficult to sleep while lying flat.

True – *This is because the change of the position of your body may encourage fluid to gather in the chest.*

7. Health care professionals are the only people who should manage my heart failure.

False – *You are a key player in the management of your condition. There are many things you can do to manage your condition, including taking your medications, monitoring your fluid levels and symptoms, building up and maintaining a basic level of fitness and taking steps to manage stress and anxiety. Your family and friends can also help to support you.*

8. Heart failure is a progressive and long-term condition. Nothing can be done.

False – *The treatment you receive and the things you do for yourself can help to improve your symptoms and slow down the progression of heart failure. There is a great deal you and your health care team can do to help take control of the condition. Section 2 will help you get started.*

Next steps

Part 2 of the manual will help you to learn the skills to manage and monitor your heart failure.

- The 'Keeping Active' section will help you to build or maintain your fitness by making your muscles work better which, over time, also reduces the demand on your heart.
- The 'Managing Stress' section will help you to manage any stress or anxiety you may have.
- The 'Taking Medications' and 'About Your Medications' sections will help you to get the most out of your medications and your health care team.

- We will also discuss how to check your signs and symptoms so that you can learn what actions work best for you.

If you work through these different sections, we hope that you will get a better sense of control over your condition, reduce your symptoms and have more energy to do things in life that you enjoy.

If you have any questions or concerns about what this may all mean for you, speak to your facilitator, or your heart failure specialist or GP. It is important that you and your family share any concerns so that help and advice can be provided.

Managing Your Condition

As discussed in Part 1, for most people heart failure is a long-term condition which requires lifelong treatment and monitoring.

Having heart failure can present significant challenges to both your physical health and your mental well-being. So what can be done? How can you adjust your lifestyle to deal with these challenges? And how can you make the best use of the health care services and other support that is available to you? That is what Part 2 of the manual is all about.

It may help to think about your care as three equally important parts:

1. Health care: The treatment and monitoring that your medical care team provides.
2. Self care: Things you do to take care of yourself.
3. Help from others: The care and support you receive from partners, family and friends and any support groups or other groups that you belong to.



Medical care

When you are diagnosed with heart failure your doctor and nurse should spend time with you to discuss what has caused it and what treatments are best for you. Treatments may include the following:

- **Medication** – Most people with heart failure have a variety of medications to help control the condition. Each medication has a specific job. This can be to reduce the workload of the heart, to manage one of the causes of heart failure, to reduce the risk of future heart problems, or to control symptoms. You can find out more about your medications in the 'Taking Medications' section in Part 2 and the 'About Your Medications' section in Part 3 of the manual.
- **Medical devices** – You may have an implanted device (a device that is inserted into your body) to help control the speed with which your heart beats, or your heart's rhythm. Other devices assist the pumping action of the heart. Further information on a range of medical devices is provided in Part 3 of the manual.
- **Surgery** – Although surgery is not a routine treatment for heart failure, it is sometimes used to correct the underlying cause of the

condition, if this is possible. Surgery may involve repairing or replacing a damaged heart valve, or improving the blood flow to the heart. Improving the blood supply to the heart muscle can involve two different techniques: grafting replacement blood vessels (coronary artery bypass graft) or putting a small metal tube called a 'stent' into the heart's arteries to increase the blood flow. Any surgical treatments that are needed will be managed by your health care team.

Getting the treatment right can take time, as people respond differently to a lot of the medications used for heart failure. It may take several weeks or months for your body to adjust. This means that the medications and devices need to be adjusted from time to time to find the right dose (or settings) for you. Indeed, this can happen quite a lot when you start taking a new medication. You may also need a series of blood tests to help the doctors get the dosage right.

Remember, the cause of heart failure and the treatments needed vary from person to person, so try not to compare yourself to someone else you know with the condition. Not everyone is treated in the same way!

Self care

What do we mean by 'self care'? Most people take actions every day to care for themselves and prevent ill health. This ranges from simple things like washing and brushing our teeth to more complicated things like deciding what to eat and drink, or keeping fit. When we become ill, we seek help or treatments which we hope will lead to a speedy recovery. We try our best to use the treatments properly and monitor our progress to ensure we don't get any worse. If you have heart failure, good self care is very important. The things that you do to keep track of changes and to help keep the symptoms under control will help you to feel better and live longer. Studies of people who live with heart failure have shown that good self care enables them to:

- reduce the bad days
- increase the good days
- do more of the things they want to do
- reduce the need for hospital admissions
- reduce symptoms
- live longer

"If you have heart failure, your life is far from over"

Heart failure is a complex condition. Just looking at the size of this manual and the amount of information it contains may make you and your partner or family question your ability to cope. But remember, hundreds of thousands of people are living with heart failure in the UK today. A lot is known about how to manage the condition and many people with heart failure have happy and fulfilling lives. Learning to live with heart failure takes time; sometimes things go well and other times we learn from our mistakes. This programme has been specially designed to help you to gradually build your self care skills and make self care part of your routine.

Here is a list of things you can do to help manage heart failure:

- learn about the condition and how best to manage it
- take your medications to help keep your symptoms under control
- stay active to keep your fitness at a good basic level
- take steps to manage stress and anxiety
- monitor your symptoms and know when to take action or seek help
- keep an eye out for any changes in swelling and weigh yourself daily
- take steps to keep your spirits up (managing low mood or depression)
- pace yourself – make the most of your good days, but don't overdo it

- eat healthily
- stop or reduce smoking
- avoid excessive use of alcohol
- communicate well with your health care team – let them know if you have any concerns, about any possible side effects of your medications, if you get a cold or infection, or if your symptoms worsen
- find other people who can help. This might involve a partner, family members or friends. You might also find a local support group to join.

There is a lot you can do! And you are not alone – your facilitator, this manual and the rest of your health care team are here to help you to manage your situation and achieve the best possible control of your heart failure.

The rest of this manual deals with all of the above self-care topics.

- Part 2 deals with topics that are important for almost everyone with heart failure such as Taking Medications, Staying Active, Managing Your Fluid Balance, Managing Stress, and Managing Breathlessness.
- Part 3 includes further information about topics that are important for some people, but not everyone, so you may want to dip into these or leave them depending on your own situation.

Taking Medications

Taking any regular medicine will mean that you need to know what the medicine is, the amount that you need to take and the time that you need to take it. As you will have been given several different types of pills which do different things, it can be a challenge trying to keep track of what they all do and why you are taking them. This section will help you to manage your medications and to manage any changes that need to be made over time.

It is important to remember that there are several people who can help if you have any questions or need some support to take your medicine correctly. Your local doctor, heart specialist and nurse can help provide you with information and tips. Your local pharmacist can also offer support and advice as well as special packaging or boxes which help many people manage their pills. If you need more help with organising your medications why not seek out some assistance from a family member or friend? This can help build your confidence and reassure you that you are taking everything correctly. Taking your medication will soon become as routine as brushing your teeth in the morning and at night.

Most people who work with or have experience of heart failure agree that taking the medication is one of the most important steps for controlling symptoms of heart failure and maintaining a good quality of life. But you may want to ask a few questions before you start taking any new medicine, or if changes are made to the ones you normally take.

Don't be afraid to ask the doctor these 3 questions:

1. What are my options?
2. What are the benefits and possible risks?
3. How likely are these risks and benefits?

This will give you a chance to both discuss any concerns you may have and help the doctor prescribe the medication that suits you best. It may be helpful to bring a friend or family member with you. A space to take notes has been provided in the medications table in your 'Progress Tracker'.

The following checklist will help you and your facilitator to identify if there are any problems with your medications that could be worth discussing.

"At first I didn't like the idea of taking all these medicines but I realise now that the tablets can help me to have a good life"

**“Did I take them
or not?”**

Action	Please circle		
1. I know what medications I am on.	yes	no	not sure
2. I understand what each of my medications is for.	yes	no	not sure
3. I try to take my medications as prescribed.	yes	no	not sure
4. I am fine about taking all these medications.	yes	no	not sure
5. I recognise all the tablets.	yes	no	not sure
6. I know if I have forgotten to take a tablet.	yes	no	not sure
7. The packaging is easy to open.	yes	no	not sure
8. I know what to do if I forget a pill.	yes	no	not sure
9. I know what side effects to look for.	yes	no	not sure
10. I know what to do if I experience side effects.	yes	no	not sure
11. I know why the dose of my medications is often changed.	yes	no	not sure
12. I have a regular pharmacist that I use.	yes	no	not sure
13. I would contact my doctor or nurse if I have a cold or stomach upset.	yes	no	not sure
14. If I buy any over the counter medications I always check if they could affect the way my other medications work.	yes	no	not sure

If you have answered 'no' or 'not sure' to any of the questions please read the practical tips below. Show this list to your facilitator who will be able to discuss anything you are not sure about.

Many people agree that it isn't always easy to take their medication as prescribed and so we have provided some practical tips to help with this.

"My father is a retired GP; even he has trouble telling one pill from another sometimes!"

Practical tips

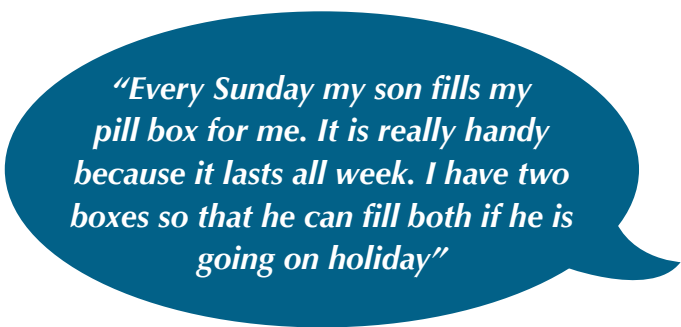
1. Make a list of your medications and keep it somewhere handy (perhaps on your phone or with your medications). This will help you to check what you are supposed to be taking and can be useful to show to any doctors or nurses that you visit. Some labels may be difficult to read as the print can be very small. If this is a problem, why not ask someone to write this information down for you. Remember to take this list with you, particularly if you are going to a doctor's appointment or clinic. Some people find it easiest to carry a copy in their purse or wallet.
2. Try to plan your medications around your normal daily routine. This will make it easier to remember your pills and you will get into the habit of taking them. Some people find it helpful to plan their medications around meal times. This is particularly useful if you have to take your medications either before or after eating.

Anything that acts as a reminder can be very useful; setting an alarm on a mobile phone or even writing yourself a note and leaving it in a place where you will see it regularly.

3. If you struggle to keep the medications organised, it might be worth asking your pharmacist for a pill box – sometimes called a ‘dosette box’ – or a pre-packed medication system. These systems help to get your medication for the whole week in order, as they have separate compartments for times of the day and days of the week. They are also great if you ever struggle to remember if you have taken your medication. You can simply check by looking to see if the space for the pills is empty or not.

If you feel confident you may wish to fill the box yourself, or you may want to ask a family member for some assistance. Many pharmacists now offer this kind of service free of charge. Why not check with your regular pharmacist to see if they can do this for you?

4. Use your calendar, diary or mobile phone to remind you when you need to order a new prescription. Be careful to avoid any public holidays as pharmacies and doctors’ surgeries are likely to be closed.



“Every Sunday my son fills my pill box for me. It is really handy because it lasts all week. I have two boxes so that he can fill both if he is going on holiday”

Changes to your medication

Sometimes your medicines need to be changed quite regularly. This may appear strange, but it is quite normal – this is part of finding the best treatment for you. Lots of research has been carried out to find the best way to manage heart failure. This can also result in changes to your medications as time goes on.

Many of the medicines which help control your symptoms and reduce the progression of heart failure can have an effect on your blood pressure and how your kidneys work. They can also affect your body’s minerals, especially sodium and potassium. It is therefore important that your body does not have too many changes made too soon. This allows your body to gradually adjust to the medicines you are taking. Making gradual changes to find the best dose for you is known as **‘titration’**. It may be helpful to think of medication titration as using a spirit level to get a picture straight. By moving the tool a little higher or lower you can get the bubble right in the middle or ‘just right’. When making changes to get your medications ‘just right’ your doctor or nurse specialist won’t use a spirit level but will use three main tools:

1. Blood tests.
2. Blood pressure readings.
3. Observing your signs and symptoms.

Getting your medication 'just right' can be a slow process, but as most people with heart failure would agree, it is worth taking things slow and steady. Once the best dose of each medicine is reached you will find that very few changes will be required. Your treatment will however continue to be monitored with regular blood pressure and blood tests being carried out. In general, this is required every 6 months but speak to your doctor or nurse to find out how often you should be reviewed and make a note in your diary to remind you.

The most frequently adjusted (or titrated) medicines include:

- Angiotensin Converting Enzyme Inhibitors (ACE inhibitors)
- Angiotensin-II Receptor Blockers (ARB's)
- Beta-blockers
- Diuretics ('water tablets')
- Aldosterone Inhibitors or Mineralocorticoid Receptor Antagonists (MRAs)

We know that some people decide to change the amount or timing of their medications for various reasons – to suit their own lifestyle or sometimes because they feel that some medications are not working. However, if you have heart failure, this can be dangerous. You should not make any changes to your medication unless these have been agreed with your GP.

If you are concerned about any of your medications or think that the number or frequency of taking them needs to be changed, please discuss this with your GP or heart failure specialist.

When changes are made to your medication or the dose that you should take, just update your medication list or record. Record the date for your next review, and any tests required (blood pressure and/or blood test) in your diary. The information on changes made will help your doctors, nurses or pharmacists to see how your titration is progressing. This can provide a useful record, if you are being seen by several different health professionals.

Other medications

Many people living with heart failure are also taking medication for other health conditions.

It can be useful to keep a list of all your medicines in one place.

It is important that you never take additional medicines without first checking with your doctor or pharmacist. Some 'natural' or 'self-help' treatments that you can buy in chemists or health food shops are not suitable to take in combination with the medicines you have been prescribed. They may produce side effects or weaken the effects of the medication. These may include cough remedies, vitamin supplements and herbal treatments. Occasionally some types of food or drink should also be avoided (for instance grapefruit can alter the way certain cholesterol medications work). Your doctor or pharmacist can check which pills you are taking and recommend what is best and safest for you. The pill packets will also say if there are any foods that you should avoid.

Carrying a list of all your medications with you can be extremely helpful. A list can also help if you have to go to hospital or if you ever forget or run out of your medications when you are away on holiday. Giving a copy to a family member or friend just in case can also be useful, especially if you need any help with your pills in the future.

Monitoring side effects

Many medicines have the possibility of causing some side effects but most side effects are not serious. If you are having any side effects it is important that you discuss them with your doctor or nurse. There are good reasons for you to have each of the pills you are taking, but if you are feeling unwell then your doctor may be able to prescribe something else. There is often an alternative so don't be afraid to ask.

The possible side effects of each medication are written on the information leaflet or on the side of the box or bottle which contains your medication. Remember to read this carefully.

Further information about possible side effects is available in the 'About Your Medications' section.

Until you are used to the medication it may be difficult to tell what is a symptom of heart failure and what is a side effect of the pills you are taking – for example feeling dizzy or faint may be either a symptom or a side effect.

Note down whether you notice any side effects, symptoms or sensations in your Progress Tracker. If you do, it is also useful to write a note next to your current medications in your medication list (or in your diary). This will help your doctor and nurse to monitor your progress and see if any medication changes are needed. This will help your

doctor and nurse to monitor your progress and see if any medication changes are needed.

Notes/possible side effects

Saturday first thing in the morning felt very dizzy when I got out of bed. Had to sit down on the edge of the bed until it stopped. Lasted about 5 minutes.

ACTION

If you develop sudden or severe side effects such as

- swelling of the mouth, face or lips,
- widespread itching of the skin or a rash,
- wheeziness, shortness of breath or difficulty breathing

contact your doctor or emergency service immediately.

Stopping your medication

It is important to remember that your medications are given to you to manage your heart failure and control your symptoms. It can be dangerous to stop some pills suddenly because your body may need time to adjust to doing without them. It is therefore essential that you do not stop any of your medication without discussing it with a doctor. Remember, side effects often will ease with time as your body adjusts. Alternative

medications can often be prescribed. So, always discuss any concerns you have with your doctor or nurse.

Missing or forgetting medication

Missing or forgetting medication is a common problem for people living with a long term condition. Indeed, forgetting to take medications or taking them incorrectly is one of the main reasons for hospital admission in people with heart failure.

Many people are concerned about taking medications and what it will do to them. Some are afraid that they may become 'addicted' to certain tablets. These feelings are normal. Most people do not like taking tablets as it can feel 'unnatural'. However, many medicines have been around for a long time. The humble aspirin was first used over one hundred years ago and is taken by millions of people who are at particular risk of some health problems. Medicines do sometimes have side effects – even aspirin can cause irritation of the stomach and other problems. What you and your doctor have to do is to weigh up the risks versus the benefits, so remember, ask the three questions mentioned in the earlier section on 'Taking Medications'.

We can also be more forgetful when we are busy or stressed. If this is the case for you, it might be worth taking some steps to reduce your stress levels. There are some ideas about how to do this in the later section on 'Managing Stress'.

Getting into a good routine and using simple systems to make sure that you take your medications at the right time can help. This will not only make taking your medicines easier, it can reduce worry, and prevent hospital admissions.

Forgetting to take your medication can cause a great deal of worry. Knowing what to do if it happens to you can help. The correct course of action might be different depending on the type of medication it is and when you last took a dose. So, you should contact a health professional to ask for advice. **Never take a double dose to 'make up' for the one you missed.** The most useful contacts are your pharmacist, your doctor or your heart failure specialist. If it is outside of normal working hours, the NHS has nurse-run helplines (like NHS 24, NHS Direct or NHS 111) which will be able to offer you advice.

Changes to your daily routine

Plan ahead for any changes to your routine which may affect how or when you take your medication. Changes can occur for many

reasons: hospital appointments, a day trip, birthday party or holiday. It is very important of course to make sure that you have a sufficient supply of all your medications. If you are going to be away from home for a longer period of time and it is around the time of a medical review or planned change in your medication, speak to your doctor or heart failure specialist before you go. He or she will be able to advise you as to the best course of action.

If you are flying, keep your medication in your hand luggage in case you are delayed or your luggage is lost, and always take a list of your medication, as you never know when you may need it. If you are going abroad, you should check in advance what rules apply to taking your medications into any countries that you are passing through, as some medicines used in the UK may be controlled in other countries.

It is worth checking with your GP surgery if you need to make any special arrangements to travel abroad.

ACTION

Go to the 'My Health Care' page of your Progress Tracker and write down these important contact numbers (for your pharmacist, GP, heart failure specialist and NHS out of hours service). If you are unsure, check your telephone book or the internet, or ask a family member or your facilitator to help you.

Some extra tips

- If you are given a new prescription especially after a hospital stay, double check all the medications you have to take and return the old medicines to your local pharmacist. Having old medication lying around the house can lead to confusion.
- Never take medicines which have been prescribed for someone else. It's not worth the risk.
- Everyone experiences a stomach upset at some time in their lives, but this can affect the amount of medication your body receives. If you suffer from vomiting or

diarrhoea speak to your doctor or nurse as soon as possible. They will guide you as to the best course of action.

- Many pharmacies now offer a home delivery service. If you are unwell or struggle to get out and about, why not ask if your local pharmacy can provide this service?

Getting vaccinated: Prevention is better than cure

One of the simplest ways to take care of yourself is to get annual vaccinations for influenza (flu). There is also a pneumococcal vaccination which does not need to be administered every year but may need to be 'topped up' should your resistance to

the condition become low. These injections do not always prevent the infections from occurring but they can reduce the need for intensive treatment and prevent a hospital admission. The NHS usually offers free vaccinations in late autumn to early winter. Check with your local doctor at this time to get your free vaccination.

Medication Quick Quiz	✓ Tick the answers you think are correct
If my medication is changed it means my condition is getting worse.	
If I forget my medication it is OK to take it as soon as possible.	
If I have side effects I should stop taking my medication.	
Over the counter or herbal remedies can affect my medication.	
I should just put up with the side effects.	
What I eat or drink can affect my medication.	
The more pills I have to take the worse my heart failure is.	
The larger the dose the worse my condition must be.	
I only really need to take the tablets when I have symptoms.	

Turn to the next page to find out how much you know about medication

Answers

If my medication is changed it means my condition is getting worse.

No, medication is often changed to get the best treatment to suit you. It sometimes takes time to get the right combination of tablets.

If I forget my medication it is ok to take it as soon as possible.

Not necessarily, it depends on when you took the last dose and the type of medication. It is best to check with your healthcare professional if this happens. Never take a double dose.

If I have side effects I should stop taking my medication.

If you develop severe side effects you should contact your doctor or emergency service as soon as possible. For other milder side effects contact your doctor, nurse or pharmacist for advice on what to do. Please also see the later 'About Your Medications' section.

Over the counter or herbal remedies can affect my medication.

Yes, a number of these can affect the way your medications work. These can include cold remedies, ibuprofen and aspirin. Check with your doctor or pharmacist before you use non-prescribed drugs or herbal remedies, food supplements or even vitamins.

I should just ignore the side effects.

No, it is always best to check with your doctor or pharmacist if you are having side effects.

What I eat or drink can affect my medication.

Yes, some foods and drinks can affect how your medication works. Please read the instructions on the packets and also read the 'Understanding Medication' section.

The more pills I have to take the worse my heart failure is.

Not true, many medications can be prescribed for heart failure. This is because they work in different ways and deal with different causes and symptoms of the condition.

The larger the dose the worse my condition must be.

*This is **not necessarily true** either. Lots of research has been carried out to identify the best dose to slow down and manage the condition. Higher doses may also be prescribed if your symptoms become particularly troublesome, but are often reduced again once you improve.*

I only really need to take the tablets when I have symptoms.

*Certainly **not true**, the tablets help control your symptoms and these may return if you stop the medication.*

Keeping Active

Why exercise is so important

Regular exercise is good for you!

It improves both your physical and mental health. For people with heart failure, exercising at least three times per week, will:

- help you to get around more easily
- reduce your symptoms – as you take more exercise and become more active, you will feel less breathless and have more energy
- reduce stress and improve your sleep
- increase the blood flow throughout your body and lower your blood pressure. This will help to reduce the workload on your heart during daily activities and prevent more heart problems in the future
- over time, it will improve the flexibility and movement of your joints and reduce aches and pains

No-one is expecting you to become an Olympic athlete and you should definitely

not overdo things. However, the better you can make your basic level of fitness, the more you will be able to do and the better you will feel. Being fitter will reduce the strain on your heart when you're doing everyday things like climbing stairs, getting up from a chair or carrying things. More importantly, it will help you to do more of the things that you enjoy.

It is worth taking a little time to think about what it would mean for you if you were to engage with exercise on a regular basis and were a little fitter.

....This might include simple things like being able to manage better in your day-to-day life, or being able to get out and about a bit more. Or it might enable you to do something more ambitious such as joining a local group, or doing activities with your family. It may be useful for you to set a 'long-term goal' – something that you struggle with at the moment that you would like to be able to do better.

The most important aspect of undertaking exercise is working at the right intensity to improve your fitness and health....when you exercise, you need to breathe heavier, feel warmer and have a faster heart beat – you should be a little out of breath, but still able to carry on a conversation. This is what we call working at a 'moderate intensity'. Any amount of exercise is better than none, but the best results come from exercise done at a moderate intensity.

*As long as you can
talk at the same
time it is OK to
feel slightly out of
breath.*





How does it work?

Strong scientific evidence tells us that:

1. The heart is the most efficient muscle we have and it can adapt really well to changes in the demands we put on it.
2. With regular exercise, the muscles in your arms and legs can become more efficient allowing you to do more work or the same amount of work for less effort.
3. As you work towards a basic level of fitness, your heart will become better at supplying the energy you need to meet the demands of daily life – you will be able to do the same work for less effort.

With regular exercise, people with heart failure can better manage their condition and improve the amount of activity they can do. The aim of our exercise programme is to get your body (and mind) into good enough shape so that you can get some of the benefits above. It is amazing how much you can gain with each week of exercise.

In this manual, we offer a choice of exercise options to help you to build up and maintain your fitness and strength. The exercise programme lasts between 2 to 3 months and the longer you stick at it, the more you are likely to gain.

Your facilitators will help you with making decisions around what is best for you. The two options are:

1. **Our walking programme:** Walking is a great way to exercise. If you take this option it is important to keep track of your level of effort, so that you can gradually build your ability to exercise safely. The aim will be to gradually increase the number of steps you take and the time you spend walking. This option will suit you if you are happy to walk outside on your own, even if just for a short while.
2. **Our chair-based exercises:** The chair-based programme will guide you through a series of exercises that you can do in your own home. There are seven stages, each increasing in time and the amount of effort needed. How you progress will depend on how easy or hard you find each stage. This option also includes resistance exercise (using your body weight) which we know helps to maintain or in some cases improve muscle strength. This doesn't mean you have to lift heavy weights! Instead we steadily use more and more of your body weight during movements that you would routinely use in your daily life. Adding resistance in this way means that the benefits from your exercise carry over directly into your daily life.

As we will see later, you can combine sessions from both the chair-based and walking programme if you like.

Your walking programme

What you will need:

- a good pair of training shoes is best, but comfortable shoes or trainers, properly laced, will be fine
- comfortable clothing
- some ideas about where you can walk safely and easily and where you might enjoy walking
- a way of tracking your progress (see below)

Step 1: Getting started

Your facilitator should help you to get started and you should discuss ...

- How much to do. This should be recorded on your 'Exercise Record' in the Progress Tracker. At the start, your exercise goals will depend on your current fitness. A good indicator of this is how far you can walk in a certain time, or how long you can walk for without needing to rest.
- Where you are going to walk on a regular basis that is safe and that might be enjoyable or useful to you in some way. Starting with somewhere that is not too hilly is probably a good idea.
- **Warming up before you exercise – this means doing 3-5 minutes of slow walking indoors or outside. And you should do the same to cool down after.** Think of it like driving a car – if you keep starting your car in fourth gear, the engine won't last very long. It's the same with your heart – you need to start off gently and 'go up through the gears'.

- How to make sure that you are working at the right intensity to build up your fitness.
- How to keep track of your progress. This might involve recording the time you walked for or using a step-counter (called a pedometer). Keeping track of your progress over time can be very motivating.

The walking programme involves going out walking at least 3 times a week. Ideally, you should spread the activity across the week, allowing a rest day in between each session. Your starting point will be one of the following ...

LEVEL ONE:	5 to 10 minutes
LEVEL TWO:	10 to 15 minutes
LEVEL THREE:	20 minutes and above

These are the times that you should walk for in addition to the warm-up and cool down time.

Each session will consist of walking outdoors. Pick somewhere that you can get to and from easily. You should also try to pick a place where you might enjoy being and where it is easy to end the walk if any problems arise. For example, local parks, beauty spots or shopping centres, with plenty of benches. It may be worth considering what shortcuts there are back to home (or your transport home). Remember that wherever you walk to, you will need to be able to get back!

If you are uncomfortable with the idea of walking outdoors on your own, it might help to walk with a companion or to take a mobile phone with you and make sure that somebody knows you are out walking.

**“You need
to get a bit out of breath,
but not so much that you
can’t talk”**

Remember, you should walk at a pace that makes you breathe heavier, feel warmer and have a slightly faster heartbeat. But you should still be able to talk. This means you are working at a ‘moderate intensity’ – you need to work at this level to build up your fitness. Always cool down after exercise as your heart needs to come gently back to a normal rate.

Step 2: Building up your walking fitness

You should stay at the same level until you feel ready to increase your walk time.

- *If you are struggling* to complete the walk or getting too exhausted, you may need to reduce your walking time and build up from a lower starting point. The important thing is to keep working at the right intensity (without over doing it).
- *If you found your last two or three walking sessions easy*, you are progressing and you should increase the length of time you walk for.
- If you want to increase your fitness further, you can add in some ‘faster walking’ sections to your walk. You can start with 2, 3 or 5 minutes of faster walking. Or you can set challenges based on getting from one landmark to another at a faster pace. Another way to increase the challenge is to change your route to make it more hilly.
- **Once you are doing 20 minutes three or four times a week (excluding warm-up and cool-down time), you will have achieved a basic**

level of fitness that is good for your heart and for your health in general – well done!

- This is a good basic level to maintain over time but if you feel able, you could build up to 30 minutes per session.
- You can replace any number of minutes of walking with other ‘moderately intense’ activities if you prefer.

It may have been a long time since you did any regular exercise. This isn’t a problem. Our experience tells us that no matter what level you start at, you can make progress. It’s all in your control and you can progress at your own pace. Your facilitator is also here to help!

You can use the Progress Tracker to record how long you walk for and how easy or hard each walk was. This will help you to see when it is time to increase your target. Over time, you may find that you need to walk faster to still be working at a moderate intensity (out of breath but able to talk) – that is normal and means that your fitness is increasing!

Depending on your age and your overall health you may not be able to get as far as 20 minutes per session. If that is the case, don’t worry – any increase in activity is good for you. The aim is to build up your fitness as much as is reasonably possible. Once you have reached a level that is as good as it can be, then the important thing is to keep it up!

If you are concerned that the amount of activity you are doing is too much, please discuss this with your facilitator, your GP or your heart failure nurse.

Monitor your progress

It can be helpful to keep track of your progress as this allows both you and your facilitator to see how your fitness is changing over time. There are many digital activity monitors (often called activity trackers) available. These include clip-on or wrist-worn pedometers and smartphone apps. There are also ways of monitoring your activity without digital trackers. It will help to choose a way of monitoring your activity that you are most comfortable with. You can monitor your progress by ...

- thinking about the amount of effort you are making – how easy or hard you are finding it
- keeping track of the amount of time you are walking for
- using a pedometer to count the number of steps you take
- you can also use a pedometer and a watch or a stopwatch to work out how many steps per minute you are doing (your walking pace) or measure distance over time instead of steps

These are all useful ways to see if your basic fitness is increasing and to see when you should move up a level. Your Progress Tracker allows you to record any or all of the things above and some digital trackers or mobile phones can automatically do this. You may find it useful to discuss your progress with your facilitator each time you meet. Your facilitator will be able to help you choose the right monitoring tool or show you how to monitor without a digital tracker.

What to do if you get very out of breath

It is normal for your breathing to get faster when you exercise and indeed it is important for you to work at a level which does increase your rate of breathing. However, if you work too hard you may find that your breathing gets too fast and you start to feel increasingly breathless. If you get to the point of being so out of breath that you can't speak, this is a sign that you are working too hard and should slow down.

At this point during your walk or exercise, you could **try using the 'active recovery' technique below to manage your breathing...**

- Monitor your breathing rate and slow down your activity as this will help you gain control of your breathing. Try to do this without stopping the exercise, but if your breathing doesn't settle, it is OK to stop!
- You should notice your breathing rate gradually slowing down and coming back to the right level – breathing hard but still able to speak.
- If this doesn't work then slow down further until your breathing comes back to a comfortable rate.
- The trick is to take action before you get so out of breath that you have to stop – by slowing down you are recovering while being active.

If at any point during exercise your breathlessness becomes distressing, then stop

*Your
energy for the week is like
having money in the bank. Don't
spend it all at once or you'll go
overdrawn!*

and concentrate on slowing your breathing to a normal rate. You can use the relaxation exercises in the 'Managing Stress' section later on in this manual.

Please note, increased breathlessness at rest, or with small amounts of exercise, may be related to fluid retention, if you are breathless and do not think it is due to exercising, please contact a health care professional.

Helpful hints

1. Plan your week and fit the walking into your everyday routine. You're more likely to stick to the plan if you have a routine, if the walk is enjoyable or if the walk helps you to do something you need or want to do regularly (like visiting a friend or relative, or going to the shops).
2. Have a back-up plan in case of bad weather. How else could you get 10, 15 or 20 minutes of moderate intensity activity? How about doing the chair based exercises? Or, going to a leisure centre or an exercise group. Your facilitator may be able to give you information on local exercise groups.
3. It is good to challenge yourself, but don't overdo it, as this can make you exhausted the next day. Bear in mind what other work or household jobs you need to do and plan accordingly.
4. As a general rule, try not to do things that make you so breathless you can't speak. If you get to this point, you should probably slow down a bit.
5. Vary your time, pace and course to challenge yourself and keep it interesting.
6. If you wish, you can combine sessions from the chair based exercises and the walking programme. You first need to set a starting point for the new programme (discuss this with your facilitator). Then it is very straightforward – any session of walking can be switched for a session of the chair based exercises (and you can switch 2 or all 3 sessions if you like).
7. If you have chest pains when exercising and your GP does not already know about this, you should stop exercising immediately. Contact your GP as soon as possible to discuss this and your facilitator.

If you have angina and you experience chest pain during exercise, stop exercising immediately. Take 1-2 puffs of your GTN (glycerine trinitrate) spray if that has been prescribed and wait for the pain to go away. Take a second dose after 5 minutes if you still have pain. Dial 999 if your pain is still not relieved after using your GTN spray twice, or if you experience unbearable chest pain at any time. If the pain goes away and you feel OK, you can carry on, but wait a few minutes and work within the limits of your angina.

8. If you have nausea, dizziness or palpitations (feeling your heart beating abnormally fast or irregularly) when exercising, stop immediately. If these symptoms don't go away, contact your GP as soon as possible.
9. If you have been prescribed a GTN spray, always carry it with you, especially during exercise.
10. If you have run out of your usual medications, or have forgotten to take them, do not exercise until this has been sorted out.
11. Talk to your facilitator or GP if you have any concerns.

Step 3: Keeping going

Walking is a great way to maintain or increase your basic fitness. Hopefully, after a few months of regular exercise, you will be feeling the benefits in terms of getting around more easily, sleeping better, improved mood and having fewer symptoms.

Once you have built up your basic level of activity, you need to think about keeping it at a good level – not just for a few months, but for the rest of your life! You can maintain your fitness by keeping going with your walking programme, your chair based exercises, or by doing other activities if you prefer. Depending on your own situation and the kind of activities that you enjoy, you may want to do other things, like joining an activity group, cycling or dancing.

The key question is: what activities can you do that you are likely to be able to keep doing on a regular basis? Keeping your basic fitness requires regular activity – if you stop doing activities your fitness will soon deteriorate. Have a look at the ideas for physical activity below and decide what you want to do to keep your fitness up. Most people find that it is easier to keep going with activities that they enjoy or that involve other people or that they can easily build into their usual routine (like using walking to get to work, taking the dog for a walk or joining a weekly activity group).

Remember to keep your fitness up you should be aiming for at least 60 to 150 minutes per week (depending on your starting level of fitness) of moderate intensity physical activity each week. So make sure that the activities you choose include this amount of time *working at a level that makes you breathe heavier, feel warmer and have a slightly faster heartbeat, but still allows you to talk.*

Feel free to use the planner to make a weekly goal and to track your progress and most of all enjoy the benefits that staying fit and active can bring – remember 'Movement is Medicine'!

Ideas for physical activity

- Keep on walking! This could be more enjoyable if it is shared with someone else?
- Try a few different activities until you find something you can stick with.
- Join an exercise group for people with heart failure or other heart problems – your

facilitator should be able to tell you what is available in your local area.

- Make walking part of your daily routine, such as how you get to work, or to the shops, or to make a regular visit to a friend or family.
- Dogs make great personal coaches as they always need walking! If you don't have a dog, do you have a friend or neighbour or family member who has a dog that would like a bit of extra walking?
- Join a club or start a regular social activity (for example bowls or darts or a singing group) that you can walk to and from. Doing things with other people can be a lot more fun!
- Do some other moderate intensity activity like cycling, swimming or dancing.
- Make it fun or make it social – what would you enjoy most?

Swimming

Swimming or other water based exercise (e.g. aqua aerobics) is enjoyed by many people and can be done safely as part of your programme. However, in some people, swimming or other water based exercise can put a higher burden on the heart than other forms of exercise, particularly if you have not done it for a while. If you would like to do these types of exercises please discuss this with your REACH-HF facilitator, GP, or heart failure specialist first.

The REACH chair based exercises

The chair based exercise videos that come with this manual are designed especially for people with heart failure. The videos contain a series of exercises (level 1 to 7) that use body weight to gradually increase the workload as you progress. Each level uses your own arm and leg weight to increase the amount of resistance you feel when carrying out each series of movements. This will make your muscles work more efficiently reducing the burden on your heart, so you will be able to do day-to-day things more easily. The exercises are organised into seven levels of increasing intensity – the lower levels can be achieved by most people but the higher levels are quite challenging. The initial levels are chair-based, but the more advanced levels require you to stand. The exercises are demonstrated in the videos. Each level of the programme includes a set of warm-up and cool-down exercises.

What you will need:

- comfortable clothing, including shoes or trainers, properly laced
- a robust chair, ideally one with arms (but a chair without arms will do)
- The REACH exercise videos

To access the chair based exercises online go to www.reachhf.co.uk

Password

Your facilitator may give you a different website address.

Step 1: Getting started

Your facilitator should help you to get started and you should discuss:

- Which level to start at. This should be recorded on your 'Exercise Record' in the Progress Tracker. At the start, your exercise goals will depend on your current fitness. A good indicator of this is how far you can walk in a certain amount of time or how long you can walk for without needing to rest.
- How to make sure that you are working at the right level of intensity. This is done through an assessment of your fitness using a walking test or through a series of questions by your facilitator about your daily activities. This information is used to prescribe a starting level for your exercise programme.

During the programme your facilitator will work with you to develop the skills to know when to progress or maintain the level of exercise. This will include using things like the talk-test, the 'Effort Scale' and importantly, acquiring an awareness of how you are feeling during and after exercise.

You should aim to do three exercise sessions each week. Ideally, you should spread the activity across the week, allowing a rest day in between each session.

Step 2: Building up your fitness

The intensity of exercise will be guided by the videos, which include prompts (both visual and audio) to help you assess your ability

to carry out exercise at this level. However, you should keep track of how much you are exerting yourself – Each level of exercise includes a warm up period which enables your heart muscle and the muscles in your legs and arms to get sufficient blood in readiness for the main exercise period. **The main exercise period should eventually make you breathe heavier, feel warmer and have a slightly faster heartbeat, but you should still be able to talk. This means you are working at a ‘moderate intensity’ – the level you need to be able to gain the most from exercise.**

Throughout the CBE level you will be prompted on the screen to consider how much effort these movements are requiring of you. Try to keep the effort to around 7 out of 10 using the Effort Scale.

You should stay at the same level until you feel ready to increase the level.

- *If you are struggling to complete the session or getting too exhausted, you may need to reduce the level you are on and build up from a lower starting point.*
- *If you found your last two or three sessions easy, you should start working on the next level (for instance you might progress from Level 3 to Level 4). As you move into level 4 and above of the chair based exercises, the movements, in the main part of each level, will start to introduce greater amounts of resistance particularly through your legs. This will make the exercise feel much harder compared to levels 2 to 3 and may make you experience higher*

rates of breathing. This is not a problem, but is something to be prepared for as you progress to these levels.

- Once accustomed to exercise some patients have enjoyed and benefited from using a combination of the chair based exercise and walking programmes. So, if you want to add some variety to your training, do talk with your facilitator about adding in either chair based exercises or walking to your exercise programme.
- **If you follow the chair based exercises for 10 to 12 weeks, you should achieve a basic level of fitness that is good for your heart and for your health in general.**
- You can build up your exercise routine further if you wish, or you can just maintain your fitness by keeping going at the level you have reached.
- At this point, you can also replace the exercise sessions with other ‘moderately intense’ activities if you prefer. If you do this, you should aim to replace each session with a similar amount of time of ‘moderately intense’ physical activity.

You can use the Progress Tracker to record what level you are working on and how easy or hard each session was. This will help you to see when it is time to increase the level.

Depending on your age and your overall health you may not be able to get to the higher levels of the chair based exercises. Don’t worry, that is perfectly normal. Any improvement from where you started means

that your fitness has increased and that is good for you – the aim is to build up your fitness as much as is reasonably possible. Once you have reached a level that is as good as it can be for you, then the important thing is to keep it up after your facilitator is no longer working with you!

If you are concerned that the amount of activity you are doing is too much, please discuss this with your facilitator or your GP.

Monitor your progress

It can be helpful to keep track of your progress as this allows both you and your facilitator to see how your fitness is improving over time. You can monitor your progress by ...

- the amount of effort you are making – how easy or hard you are finding it
- the amount of time you are exercising for

Your Progress Tracker allows you to record this and you may find it useful to discuss your progress with your facilitator each time you meet.

What to do if you get very out of breath

It is normal for your breathing to get faster when you exercise and indeed it is important for you to work at a level which does increase your rate of breathing. However, if you work too hard you may find that your breathing gets too fast and you start to feel increasingly breathless. If you get to the point of being so out of breath that you can't speak, this is a

sign that you are working too hard and should slow down.

If you start feeling that you are getting seriously out of breath and may be approaching the point where you can't speak, you could **try using the 'active recovery' technique below to manage your breathing**

...

- Monitor your breathing rate and slow down your activity as this will help you gain control of your breathing. Try to do this without stopping the exercise, but if your breathing doesn't settle, it is OK to stop!
- You should notice your breathing rate gradually slowing down and coming back to the right level.
- If this doesn't work then slow down further until your breathing comes back to a comfortable rate.
- The trick is to take action before you get so out of breath that you have to stop.

If at any point during exercise your breathlessness becomes distressing, then stop and concentrate on slowing your breathing to a normal rate. You can use the relaxation exercises later in this manual to help you to do this.

Helpful hints

1. Plan your week and fit the exercise time into your everyday routine.
2. It is good to challenge yourself, but don't overdo it, as this can make you exhausted

Your energy for the week is like having money in the bank. Don't spend it all at once or you'll go overdrawn!

the next day. Bear in mind what other work or household jobs you need to do and plan accordingly.

3. As a general rule, try not to do things that make you so breathless you can't speak. If you get to this point, you should probably slow down a bit.
4. If you wish, you can combine sessions from the chair based exercises and the walking programme. You first need to set a starting point for the new programme (discuss this with your facilitator). Then it is very straightforward – any session of walking can be switched for a session of the chair based exercises (and you can switch 2 or all 3 sessions if you like).
5. If you have chest pains when exercising and your GP does not already know about this, you should stop exercising immediately. Contact your GP as soon as possible to discuss this and your facilitator.
6. If you have nausea, dizziness or palpitations (feeling your heart beating abnormally fast or irregularly) when exercising, stop immediately. If these symptoms don't go away, contact your GP as soon as possible.
7. If you have been prescribed a GTN spray, always carry it with you, especially during exercise.
8. If you have run out of your usual medications, or have forgotten to take them, do not exercise until this has been sorted out.

If you have angina and you experience chest pain during exercise, stop exercising immediately. Take 1-2 puffs of your GTN (glycerine trinitrate) spray if that has been prescribed and wait for the pain to go away. Take a second dose after 5 minutes if you still have pain. Dial 999 if your pain is still not relieved after using your GTN spray twice, or if you experience unbearable chest pain at any time. If the pain goes away and you feel OK, you can carry on, but wait a few minutes and work within the limits of your angina.

9. Talk to your facilitator or GP if you have any concerns.

Step 3: Keeping going

Our chair based exercises are a great way to build up your strength through the body weight exercises. Hopefully, after a few months of regular exercise, you will be feeling the benefits in terms of getting around more easily, sleeping better, improved mood and having fewer symptoms.

Once you have built up your basic level of activity, you need to think about keeping it at a good level – not just for a few months, but for the rest of your life! You can maintain your fitness by keeping going with your walking programme, your chair based exercises, or by doing other activities if you prefer. Depending on your own situation and the kind of activities that you enjoy, you may want to do other things, like joining an activity group, cycling or dancing.

The key question is: what activities can you do that you are likely to be able to keep doing on a regular basis? Keeping your basic fitness requires regular activity – if you stop doing activities your fitness will soon deteriorate. Have a look at the ideas for physical activity below and decide what you want to do to keep your fitness up. Most people find that it is easier to keep going with activities that they enjoy or that involve other people or that they can easily build into their usual routine (like using walking to get to work, taking the dog for a walk or joining a weekly activity group).

Remember that, to keep your fitness up you should be aiming for at least 60 to 150 minutes per week (depending on your starting level of fitness) of ‘moderate intensity’ physical activity each week. So make sure that the activities you choose include this amount of time *working at a level that makes you breathe heavier, feel warmer and have a slightly faster heartbeat, but still allows you to talk.*

Feel free to use the Exercise Record in the Progress Tracker to make a weekly goal and to track your progress and most of all enjoy the benefits that staying fit and active can bring – remember ‘Movement is Medicine’!

Ideas for physical activity

- Try a few different activities until you find something you can stick with
- Join an exercise group for people with heart failure or other heart problems – your facilitator should be able to tell you what is available in your local area’.

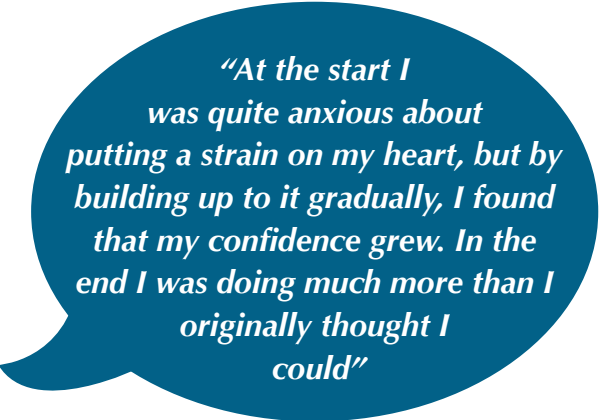
- Make walking part of your daily routine, such as how you get to work, or to the shops, or to make a regular visit to a friend or family.
- Dogs make great personal coaches as they always need walking! If you don’t have a dog, do you have a friend or neighbour or family member who has a dog that would like a bit of extra walking?
- Join a club or start a regular social activity (for example bowls or darts or a singing group) that you can *walk to and from*. Doing things with other people can be a good motivation!
- Do some other ‘moderate intensity’ activity like cycling, swimming or dancing.
- Make it fun or make it social – what would you enjoy most?

Swimming

Swimming or other water based exercise (e.g. aqua aerobics) is enjoyed by many people and can be done safely as part of your programme. However, in some people, swimming or other water based exercise can put a higher burden on the heart than other forms of exercise, particularly if you have not done it for a while. If you would like to do these types of exercises please discuss this with your REACH-HF facilitator, GP, or heart failure specialist first.

Problem-solving: Overcoming barriers to activity

Things don't always go according to plan – real life often gets in the way! If you find it hard to get as much exercise as you would like, it can be useful to think about what is making it difficult and how to overcome any barriers and what you can do to overcome them. Most people encounter some challenges when trying to change any aspect of their lifestyle – this is normal. However, the key to success is to figure out what the problem is and how to overcome it – it's all about learning from experience or 'trial and error'. If you keep trying and adapting your plan, eventually you will find the best way of staying fit that works for you and fits best with your situation. We have provided a list of ideas in the table below for overcoming common problems.



“At the start I was quite anxious about putting a strain on my heart, but by building up to it gradually, I found that my confidence grew. In the end I was doing much more than I originally thought I could”

The problem is...	Possible ideas...
“I can’t find the time”	If you are busy, try breaking the problem into smaller chunks. Try thinking of ways to build in a 10-minute block of activity into each day (or a number of days in the week). If you can do that, then try finding space for another 10-minute block. Planning ahead can be useful here – think about a typical day – how do you get from one place to another – perhaps walking could be part of your journey? Another possible solution is ‘multi-tasking’: How much time do you spend watching television? Do you need to be sitting down whilst doing this, or could you sometimes be doing some exercises at the same time? One good thing with the chair based exercises is that, once you have got the hang of them, you can do them without watching the videos. So you could do them whilst watching TV or listening to the radio.
“I am worried about going outdoors on my own”	It might help to go with a companion or to take a mobile phone with you. Make sure that somebody knows you are out walking and is available to call if you get into trouble. You can also choose a circuit for your walk, where there is always a bench or other place to sit close by. Choose a route where there are people around, or where it is easy to get home from. Build your outdoor time gradually – little by little.
“I have arthritis or other joint pain which stops me from exercising”	Joint pain can be a real problem, but research shows that doing more physical activity actually reduces joint pain in the long run. To get more activity despite having joint pain ... • Break it up into chunks – little and often. Build it up slowly. Once you get started the pain should decrease over time. • Try to find activities that don’t strain the affected joints or have low impact. If you struggle with walking, why not try our chair-based exercises as a starting point? A regular exercise programme is possible for most people, but you might need some more advice on how to get started. Your facilitator will be able to advise you further.

“My family thinks I should ‘take it easy’ and not exercise because it will strain my heart”	Explain to them how beneficial and safe regular exercise is for you and encourage them to join in with the exercise sessions – it will probably do them good too! See earlier, on the first page of this ‘Keeping Active’ section, for information about safety of exercising and how physical activity affects your heart.
“I am exhausted the next day”	It’s not uncommon to feel tired the day after an exercise session, especially if you haven’t exercised in a while. However, if you are exhausted you may be pushing yourself too hard/too soon. Remember that you need to build up slowly and progress gradually. It can be tricky to get the balance right, but it may be worth setting an easier target. Try to build up your fitness more slowly from a lower starting point. Your facilitator may be able to help you with this. Exhaustion may also be due to all the other things you have to do such as housework, gardening, shopping, hobbies, busy/stressful family life, work. Try to balance these activities with your exercise and plan how all these activities can be spread throughout the week.
“I have diabetes and it might affect my blood sugar”	Physical activity can reduce your blood sugar and regular exercise should be part of your treatment plan. However, you don’t want to lower your blood sugar too much or you may feel ill (this is often called ‘having a hypo’). To reduce the risk of a hypo 1) Follow a regular routine for eating, taking medicines and taking exercise. 2) Avoid working at more than a moderate intensity unless you are confident about how to manage this. 3) If you use insulin, you may need to adjust your dose to account for planned exercise. Talk to your doctor or facilitator about this. 4) Exercise with someone who knows what to do if you have a hypo. 5) Take a fast-acting carbohydrate snack with you to eat if you start to feel dizzy (e.g. glucose tablets). 6) Check your blood sugar before you go out and if it is low, have a carbohydrate snack. It may also be worth checking after your work-out. Remember: Blood sugar can be lower after exercise for a few hours. 7) If you do have a hypo, discuss with your doctor about how to avoid this happening.

“I don’t enjoy the exercise – it’s boring”	Would it be more enjoyable if you had some company? Why not invite a friend or partner or family member to join you. Could you vary where you do your activity? Maybe go for a walk in your local country or town park. Once you have built up your basic fitness, think about what alternative activities might be more interesting or enjoyable for you. You can replace your walking time or time spent doing the chair based exercises with any activity that is of ‘moderate intensity’ (as described above). Some exercises you can also do whilst watching your favourite TV programme (like our chair based exercises or walking on the spot). Background music or an audio book can also help to pass the time while you are walking (with headphones) or doing your home-based exercises. Even if you still find it a bit boring, weigh this up against all the benefits (at the start of this section). Maybe on balance it’s still worth doing?
“I have no energy to exercise”	Without doing some exercise you will have no energy – it’s a vicious cycle. If you start at the right level and progress slowly, your energy levels will increase over time as you become fitter. Try exercising at a time of the day when you feel you have more energy... for instance, some people find they are more exhausted in the late afternoon...so maybe exercising earlier in the day would be a better option? If you have a bad day with very low energy or through being ill, it is OK to miss a session – indeed you should aim to pace yourself and conserve energy on these days. If you are having mostly bad days in most weeks, then you should discuss this with your facilitator or your GP. If you are ill for several days, it may be best to rest or cut back on your exercise. When you feel better, gradually return to your exercise plan – you may need to set your target at a lower level and build back up from there. Also, make sure that you are taking your medicines appropriately and that your fluids are well-balanced (see the ‘Managing Your Fluid Balance’ section later on). Having fluid build-up or other heart failure symptoms can also drain your energy.

“I find it difficult to motivate myself”	It can sometimes be difficult to get going. Try thinking about the benefits of increasing your fitness – what would it mean for you if you had more energy overall, could sleep better and had fewer symptoms like breathlessness? This is what you can gain from increasing and maintaining your basic fitness. On the other hand, it might be cold outside or the activity might be boring or hard work. So, on balance, is it worth doing? Only you can answer this question. Other people (or kids, or animals) can also be good motivators, so is there anyone you can do the activities with or who you can talk with to help you get motivated?
“I’m too old to exercise”	You’re never too old to start exercising or to feel the benefits of exercising... you just need to start at the right level. Apart from the effect on your heart’s health, even a small increase in fitness can affect your mobility and your ability to do basic day-to-day tasks like getting out of a chair or doing the housework.
“I’m depressed and don’t feel like going out”	If you are feeling low, check the information about managing low mood in the ‘Keeping Your Spirits Up’ section later in the manual. Also, remember that exercise stimulates “feel-good” hormones. If you try a walk it will get you out of the house for some fresh air and a change of scenery and this can help lift your mood. If you really don’t want to go out, you can still do the chair based exercises in your home. If you can keep up some kind of physical activity, this can also help you to sleep better, which also improves your mood.
“I keep putting it off as other things in life are more important”	Your exercise sessions are very important to your health and quality of life. What will it take before you decide to make your own health a priority? Physical activity also helps you to sleep better so as time goes on, you should have more energy and be more mentally alert. Spending a bit of time on your fitness might help you to do more ‘other things’ rather than fewer.

“The weather is bad”

It's good to have a back-up plan for bad weather. How else could you get 10, 15 or 20 minutes of moderate intensity activity? For instance, this might include using the chair based exercises, going to a leisure centre or an exercise group.

If you want to be more active and are still stuck, you could discuss it with your facilitator. Friends or family members might also have some useful ideas.

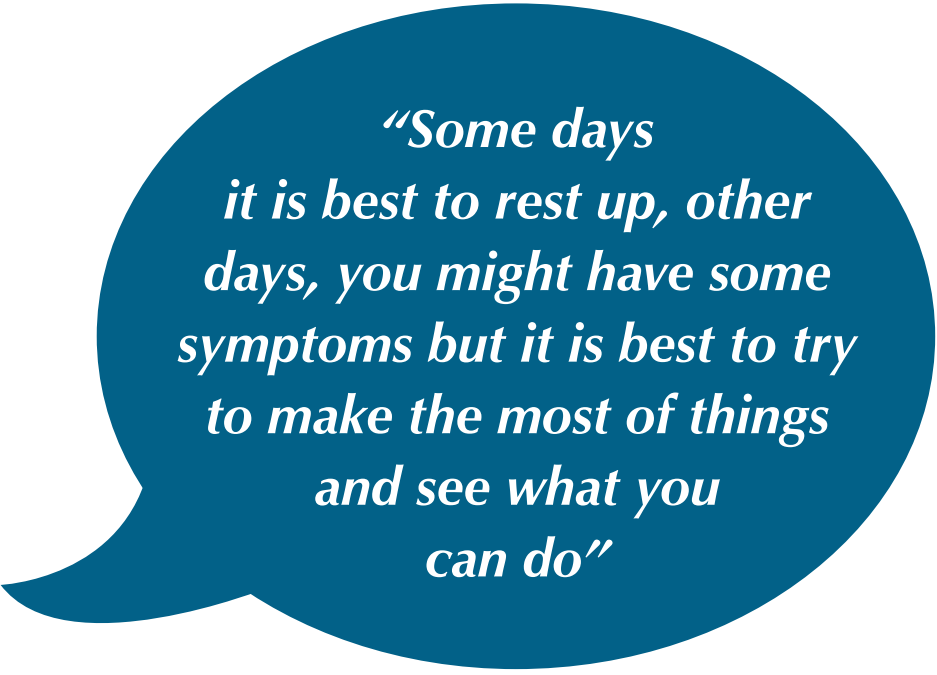
Setbacks and ‘re-starts’

Sometimes, if you have been ill or ‘out of action’ for a while, you may have lost some of your fitness and you may need to go back a few steps and work your way back up. This is normal and to be expected – your fitness will always increase or decrease, depending how much exercise you do.

If this happens, then it is absolutely fine to start the programme again from a level that is comfortable for you (you may wish to ask your doctor or facilitator if you have any concerns). This may be as little as 5 minutes per session to start with, but you should aim to build up your fitness over around 10 to 12 weeks.

If walking for 5 minutes is too difficult, then you could try the chair based exercises to start with – the lower levels of this programme are designed for people who cannot walk well.

If you have ‘good days’ and ‘bad days’ with your heart failure symptoms, don’t worry, this is normal – you have to be a bit flexible sometimes, but try to get your target amount of activity for the week by switching your activity days around. If you are having mostly bad days in most weeks, then you should discuss this with your facilitator, your GP or your heart failure nurse.



*“Some days
it is best to rest up, other
days, you might have some
symptoms but it is best to try
to make the most of things
and see what you
can do”*

Managing Your Fluid Balance

As we said in Part 1, when you have heart failure, this can cause fluids (water) and minerals (such as potassium, calcium and sodium) to build up in the body. This may be even more so the case if you have preserved ejection fraction. The build up of fluid in the body happens when the kidneys struggle to deal with having a reduced blood flow. The situation can be worsened by drinking too much fluid or forgetting to take your diuretic 'water tablets'. It can also be affected by how active or inactive you are.

If the fluid levels get too high this can be dangerous. Fluid can build up around the lungs, causing breathlessness and coughing and can put extra strain on the heart. On the other hand, being too dehydrated (having too little water) can also be dangerous. We require the correct amount of fluid and minerals to ensure the body works well – not too much and not too little.

When your fluids are building up too much, it is important to contact your GP, nurse, or facilitator for advice. You may need to:

- increase your diuretic medication – however, **make sure that your GP or heart failure specialist has approved of and is aware of any changes in your medication**
- restrict your fluid or salt intake – again you should only do this if advised to do so by a doctor or nurse
- in more severe cases you may need urgent hospital care – but if you take action early (as above), often this can be avoided

The key to keeping on top of the situation is to keep track of any changes in your fluid balance so that you can take early action. The following sections provide information on how to monitor for signs of fluid build-up, and what to do if this happens.

1. Checking for signs of fluids building up

There are various things you can do to check if your fluids are building up. These include checking for:

- a sizable increase in weight (+/- 2.3kg or 5lbs over 1 to 3 days)
- signs of swelling (for example in ankles or abdomen)
- an increase in shortness of breath or an increase in coughing or wheezing (such as becoming more breathless when lying down and requiring more pillows to relieve these symptoms)

Sudden changes in weight

Weighing yourself every day is a very easy way of monitoring your fluid balance. It is also the most accurate in telling you if your fluids are building up. Ideally, you should make it part of your daily routine, **even if there is no change over a long period of time – no change is a good thing!**

It is important that you weigh yourself in exactly the same way every day, to be able to detect a sizeable change in weight that is due to fluid rather than due to something you have eaten that day. Weighing yourself first thing

in the morning, before you have eaten, is the most accurate way of monitoring your weight to check for fluid retention. Even if you are unsure if your scales are completely accurate, using the same scales every day will still be able to guide you as to whether your weight has changed. If however you feel your scales are very unreliable, perhaps now would be a good time to invest in a new set. Remember you don't need anything fancy just a standard set of scales will do for most people. If you are concerned that you weigh much more than you should, have a chat with your facilitator, or GP, or your heart failure specialist for advice.

To help you get into a good routine we would encourage you to follow these 4 steps:

1. When you wake up in the morning go straight to the toilet and try to urinate to get rid of any removable fluid.
2. Stand on your scales and record your weight in the 'Checking for build up of fluid' section of your Progress Tracker.
3. Check against your weight over the past 1 to 3 days – how much has it gone up or down?
4. If your weight is going up, follow the advice in the 'Traffic Light Action Plan' at the back of your Progress Tracker.

Other tips - Weigh yourself in the same place, with the same type of clothing or without any clothes if you prefer, before you eat or drink, with the same scales, sticking to the same type of measurements either stones (st) and pounds (lbs) or kilograms (kg)

It is important to remember that you are not doing this to see how much you weigh, but to check if there are *sudden* changes in weight due to fluid build up.

What should I look out for? The general advice is to take action (with advice from your GP or nurse) if your weight increases by more than 5 pounds (2.3 kilograms) over a period of 1 to 3 days. It is important that you know the exact changes in your weight which require you to take action.

Swelling

Swelling is another sign that your body is retaining or holding on to fluid instead of getting rid of it when you go to the toilet to urinate. You may develop swollen feet if you sit for a long time for instance. Fluid tends to build up during the day, so it is most noticeable in the evening.

If you develop swelling and it doesn't improve with treatment, it is a sign that action needs to be taken. Checking for swelling is not as accurate as monitoring your weight every day, but can still be a useful method of monitoring your fluid balance. Recognising swelling is not always easy or accurate, but here are some tips.

Last thing in the evening when you are getting ready for bed, check for swelling of the feet, ankles, legs, abdomen and lower back. You can do this by gently pressing your feet and ankles, taking care not to damage your skin with your fingernails. Hold for 5 seconds and release. When you do this:

- is there an indentation or dip left in the skin?
- is it deeper than normal?
- does it take longer than normal to go away?

An alternative way of checking for swelling is to check whether your footwear feels tighter or whether your socks are leaving more of a mark than usual.

Remember to check both legs.

If you tend to develop swelling around your lower back, follow the same steps as above by gently pressing your lower back and noting down any changes. This may be awkward. Try not to strain yourself by twisting around. Get someone to check for you if you can.

Swelling of the abdomen can be difficult to see, so some people like to measure their waist. To be able to tell if your waist measurement is changing you need to be able to measure the same place each time. Your facilitator can also give advice as how to do this. Changes in how tight your clothes are (e.g., trousers, skirt, bra), or how they fit, can also be a good way to notice swelling.

Shortness of breath, cough and wheeze

Another way of checking for fluid build up is to be aware of changes in breathlessness, coughing or wheezing. Monitoring breathlessness is relatively simple, but it is important to specifically look for changes from your usual situation.

The first step is to be aware of what your “normal” is. Ask yourself “what situations do I tend to get breathless in?”

Do you normally get breathless when:

Yes

- ☐ You are exerting yourself more than normal?
- ☐ You are doing your normal activities?
- ☐ You are doing fewer than normal activities?
- ☐ You do any activity at all?
- ☐ You are resting?

The lower down you are on the list above, the worse your breathlessness is. You can recognise changes in your breathlessness when you are moving up or down on that list from where you would normally be. You can also keep track of how long it takes for your breathlessness to settle down when you stop doing an activity that makes you breathless. If this starts taking longer and longer, then your breathlessness is increasing.

Remember to also think about any changes in your sleeping pattern which may have been caused by your breathlessness. This may include the need to sit up or get out of bed during the night due to shortness of breath, or the need to prop yourself up with more pillows to help you sleep. So another way of monitoring breathlessness as a way of checking for fluid build up, would be to take note of changes in your sleeping pattern.

Finally, you could ask yourself: have I developed a new cough or wheeze, or is my cough or wheeze getting worse?

If your cough is 'productive', which means that you are coughing up phlegm (spit), you need to report this to your doctor or nurse if:

- the amount of phlegm is more than normal
- it has blood in it (red or pink), or

- if there is any sign of infection (a high temperature and/or yellow/green phlegm)

You can keep track of all the possible signs and symptoms of fluid build-up in the 'Checking for build up of fluid' section of your Progress Tracker. It can also help your care team to see whether your dose of diuretic medication needs to change.

Example:

	M	T	W	T	F	S	S
Weight today	70kg	71kg	73kg				
Weight gain	0	1	2				
2.3kg or 5lbs? over 1-3 days			✓				
More breathless							
More swelling			✓				

2. What to do if your fluids are building up

There are several ways to get your fluids back into balance, as we mentioned above. These can range from changing the dose of your diuretic medication to seeking hospital care.

Use the Traffic Light Action Plan to guide you

The 'Traffic Light Action Plan' at the back

of your Progress Tracker tells you how to recognise when to take action if your fluids might be building up, or if other important symptoms occur. Depending on your symptoms, you will be in either the 'Green Zone', the 'Amber Zone' or the 'Red Zone'. The actions you should take depend on which zone you are in:

- **Green Zone** – when you have a normal day (for you), this reminds you of your usual self-care activities

- **Amber Zone** – this describes the early actions which you should take when your signs and symptoms start to get worse

- **Red Zone** – this describes the signs and symptoms which require urgent action and what action you should take

Managing your fluid intake (if you have been advised to do so)

Sometimes, your nurse or GP or heart failure specialist may advise you not to consume more than a certain amount of fluid each day (usually 1.5 litres to 2 litres). **You should only do this IF a health professional has asked you to restrict your fluid intake.** You might also be asked to keep track of the amount of fluid you have over 1 or 2 days, just to check the amount you are taking in.

- Remember fluid is not only in all types of liquid you drink but also in the food that you eat. Foods high in fluid include: soup, ice cream, sorbet, gravy and jelly. Check the labels to find out the volume of fluid contained.
- Check the amount of fluid that can be held in your normal cups, mugs, glasses and bowls. Note this down in your fluid record.

ACTION

Discuss the ways you can monitor your fluid balance and the specific actions you should take if your fluids build up with your facilitator. Write the emergency telephone numbers for your local area on your Traffic Light Action Plan in case you need them in the future.

ACTION

Write down the amount you have been advised to limit your fluid intake to on the special 'Fluid Record' later in Part 3 of this manual. Use the Fluid Record to note down the amounts of all the fluids you consume in one day. At the end of the day, add the amounts together to give you a total.

- If you are worried about taking more fluid than you should fill a container with the amount of fluid you can have each day (such as 1.5 or 2 litres). Each time you have a cup of tea etc., pour out and dispose of the same amount of fluid from the container. When your container is empty you have reached your limit.
- If you have been asked to restrict your fluid intake, try to overcome thirst by sucking a boiled sweet or ice cube or chewing some chewing gum. Even swirling a small

amount of mouthwash, juice or water around your mouth can help reduce thirst.

Managing your salt intake

If you are struggling to manage your fluid balance, it might also be worth looking at the amount of salt you eat. This is because salt is directly linked to the amount of fluid you store. The more sodium in your body the more fluid it holds. There is information on how to manage the amount of salt you eat in the Healthy Eating section later in the manual.

Avoid getting dehydrated

It is also important to avoid taking too *little* fluid. This can cause dehydration particularly when you are: taking diuretics (water tablets) which make you urinate more, are in a hot environment, or if you have a stomach bug which causes vomiting or diarrhoea. Dehydration can cause tiredness and dizziness and can upset the mineral balance in the blood, all of which can make your symptoms feel worse. So, you should aim to keep a healthy balance – not too wet and not too dry!

“My weight increased over the last three days by 3 pounds, which put me in the yellow box on my Traffic Light Plan. I called my heart failure nurse – she advised that this may be because of eating too much salt and fluid, which made sense due to a bit of over-indulgence on crisps and such when my friends visited yesterday. She advised me to cut back on the salt and call again if my weight goes up again tomorrow”

Managing Stress

What is stress?

Stress is a natural reaction we experience to help us cope with challenging situations in daily life. In fact, we all need a certain amount of stress to enjoy life to the full.

However, stress can be a problem when we feel that life's demands are greater than our ability to cope with them. Sometimes we feel we cannot cope, either because we think these pressures are too great or that they never seem to end. Being diagnosed and living with a long-term condition like heart failure can be particularly stressful, especially if your symptoms are causing discomfort or your life has changed because of the condition. As a result, you may be feeling uncertainty, fear or worry. This is sometimes described as anxiety. Being anxious may, in turn, make symptoms like shortness of breath worse, making you worry even further.

The good news is that it is possible to manage or reduce stress in a number of ways. Many people have found that actively managing their stress levels also helps them to cope with the symptoms of heart failure. This manual and your facilitator will show you practical steps that you can take to become less stressed, despite the considerable challenges you may be facing.

When we are stressed or anxious our bodies produce adrenaline. Adrenaline produces a state of alert and gives us the extra drive to cope with challenges.



The stress thermometer

The stress thermometer shows how the rise in adrenaline affects the way we feel, from a state of deep relaxation to feeling excited. If the levels of adrenaline rise too high we may experience stress or anxiety.

What are the symptoms of stress and anxiety?

Fear is the main **emotional** symptom of anxiety. Stress and anxiety can also trigger **physical** symptoms including:

- dry mouth
- headache
- tense, sore muscles
- difficulty sleeping
- feeling tired
- indigestion
- 'butterflies' in the stomach

Anxiety can also affect your thoughts and attention, for example:

- loss of confidence
- lack of concentration
- difficulty making decisions
- racing thoughts
- difficulty with remembering
- Hostile or angry thoughts

Anxiety can also affect what you do, including:

- restlessness
- losing your temper
- increasing the amount of alcohol you drink
- increased the amount you smoke
- stopping you from meeting up with friends and family or even going outside
- being less physically active

When is stress or anxiety a problem?

A little stress or worry is not a problem. BUT if you find you are feeling stressed or anxious a lot of the time and it is getting in the way of your daily life then it may be worth taking action to reduce your stress levels.

Stress can also increase feelings of hostility and anger. This can lead to blaming others and strained relationships.



What can I do about it?

The stress management programme in the Heart Failure Manual is designed to help you recognise, reduce and manage your stress, anxiety, anger or frustration. You will be guided through this process by your facilitator.

How can I tell if I am stressed?

Fill in the checklist below to see how many signs of stress and anxiety you have. It is possible that you have been feeling like this for a while but have not recognised it.

Stress and anxiety checklist

Do these describe you at all?	
☐	Always rushing
☐	Impatient with others
☐	Keyed up
☐	Often losing your temper
☐	No time for chatting
☐	Critical of others
☐	Often changing your mind at last moment
☐	Feeling fidgety
☐	Can't make decisions easily
☐	Often tired
☐	Sigh a lot
☐	Quite forgetful
☐	Not often relaxed and peaceful
☐	Too busy for hobbies
☐	Often frowning
☐	Often worried
☐	Working till you are exhausted
☐	Life is not much fun any more
☐	Feeling frustrated

If you have ticked many of the boxes you may find the following pages on stress management helpful. The guidance and tools will be useful for yourself and also your partner and family.

Coping with stress and anxiety

You do not need to be stressed or anxious in order to benefit from the relaxation and stress-busting activities we suggest here. These are skills that everybody can benefit from, so feel free to share these with your friends and family. Not all activities suit everyone, but if you pick one or two and keep practising on a regular basis you should notice the benefits quite quickly. Most people find that using the relaxation techniques reduces their stress levels, helps them to sleep, gives them more mental energy to cope with any problems that arise and helps them to feel much more in control of things. Also, the more these activities become part of your daily life, the easier they become.

Relaxation techniques

The relaxation methods used in this Heart Failure Manual are a mixture of deep relaxation techniques which help you to let go of tension in the mind and body. It is not the same as sitting back to watch TV or having a cup of tea. You can learn these techniques by listening to and following the relaxation audio and practising the exercises regularly. Deep relaxation takes practice but you will feel the physical and mental benefits of this very quickly and these will continue if you keep it

**“I’m
glad I kept practising
when I look back at the
progress I have
made”**

up. Relaxation is an important part of stress management and can help you cope with problems and feel more in control of your condition. It has also been known to reduce pain, help with sleep and lower the heart rate (putting less strain on your heart). It helps to practise these techniques when there is no particular need for them, so you are ready to use them when you need them in future.

How long does it take?

To start with, we recommend that you practise by following one of the relaxation audio exercises for 20 minutes once or twice a day. The aim is to get you to a stage where you can use the skill at any time and get the same feeling of relaxation in a much shorter time, as and when you need it. Once you have learned how to do it, you will no longer need to use relaxation audio. It takes regular practice to develop this skill, but it is worth it!

Instructions for using the relaxation audio exercises

Make sure that no one will interrupt you.

Sit or lie down somewhere comfortable.

If you are using a chair it must be big enough to support your whole body when you are completely floppy.

The idea is to be able to let every last bit of tension out of your muscles. If you would not feel safe falling asleep in the chair, then it is not providing enough support. For instance, if you have to keep your arm balanced on the arm of the chair, you will still be using your

muscles to keep your arm there, and they won’t be completely relaxed. If this happens, maybe try resting your hands on your lap. It may help to put a thin pillow or cushion behind your neck for your head to rest on. The main thing is that you should be completely comfortable when relaxed. If you are not comfortable, try relaxing somewhere else – perhaps on the sofa or bed.

Points to remember:

- If your mind wanders, it doesn’t matter.
- Don’t worry or get annoyed if you feel you have missed a bit. Just start listening again and carry on.
- Remember – relaxation takes time to learn. It can’t be forced.
- You can’t make it happen by trying hard. Trying hard is the opposite of being relaxed.
- It doesn’t always work. How relaxed you feel after listening to the relaxation audio can depend on how you felt at the beginning.

But don’t give up. Remember that it has been proven that relaxation can help people manage their heart condition, including symptoms such as breathlessness and angina.

Ready to start?

Try using the relaxation audio exercises now.

To access the relaxation audio online go to www.heartmanual.scot.nhs.uk/heartfailure.

Your facilitator may give you a different website address.

Continuing relaxation

If you listened to the instructions on the relaxation audio you will have heard about two different ways of relaxation. Please go on listening to the audio once a day using whichever method you find best. If you like them both, listen to them both. Once you have got used to it, you can stop using the audio and just use the relaxation techniques whenever you need them.

The relaxation course also includes several other ways of relaxing that you may find useful. You may want to work through them in your own time.

Two reasons for learning relaxation:

1. To help you manage your condition by lowering your stress levels. If you keep listening to the relaxation audio twice a day this will happen automatically.
2. To control stress when stressful situations occur in real life.

The rapid relaxation exercises are some of the ways that you can do this. It is important to learn to do them without the audio. Then you will be able to use them whenever you want – at home, at work, on a bus or in the doctor's surgery. Wherever you are and whenever you feel tense, angry or frustrated you will be able to control your adrenaline level.

Try it now

- Can you relax your shoulders a little more?
- Let them slump down.
- Let the lines smooth out in your forehead, and relax your jaw.
- Can you relax your arms and your hands a little more?
- Let your hands spread open and relaxed.
- If you go through all your muscles as it describes on the relaxation audio you should find plenty of tension to get rid of.

Try to do this each time you look at your watch, or look out of the window.

You could add a small stick-on note on your fridge to remind you.

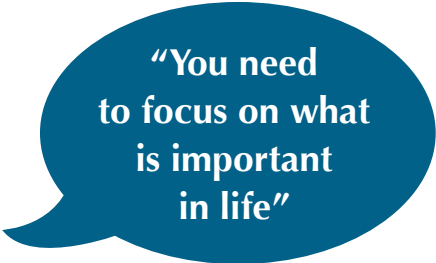
If you notice feelings of anger and frustration, do it then. Once you are relaxed, think:

- Is it really worth getting anxious, cross or upset about?
- If it is, then go ahead, but you will probably find that a lot of the time it isn't, and that it was just tension inside you that made it seem so bad, or annoying, or so important.

The other thing that is important is to avoid carrying muscle tension around with you. We usually don't notice this. It's only when you check your body that you notice the tension. If you check as often as possible during the day you will find that you become a more relaxed person.

Sometimes people who are learning relaxation find it difficult to get started. It can be hard to find the time or to concentrate. It is important not to give up. Discuss your feelings with your facilitator, who may be able to help.

Relaxation is good for everyone. It is good for both the body and the mind.



**"You need
to focus on what
is important
in life"**

Mindfulness – Focus on the 'here and now'

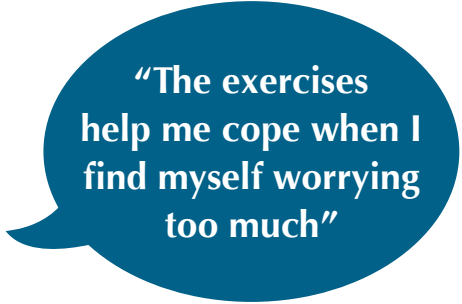
Mindfulness is another way of coping with stress, anxiety or anger and is the skill of being aware of and paying attention to the present moment. It is about being in tune with ourselves and others and our surroundings. When we pay attention to what is happening in the present moment we avoid constantly worrying about things that have happened in the past and what will happen in the future.

High quality research has shown that using mindfulness techniques can help people to relax and to overcome anxiety, depression and stress. It is therefore increasingly popular among people with long-term conditions like heart failure.

Mindfulness encourages us to accept ourselves as we are and to avoid judging ourselves and others. Similarly, it is also about accepting our experiences whether they are pleasant or not. This means resisting the urge to try and change things or to avoid thinking about problems. By being aware of negative emotions as they happen and how we react (often in unhelpful ways) we can learn to respond 'mindfully' instead. For example, if we have an unpleasant experience of pain, we may react with fear and anxiety and try to push the sensations away. Mindfulness encourages us to embrace whatever we are feeling and to know it is OK to feel the way we do. Often by doing this, the unpleasant experience passes more quickly and we find alternative ways of dealing with problems.

Mindfulness is an ancient skill based on meditation techniques that have been used for many hundreds of years, but it still seems highly relevant to the way we live now. We have all experienced the sense of being engaged with an activity that we really enjoy, sitting in the sun at a café, dancing, reading a good book, or watching a lovely sunset. This is being mindful or 'in the moment' – it is not an experience that is completely new to us. The skill is to avoid being distracted by other

thoughts about what happened yesterday and what will happen tomorrow which can spoil our enjoyment of the moment. Mindfulness is particularly useful in dealing with difficulties with health and daily living.



“The exercises help me cope when I find myself worrying too much”

Skills

Here are some mindfulness exercises. We suggest you try and practise one of these on a regular basis whether you are feeling calm or stressed. In addition, you could try the 3-minute breathing space exercise whenever you find yourself in a stressful situation.

Mindfulness approach to daily activities

Breathing space (3 minutes)

Back to the moment or *STOP* exercise (20 seconds)

Mindfulness approach to daily activities/Mindful eating

Mindful eating:

Eating is an activity we engage in a few times every day. But how often do you find yourself finishing a sandwich or a meal and not remembering what the food tasted like? Perhaps this was because you were caught up thinking/or worrying about something else or eating in a hurry. This exercise is an opportunity to increase our enjoyment of the food we eat. It can be very useful if we have a tendency to overeat – if you get more enjoyment out of less food, this can help to reduce the amount of food you feel that you need.

When eating just eat. When you are eating or drinking don't do anything else. Look at the colours, shapes, surface textures. Notice the smells and flavours in your mouth. Listen to the sounds of eating and drinking. Enjoy each bit, enjoy each moment.

If you like the sound of it, try this mindful eating exercise each day for a week and see how it feels.

"I find the short exercises simple but helpful. They do take a bit of practice though"

The 3-Minute Breathing Space

This is an exercise that we suggest you practise on a regular basis. You can use this technique whenever you feel you are stressed or running on automatic pilot. Think of it like a reset button or opportunity to step off the conveyor belt for a few minutes.

At various times of the day, see if it is possible to step out of automatic pilot for about 3 minutes. It can help if you set a reminder to do this.

The 3-Minute Breathing Space

1. What's here? (becoming aware)

Notice your posture. Straighten your spine and generally relax your body.

With your eyes open or shut, silently ask yourself:

What is my experience right now. ...in my thoughts ...and my bodily sensations

Allow and accept your experience even if it is unwanted.

2. Breathing

Gently guide your full attention to each 'in' breath and to each 'out' breath as they follow one after the other. Try noticing at the back of your mind Breathing in...breathing out or counting the breaths.

Do this for one or two minutes, using the breathing as a point of focus to bring you

into the present and help you tune in to a state of awareness and stillness.

3. Extend your awareness

Extend the focus on your breathing to your body as a whole, noticing your posture and your facial expression.

Recognise any sense of discomfort, tension or resistance. If these sensations are there, then guide your awareness to them by 'breathing into them' on the in breath. Then breathe out those sensations, letting them go with the out breath. If you can say to yourself on the out breath "It's ok whatever it is, it's ok". Let the sensations come and go. Don't try to hold on to them.

See if you can keep this sense of awareness throughout the day.

STOP

Use this short exercise to remind yourself to be aware of the present moment, bringing your mind back to the present without judgement or worry.



S = Stop

T = Take a breath

O = Observe

P = Proceed

Whilst you observe just try and be aware of your posture: how you may be standing, sitting or lying down. Be aware of the sensations of the body, what you see, hear, taste smell or touch. Practise a half smile while trying to relax your facial muscles. Then carry on with whatever you were doing.

You can do this at any time of the day. Try and find cues to do this: making a cup of tea

or coffee, standing in a queue, when you first wake up, whenever you feel angry or annoyed, when you lie down to go to sleep.

Managing demands and increasing the things you enjoy

Often, we find ourselves in a situation when the demands on us are too frequent or too great. We don't know where to start but this still doesn't stop us from constantly worrying about them. You may not be able to get rid of the demands, but there may be ways to help you reduce them and feel more in control and less stressed. Here are some suggestions and practical exercises. These come from research evidence and also directly from the reports of people with heart failure who have found these activities have worked for them.

"I set myself a reminder, it's amazing how I got so caught up with worrying about what has happened, or what might happen – the exercises get me back to enjoying the here and now"

Skills

Setting aside time to worry

It often helps to write down your worries and set aside a 'worry time' so that you are not constantly thinking about them. This is important because you may not be in a position to do anything about them

immediately. So you may as well not keep on thinking about them. For example, when you are in bed at night and worrying keeps you awake at night, write your worries down and tell yourself that you will look at the list and deal with them in the morning. This way you have made a decision and you may find that you will get to sleep easier.

Scheduling jobs/chores

Try this technique. Get a piece of paper and write three headings across the page.

Job	Deadline	Importance
A single word is usually enough.	This should be the absolute latest date by which the job must be done.	Score this out of 10 where 10 = most important and 1 = not all important.
Just as long as you know what it means.	If there isn't time, set one that you can manage easily.	If you can't find the time for all the things on your list, you should decide which ones to get rid of.

A common reason for having too much to do is taking on other people's problems and trying to solve them. Another reason is not being able to say 'no' to people or to 'work' (see Saying No later in this section).

You can check to see if you fall into this trap by using scheduling. Look at each deadline and write an 'M' for me beside each one that you set or an 'S' (someone else) if it was not you that set the deadline.

In this way you can see if you are setting yourself a lot of unachievable deadlines and if you are taking on work that you don't actually have to do.

For each 'M' deadline ask yourself 'Who got me into this', 'Should someone else be doing this?' and 'Am I trying to do the impossible?'

Job	Deadline	Importance
Buy birthday present for my wife	Next week	8 M
Fix car	As soon as possible	10 M
Tax return	Next month	10 S
Council tax rebate	Next month	10 S
Paint bedroom	As soon as possible	5 M
Weed garden	Couple of weeks	2 M

Scheduling is a way of gaining control when you are feeling under pressure from demands at home or at work.

- It helps us get an overall picture of what we have to do
- It helps us decide what needs to be done and what can be left until later
- It helps us relax when we feel more in control and know what we have to do

Saying NO

How good are you at saying NO? Some of these ideas might be helpful ...

‘No, I’d like to help but I just don’t have the time.’

‘No, I don’t know what you can do about that.’

‘No, I can’t come round just now.’

‘No, I’m going fishing ... for a walk ... to the pictures.’

‘No, I don’t work on Saturdays.’

Saying **NO** seems to be very hard for some people. They feel that they should always help other people out or be prepared to take something on whenever they get the chance. This is fine so long as it doesn’t lead to having too much to do and feeling under pressure all the time. **We have a duty to look after ourselves as well as other people.**

The best way to overcome the problem is simply to practise saying NO.

To start with you may find this very hard. You may feel quite guilty about not helping the person or not doing the job, but it is important for you to learn how to do this. If you refuse politely and explain that you are too busy, most people will accept it. If it is someone who you have always said yes to in the past,

they may be surprised and take some time to get used to it, but they will.

Set aside times when you do not work or will not be available (for example, “I have decided not to do any work after nine o’clock in the evening” or “Saturday morning is my book-reading time”). Stick to these protected times – however much piles up and whoever tries to persuade you against it. There are only so many hours in the day!

Choose other ways to relax that you may enjoy:

- playing with grandchildren
- playing a musical instrument or listening to music
- painting
- yoga
- t’ai Chi
- going for a walk

Often it is the enjoyable activities that we put off, but these are equally important, if not more important than the other activities we may have planned. Try and make a conscious decision about making space for some enjoyable activities in your life. Plan dates in your diary to do them – ideally plan at least one enjoyable thing each day.

Keeping up the UP things

One reason for stress can be that we have lost the balance between pleasure and work, or between pleasure and doing all our day-to-day chores.

Here is an exercise which can help to put this right. To do it you need a pencil and paper. Write all the good things in your life down one side of the sheet of paper – the **UP things**. Then write the worrying, unpleasant, adrenaline-raising things down the other side – **the DOWN things**.

Here are four ways to use your list to reduce stress:

- 1 Increase the amount of time that you spend doing the UP things.
- 2 Add new things to the UP side.
- 3 Reduce the number of things on the DOWN side.
- 4 Plan UP things for the future.

UP Things Enjoyable and relaxing activities	DOWN side Unpleasant, stressful activities
Painting Taking grandchildren out Relaxation Chatting with wife TV Seeing old friends Walking dog	Signing on Fixing car Money Arguing with the kids Gardening

This is how Jim McClure used the UP exercise.

- 1 He increased the amount of time he spent on UP things. He decided to take his grandchildren out once a week instead of just occasionally.
- 2 He added new UP things. He took up fishing again. He started tying fishing flies.
- 3 He reduced the number of things on the DOWN side. Jim hated looking after his car. He didn't really need it much now, so he sold it and immediately felt less stressed. He also hated gardening. He decided with his wife that they would put the back garden down to grass and pave over the front.
- 4 He planned UP things for the future. He suggested saving for a holiday and his wife was amazed, as he'd always been 'too busy' to take proper holidays. They got some brochures, looked online and had a lot of fun choosing a holiday. Just thinking about it was enough to make Jim feel more relaxed.

Why not try this for yourself?

If you have any questions about these exercises and activities, please discuss these with your facilitator.

Use the 'Managing stress and anxiety' section on your Progress Tracker to note down what you do to manage your stress or anxiety and whether this works for you or not. Try out different strategies and techniques until you find out what works best for you.

Skills for managing acute stress or sudden anxiety

So far the exercises and activities we have

described are ones that we practise to prevent or stop stress or anxiety building up. There may be times however that you experience sudden waves of stress or anxiety that can be very unpleasant. If you have been practising the exercises you will find that you can use these at such times. However there are some further ideas below that you can use to help manage any sudden bouts of anxiety or stress.

As soon as you find yourself becoming anxious think about the statements below. These are called the 'Five Rules for Coping with Anxiety'.

Example

What did you do to help manage stress this week?	How many times did you do it?	How did it make you feel?
1. Stop exercise	Once every other day	Felt good and could fit into my lunch break easily

Five rules for coping with anxiety

1. The feelings I have are the effects of adrenaline
2. These feelings are not dangerous – just unpleasant
3. Notice what is happening in my body right now – not what I fear may happen
4. Remember that when I stop adding to my anxiety with worrying thoughts, it starts to fade by itself

5. Wait, and give the fear time to pass. Do not fight it or run away from it. Just accept it.

Doing the rapid relaxation techniques or mindfulness exercises that work for you will help the feelings to go more quickly.

It is important to practise the relaxation or mindfulness exercises on a regular basis so that if you are affected by an episode of acute stress or anxiety you are ready to cope with it.

Panic attacks

Sometimes when we are stressed or worried we get a build up of adrenaline in our body that leads to feelings of panic. A panic attack can happen when we least expect it and can be triggered by a worrying thought that tips our adrenaline into panic levels [see the stress thermometer at the start of this 'Managing Stress' section]. In a panic attack your thoughts race, your breathing and heart rate increase (these are also effects of the adrenaline).

You may feel butterflies in your stomach and feel cold and clammy. Panic attacks can be frightening but they are not dangerous and are usually over within 10 or 30 minutes once the body has run out of adrenaline. Try to think of the five rules for coping, these will help.

Speak to your facilitator or GP if you need further help in dealing with stress, anxiety or panic attacks. This is an important part of your overall well-being.

Managing Breathlessness

In the 'Understanding Heart Failure' section we said that breathlessness is a common symptom of heart failure that is caused by a build up of fluid in the lungs. You may become more out of breath when doing physical activities such as walking or climbing stairs for instance. Some people also find that lying flat brings on breathlessness.

This section will help you manage and minimise the physical discomfort of breathlessness and give you advice on how to avoid feeling worse through worry or being anxious about breathing difficulties. However, if your symptoms cause you to black out, become dizzy, or become confused, it is essential that you contact the emergency services by telephoning 999.

If you have heart failure you may become breathless at any time, but there are times when this may be more common.

Ask yourself "Looking back, when am I most likely to have breathing difficulties?"

- When I am physically active?
- When I am stressed or anxious?
- When my heart failure symptoms are worse than usual?

Breathlessness during or after physical activity

Becoming slightly short of breath during physical activity and exercise is normal. Indeed, this is important in order to build your fitness (See the 'Keeping Active' section

earlier in the manual). Even athletes get breathlessness. BUT, being so breathless that you cannot catch your breath or speak during or after exercise can be uncomfortable and frightening. Although it may seem strange to do something that makes you have symptoms, building up your fitness will reduce breathlessness in the long run – so please keep going with your exercises!

- The exercises recommended in the Physical Activity section of this manual are designed for you to build your fitness gradually and safely, with a minimum of discomfort.
- Make sure you take regular rests if you find yourself getting breathless during everyday activity like walking or housework.
- Avoid rushing. Take things steadily and calmly.
- Try and slow down before you totally lose your breath. This way your breathing will return to normal quicker.
- Try to plan your activity and if necessary do smaller amounts and take short rests in between. 'Little and often' sometimes works better than long stretches of physical activity followed by long rests (and this is also a good strategy if you have joint pain that makes it difficult to exercise).
- Think about how you are breathing when you exercise. Resist the temptation to hold your breath as this will make you more short of breath. Breathe as normally as you can.

Breathlessness due to stress or 'hyperventilation'

If you are anxious, stressed, uncomfortable or in pain, this can affect your breathing. For example you may be taking short, shallow, frequent breaths and breathing from the chest rather than the stomach. You may also find yourself sighing a lot or holding your breath. All these things can lead to 'hyperventilation' – this is when you breathe very fast and find it hard to control your rate of breathing.

Symptoms of hyperventilation include:

- feeling nervous, fidgety or frustrated
- feelings of light-headedness
- tingling or numbness in the mouth
- feeling unable to catch air
- wobbly legs
- a feeling that you want to escape from something

Hyperventilation can also lead to a 'panic attack' (as described in the 'Managing Stress' section), which although not dangerous can be frightening and unpleasant.

What can I do about it?

It is important to try and maintain or get back to a normal pattern of breathing. The relaxation techniques that we have provided in the 'Managing Stress' section and on the relaxation audio can help with this. When you breathe, your stomach not your chest should rise and fall, and it is worth checking this quickly ...

Sit down and get comfortable in your chair. Put one hand on your chest and the other on your stomach. Shut your eyes and notice what is happening when you breathe in and out. If your hand on chest is rising and falling and your hand on your stomach is not moving this is wrong. The idea is to breathe with the stomach rising and falling, not the chest.

Keep practising the breathing and relaxation exercises. If you are unsure of the correct way of breathing, ask your facilitator to check how you are doing. It may feel quite unnatural at first. Some people have fallen into a habit of hyperventilating over the years so it may take some practice to correct this.

What else can I do?

Because breathing difficulties may be a symptom of stress and anxiety, taking steps to reduce your stress levels may be helpful. Please look at the 'Managing Stress' section earlier in this manual. This is designed to help you learn how to cope with stress and anxiety. If you practise the exercises regularly, this should prevent a lot of stress and anxiety from happening. The exercises also prepare you to deal with any sudden bouts of breathing difficulties or anxiety that may develop.

Breathlessness due to a flare-up in symptoms

Being more breathless than usual can be an important symptom requiring a change in your medication. Your fluid management may need to be reviewed. Use your Traffic Light Action

Plan at the back of your Progress Tracker to decide what kind of action you need to take.

It is also worth doing the following:

- check your breathing patterns (stomach or chest?)
- practise your breathing exercises from the relaxation audio and the exercise videos

Whatever is causing your breathlessness, the breathing and relaxation techniques in the 'Managing Stress' section earlier on in this manual, will help you manage this on a day-to-day basis. Please read this section if you haven't already done so.

Managing Changes in Your Symptoms: Ups and Downs

People with heart failure often manage very well for long periods of time by sticking to the right treatment and making some lifestyle changes. However, occasionally you may experience worsening of your signs and symptoms. This may be caused by a build-up of fluid, eating or drinking foods which are high in salt, or not taking medications correctly. It can also be caused by not sleeping well, or getting other illnesses such as colds or stomach bugs. You may also experience some natural variation in symptoms.

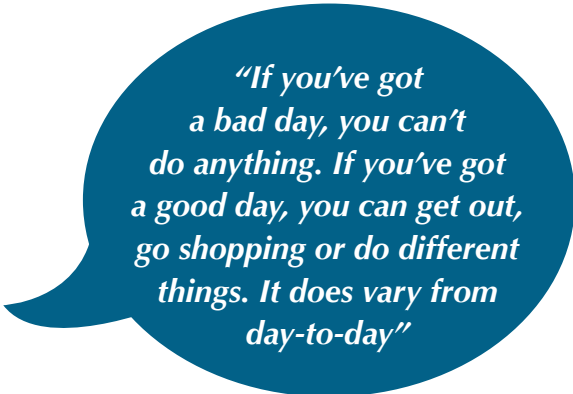
Sometimes the reason for increasing symptoms is unclear. What is clear, however, is that self care plays an important role in spotting when there is a problem and taking action to control these changes and prevent any further problems.

Having a 'Plan B' for bad days

If you find that your symptoms are changing, the Traffic Light Action Plan in your Progress Tracker gives advice on what you should do. If you are in the Green Zone, you are having a good day and that is fine. If you are in the 'Amber Zone' you may not have as much energy as usual, which may get in the way of you being able to get on with your daily activities.

Having a Plan B ready can help you make the most of these days while preventing any further problems. A Plan B should consist of things that try to conserve your energy and stop you getting over-tired. This might involve:

- taking small loads of washing to the line, rather than doing it all at once
- resting after activities and stop if you feel tired – you can come back to it later
- using a wheeled trolley for shopping rather than carrying bags
- don't plan to do harder activities right after a meal – a lot of energy goes into digesting your food, so you may have less energy after a meal
- using a high stool to sit at while doing the washing up, instead of standing
- getting other people to help you
- if you live alone, then you may be able to get help from Social Services – ask your facilitator or GP about this if you don't know how
- it is good to keep active if you can, but don't overdo it. If your symptoms are worse than usual then you may need to do your exercise at a lower level, or even have a rest day.



"If you've got a bad day, you can't do anything. If you've got a good day, you can get out, go shopping or do different things. It does vary from day-to-day"

If your daily activities are too tiring, discuss this with your facilitator or GP.

It is also important to avoid overdoing it in general – if you work too hard on one day you can be exhausted for one or two days afterwards. So avoid extreme activities and spread your exercise and other activities that require more energy through the week. It is very useful to know your limits so you don't try to do too much in a day. 'Little and often' sometimes works better than trying to get all your activity done in one go.

Figuring out what causes changes and how to manage them

Because heart failure can be quite unpredictable, it is important to keep an eye on how your signs and symptoms change over time and to try and figure out what causes these changes (i.e., what makes you feel better or worse). If you do this you may be able to avoid more serious problems which require attendance at a hospital. For instance, you might be able to improve things by getting a better night's sleep, by building up your fitness or by working with your health care team to adjust your medications.

A Progress Tracker has been provided as a separate booklet to use alongside this manual. This allows you to record how you have been feeling from week to week over a 12-week period. Using this tool may help you to learn what things seem to make your symptoms better or worse. An example of how you might use this is given on the next page...

Give it a go and see if keeping track of your progress from week to week gives you any useful ideas about how best to manage your symptoms and how to keep your spirits up.

Summary

There are many things which doctors and nurses can do to support you if you have heart failure. However, the most important person who manages the condition is **you**. There are lots of simple things which you can do to help you to lead a happy and fulfilling life and prevent the need for further treatment or hospitalisation.

This includes;

- taking your medicines as they have been prescribed
- monitoring your fluid balance
- keeping active
- managing stress and worry
- taking action if your symptoms change
- communicating well with your health care team

The Progress Tracker will help you to make these 'self-care' tasks part of your daily routine. It has been designed to help you: get the most from your medications, monitor any signs and symptoms you may have, and build up your fitness in a safe and manageable way. It will also help you to learn what things help to improve your symptoms and help you to identify any concerns that you may want to discuss with your health care team.

Example

	How have you been?	What have you done that made you feel better or worse than previous weeks?
Physically		Increased walking time and spread my shopping and washing load through the week
Mentally		Scheduled my worry time
Sleep quality		Drank too much water before going to bed

Your health care team need to know if you develop any new symptoms or if your current signs and symptoms are getting worse. Knowing what is happening to you is important, this allows your doctors or nurses to take early action to help put things right.

Finally, other people around you can often be a big help. It is worth considering what other people you live with, or friends or family who live elsewhere can do to help with any of your self-care plans. This might include helping to

monitor your signs and symptoms, helping to organise your medications, doing physical activity or exercises with you, or helping to remember things. Joining groups can also be very helpful as well as providing a bit of social activity.

Your facilitator should be able to tell you if there are any heart failure support groups or other heart disease support groups in your area.

Managing Heart Failure Quiz

*Please tick what you think is the right answer
(only tick 1 box per question)*

1. How often should people with heart failure weigh themselves?

- ☐ Once per week
- ☐ Now and then
- ☐ Every day

2. Why is it important that people with heart failure should weigh themselves regularly?

- ☐ because being overweight is unhealthy
- ☐ to check whether the body is retaining fluid
- ☐ to assess the right dose of medicines

3. Which of these statements is true?

- ☐ when I cough a lot, it is better not to take my heart failure medication
- ☐ when I am feeling better, I can stop taking my medication for heart failure.
- ☐ it is important that I take my heart failure medication regularly

4. What is the best thing to do in case of increased shortness of breath or swollen legs?

- ☐ call the doctor or the nurse
- ☐ wait until the next check-up
- ☐ take less medication

5. What can cause a rapid worsening of heart failure symptoms?

- ☐ a high-fat diet
- ☐ a cold or the flu
- ☐ lack of exercise

6. What does heart failure mean?

- ☐ that the heart is unable to pump enough blood around the body
- ☐ that someone is not getting enough exercise and is in poor condition
- ☐ that there is a blood clot in the blood vessels of the heart

7. Why can the legs swell up when you have heart failure?

- ☐ because the valves in the blood vessels in the legs do not function properly
- ☐ because the muscles in the legs are not getting enough oxygen
- ☐ because of accumulation of fluid in the legs

8. What is the function of the heart?

- ☐ to absorb nutrients from the blood
- ☐ to pump blood around the body
- ☐ to provide the blood with oxygen

9. Which statement about exercise for people with heart failure is true?

- ☐ it is important to exercise as little as possible in order to relieve the heart
- ☐ it is important to exercise and to rest regularly in between
- ☐ it is important to exercise as much as possible

10. Why are water pills prescribed to someone with heart failure?

- ☐ to lower the blood pressure
- ☐ to prevent fluid retention in the body
- ☐ because then they can drink more

11. Which statement about weight increase and heart failure is true?

- ☐ an increase of over 2.3kg (5lbs) in 2 or 3 days should be reported to the doctor at the next check-up
- ☐ in case of an increase of over 2.3kg (5lbs) in 2 or 3 days, you should contact your doctor or nurse
- ☐ in case of an increase of over 2.3kg (5lbs) in 2 or 3 days, you should eat less

Heart Failure Quiz: The correct answers

If you are unsure about any of these answers please talk to your facilitator.

- 1. How often should people with heart failure weigh themselves?**
☒ Every day
- 2. Why is it important that people with heart failure should weigh themselves regularly?**
☒ to check whether the body is retaining fluid
- 3. Which of these statements is true?**
☒ it is important that I take my heart failure medication regularly
- 4. What is the best thing to do in case of increased shortness of breath or swollen legs?**
☒ call the doctor or the nurse
- 5. What can cause a rapid worsening of heart failure symptoms?**
☒ a cold or the flu
- 6. What does heart failure mean?**
☒ that the heart is unable to pump enough blood around the body
- 7. Why can the legs swell up when you have heart failure?**
☒ because of accumulation of fluid in the legs
- 8. What is the function of the heart?**
☒ to pump blood around the body
- 9. Which statement about exercise for people with heart failure is true?**
☒ it is important to exercise at home and to rest regularly in between
- 10. Why are water pills prescribed to someone with heart failure?**
☒ to prevent fluid retention in the body
- 11. Which statement about weight increase and heart failure is true?**
☒ in case of an increase of over 2 kilograms in 2 or 3 days, you should contact your doctor or nurse

Adapted from: DUTCH HEART FAILURE KNOWLEDGE SCALE. M.H.L. van der Wal, T. Jaarsma. June 2003

Further Information

This section provides more information and ideas on topics which may or may not be relevant for you. Nevertheless, many of these topics can have a substantial effect on the quality of life of many people with heart failure.

Keeping Your Spirits Up

Having any long-term illness can have a big impact on your life – you may not be able to do all the things you could do before – you may feel unwell or tired a lot of the time – you may miss your old life and the way things used to be. This can lead to people with heart failure feeling low or depressed, particularly in the early stages – it can feel like your whole life has been turned upside down.

But it's not all doom and gloom. These feelings often change as time passes. It's all about adapting to your circumstances. To adapt successfully you need to find ways to make the most out of your life *as it is now*. This section provides some tips and ideas about how to check your mood and what to do if you are feeling down or depressed about your situation.

Is there a problem?

Feeling low down or blue from time to time is a normal part of life and usually this is nothing to worry about. However, if you are feeling low or depressed, or angry and frustrated a lot of the time, then it might be worth seeing if there is something you can do to improve your situation.

It is worth saying at this point that most people with heart failure are not depressed and many find ways to enjoy their lives despite any limitations that having heart failure puts on them. So there is hope!

The first step is to figure out how well you are doing. The common symptoms of depression are:

- feeling low or depressed, sad or empty
- feeling irritable or angry
- loss of interest and enjoyment in things you would normally enjoy
- change in appetite
- difficulty sleeping or sleep being more disturbed than usual
- mood swings or bouts of crying

“Your heart may not recover its full capacity, but YOU can recover”

- decreased activity – withdrawing from usual activities
- feelings of worthlessness or guilt
- reduced concentration – finding it hard to think straight or talk
- low self esteem and confidence
- pessimistic thoughts – for instance feeling that everything is pointless

If you feel low or depressed from time to time, then it may be worth reading the next few pages and trying out some of the ideas.

If most of the things above apply to you and this has been the case for two weeks or more, then you should mention this to your heart failure nurse or your GP, as there are treatments that could help.

If you feel that these symptoms are very strong to the point that they make your life unbearable or if you have got to the point where you often think about ending your life, then you probably have a serious bout of depression. If this is the case, please contact your GP or heart failure nurse straight away. There are both medicines and therapies that can make a huge difference for people with depression – indeed most people make a full recovery. OR, if you are not comfortable contacting the NHS, you can phone the Samaritans.

What can I do about it?

First of all, it is useful to understand the problem a bit better. This may help you to see ways around it.

How it works – low mood leads to low activity

As we said earlier, feeling low or depressed occasionally is normal and particularly if you have been seriously ill recently. For example if you have been in hospital recently with heart failure, the whole experience can be exhausting and can knock you sideways both physically and mentally.

A natural reaction to feeling ill is to withdraw from activities that you would normally do, such as meeting friends and family, doing things you enjoy and getting out and about in general. This gives you the space and time you need to recover and is fine. Up to a point. The problem is that withdrawing from things that you do also tends to affect your mood. It makes you feel more sluggish or tired, you are more likely to feel isolated and your mood generally sinks lower. The more you feel low or depressed, the more you withdraw from doing things that you might enjoy or that you find rewarding. Then of course your mood becomes even lower – it's a downward spiral. The picture below shows how this works.

"It is important to maintain a positive mental attitude; You may need to make compromises, but you can still have a good life"



Some people who are depressed say they feel like they are “looking up from the bottom of a black hole” and they can’t see how it is possible to get out.

A very important thing about the above diagram is the way that your feelings and actions can reinforce each other. It can become an unhealthy cycle, or a ‘downward spiral’ that is hard to break out of. These negative effects can spill over into other areas of your life. For example, giving up on physical activities not only affects your mood and how well you sleep, but your physical health as well.

The good news is that the ‘mood spiral’ works both ways – the way out of the black hole is to put the spiral into reverse! If you start doing more activities your mood starts to lift. Then you have more energy and are able to do more, which lifts your mood further. You don’t have to do this all at once of course – most people find that a step-by-step approach works best.

It can take a lot of willpower because you may not feel like doing things. However, once you start, you will often find that you enjoy them more than you thought you would. It will be much easier to stick at it if you make a plan.

Activity planning

Every day should contain some enjoyable activity

- Make a list of the days of the week and make sure there is some activity for each day – try to think of something that you will enjoy.
- For example, this might include going out, seeing friends or family, reading a book, listening to music, buying yourself a small gift or getting a bit of exercise.
- You may find that you don’t enjoy these activities as much as you would normally – Sometimes it takes a few goes before you start enjoying things again, so it is worth sticking at it. Also, you will probably feel

**“if you are
angry – go and punch
pillows/scream...
let it out”**

better than you would if you just sat around feeling miserable.

- If you can make these things into a routine, even better.

MON	Go to library
TUE	Buy new shirt
WED	Visit Bob Smith
THUR	Walk round park
FRI	Meet friends for lunch
SAT	Go shopping
SUN	Walk round harbour

You can also use the ‘Weekly Progress’ section in your Progress Tracker to keep track of your mood and see how it changes on days when you do more things, this might help you to see how doing a bit more each week can help to lift your mood.

An added bonus: physical activity

If you decide to do some **physical** activities, such as walking or gardening, these have extra benefits. When you do physical activity, it increases your energy levels and improves the quality of your sleep. Physical activity reduces stress and muscle tension and it can provide opportunities to interact more with other people, which many people enjoy. Physical activity also releases ‘feel-good’ chemicals in the brain. These can help to improve your mood, especially if you also enjoy the activity

itself. It can also provide a distraction from negative thoughts. Physical activity can also have an important effect on your heart failure symptoms and keep your muscles fit so that you do more in life.

How it works – Persistent negative thoughts

One of the most unpleasant things about depression is what it does to the way you think. People with depression often only see the bad side of things. They often report having very negative thoughts about themselves or their relationship with other people – thoughts like “I’m no good any more”, “I am always letting people down” or “My life is as good as over”. Negative thoughts can be images and memories as well as words. You may also tend to jump to negative conclusions about what other people do or say.

If you tend to look at things negatively (especially yourself, your future and the world around you), over time this can affect your mood. The more you think like this, the more likely you are to withdraw from a lot of activities that you would normally do. This might include meeting friends and family, doing activities that you enjoy and getting out and about in general. Then you will have even more negative thoughts and your mood becomes even lower – and we are back on the downward spiral (as in the diagram above). Over time these thoughts can become very persistent and often come to mind unexpectedly – they can become ‘automatic’.

So, how can you break the downward spiral and find your way back to a normal way of thinking and feeling? There are four simple steps which might be worth trying here:

1. Spotting the signs

The first step is to accept and acknowledge the thoughts you are having. Try to recognise when you have negative thoughts. What are the thoughts? It can be useful to make a note of these so you can think them through properly.

What situations trigger your negative thoughts? It may be that you can avoid these situations or prepare yourself for them in the future.

Negative thoughts often involve one of the following:-

- **Imagining the worst** (catastrophic thinking). For example, "I'll never be able to enjoy life again", "my life is over".
- **Exaggerating the negative** aspect of any slight setback. For example, if someone you know passes you in the street and doesn't seem to notice you, you might think "he doesn't want to know me anymore".
- **Feeling guilty**. YOU may blame yourself for all the bad things in your life – for example, "all this is my fault"; "I am just a burden on my family".

MIKA'S STORY:

"I think ACCEPTANCE is the first medicine of depression. It drove me batty when everyone kept telling me to be positive all the time ... when you're exhausted and feel horrible this isn't possible. So I'd say again ACCEPTANCE of the thoughts you are having is important. At first the thoughts might be worrying of course. But the main thing I learnt with being ill is the more you fight it the worse you feel – the more I tried to "be positive" and tell myself I had energy when I didn't, the worse I felt. It's an arm wrestle with yourself that you just keep losing. I like to describe it like trying to run against the waves in a very wavy sea. You will just get knocked down (the waves being the negative thoughts). If you dive with the waves (the thoughts) and accept the fact that you are having them, then it is easier and you can be more relaxed. It's all about GOING WITH THE FLOW. This doesn't mean accepting that the thoughts are true – it means acknowledge that these are the thoughts you are having, and then you can start thinking about your alternatives."

- **Ignoring positive things that happen.** For example, if a relative phones you up to ask how you are doing, instead of thinking “that was nice of her” you might think “she’s only calling because she thinks I’m going to die”.

2. Explore the negative thoughts

Ask yourself – is this idea really true?

- What would be a more realistic thought to have in this situation? Come up with some positive “self talk” – a statement that you can say to yourself when you find this negative thought arising. For instance, if you find yourself thinking “I am useless – I am letting people down again” if you are too tired to go to a family event, instead you might think “I need to take care of myself so I will have more energy tomorrow. They are good people and will understand”.
- If you tend to have negative thoughts in the same situation, is there a way that you can avoid that situation or prepare yourself in advance by using a bit of positive self-talk? For instance, if you tend to think more negatively when you are tired, you may be able to do some things to avoid getting over-tired.
- If you tend to think negatively when you are stressed, you may be able to use some of the relaxation techniques from the ‘Managing Stress’ section earlier in the manual or from the relaxation audio to

“Having heart failure has made me realise what’s important in life. I don’t fret about the little things now”

reduce your stress levels. The relaxation and mindfulness exercises in the Stress Management section can also help people to first accept and then distance themselves from persistent negative thoughts.

- Remind yourself that although sadness is unpleasant, it is not dangerous. Often these thoughts will fade away over time.
- If your negative thought is about what other people think about you, turn the problem around – if somebody else was in your situation, what would you think of them. For instance, if you think you are being a burden to your family, consider what you would think about them if they had heart failure – would you see them as a burden, or would you want to help them?
- Find ways to test your negative theory – instead of jumping to your own conclusions, you could ask other people how they feel for instance.

There are some more examples of negative thoughts and ways to challenge them in the table below.

3. Figure out what works for you

It doesn’t always work first time. Like a lot of things, learning how to deal with negative thoughts takes a bit of practice. If something that you try doesn’t work, don’t see it as a

“It’s about listening to yourself and doing what’s right for you”

failure, just try a different approach next time. The key thing is to learn from your experiences. Try out different ideas about what you can say to yourself or how you can avoid the situations which lead to negative thinking. Over time try to learn what works best for you.

If there is a particular situation that often triggers negative thoughts or makes you

upset, it may be worth making a plan in advance about what you will do next time this happens. For instance, you might say, if I get tired in the next week and I notice that I am starting to have negative thoughts, I will take a break and do my relaxation exercises (or have a nap) and see if that helps.

Negative thinking	Alternative ideas
This heart condition is really serious – this is the end of the line for me	Am I exaggerating this? • Heart failure is a challenge, but it is still possible to find happiness. • Many people with the same condition find ways to cope. Try some positive self-talk: • I need to focus on what I can still do and find ways to enjoy my life as it is now • Feeling low can be caused by the symptoms of heart failure. If I focus on managing the condition, things might get better • I will adjust to my new reality • I’m not dead yet!
My family used to rely on me, but now I’m no use to anyone	Is this really true? • OK, things have changed and, but there are lots of things I can still do. Let’s turn this around ... How would you react if one of your friends or family said that they were in this situation? • Maybe you would re-assure them that you still care about them. • Maybe you would want to help them for a change? • Maybe your family feel this way about you?

I can't be bothered to go out for my walk – what's the point?	Maybe you are thinking this way because you are feeling low or depressed. Remember that withdrawing from activities is the sort of thing that can make your mood even lower. • Going for a walk often helps me to relax and clear my mind • Even though I don't feel like it, I'll give it a try and see how I feel after the first 5 minutes
You ask your partner if he wants to go out tonight and he says he is tired. You think "he doesn't want to go out with me because I am no fun. He doesn't love me anymore".	Is there another explanation? Am I jumping to conclusions? • Maybe he is tired like he said. Is there a way to test this? • Let's see if we can go out another night – I'll ask him.
My friends won't want to do things with me anymore. I will just slow them down or be in the way.	• I may not be able to go out as much as in the past, but I will tell my friends and they will understand. • I will organise to do more things at home – sometimes with my friends, sometimes without them. Let's turn this around ... How would you react if one of your friends said that they were in this situation? • You might tell them not to be so crazy – of course you still want to do things with them. • You might be happy to change some of the things you do. Being friends is more important than what you do.
I can't cope with living like this. It would be better if I was dead.	If your thoughts are this dark, then you should put down this manual and seek help from your GP – there are both medications and therapies that can help you turn this around.

4. Look after yourself generally

The physical effects of heart failure like tiredness and sleep problems can all contribute to feeling low. So, the better you can manage your condition and keep the symptoms under control, the better you will feel. You can use the ideas in the rest of this manual to help you keep your heart failure symptoms to a minimum.

If you have any concerns or difficulties in managing low mood, please discuss them with your facilitator or heart failure nurse or your GP – they are there to help! He or she may be able to prescribe medicine that can help, or arrange for you to see a specialist who can help you to control the negative thoughts. Often you can be seen in a matter of weeks.

Other tips and tricks for managing low mood

EXPECT TO HAVE GOOD DAYS AND BAD DAYS

Hopefully after a few weeks you will notice a gradual improvement in your mood. However, you will also have bad days, and this may

affect how you use your programme or engage with the activities. This is all perfectly normal and to be expected.

DON'T OVERDO IT

Slow and steady is often the best strategy. Think about breaking things down into small, manageable chunks – that generally makes things easier.

INVOLVE FAMILY AND FRIENDS IF YOU CAN

Like all of us, when you are feeling a little down, you may have found that just having others around can be helpful. They may help you look at things differently, find ways to solve problems or maybe just be there for a chat. Getting others involved isn't for everyone and you may not be ready to take this step yet – don't worry if that is the case. But if you think you might find the support of others helpful and they want to help then why not ask? If you show them these pages, and the Family and Friends Resource they may find them helpful too.

Getting Support From Others

People are often reluctant to ask for help. However, help is available and you are not alone. Help may come from family, friends, work colleagues, voluntary organisations, and social services. Help or support may include:

- emotional support or encouragement
- practical support such as giving lifts, cooking, giving time
- support in getting information or making plans

People who get help from others often cope better. You may be worried that people will think you can't cope or that you have 'failed' in some way, or that you don't want to be under an obligation to anyone or become a burden to someone else. However, giving support can provide peace of mind and be rewarding for the person providing the support. It sometimes helps to think about this the other way round – if you had a relative or a good friend who was ill and struggling cope, would you want them to tell you about it or would you want to do what you can to help them?

"I have always been a bit independent and stubborn ... but you need to ask for help, and that is really hard ... Even if it's from a really close relative or a friend, it's really hard to make yourself ask for help. Once you've done it, it's like: Well I'm glad I did that"

Tips for getting help

- Take a little time to think about and identify all the people who you think may be able to help you.
- The first time you ask is the hardest but it will get easier. Sometimes people may be waiting to be asked as they may not know what help to offer. Practical tasks include cleaning, laundry, putting out dustbins/recycling, shopping, providing transport or just being around to talk to.
- Start with asking for help with a small task such as getting a few items of shopping while they are out anyway. Once people know what you need, over time, help may be offered without you needing to ask.
- If you turn down help it may not be offered again. Perhaps you could think of a simple task to be completed.
- Organisations such as local carer and support groups can be valuable support.
- Keep in touch with friends and family via telephone, e-mail, text messaging, Skype, Face time or other social media such as Facebook. You may have someone in your family or a friend who can help you set this up. Friends you know through clubs and organisations may also help.

*"I can just sort of say
hello without being on the
phone for three hours"*

Organising your support

Once you have a better idea of who can help you and what they may be able to do, it may help to plan how you are going to involve your friends and family. You might like to use the table below.



Example

Who can help?	How can they help?	When can they help?
Relative	Shopping	Once a week - Thursday evening
Neighbour	Pop in for a chat	Once a week - Saturday morning
Alice (friend)	Meet with me for coffee/tea etc.	Once a week - Friday afternoon

Getting help from social and council services

This section is about what support you and any person who may be helping with your care may be entitled to and how you can find out more. This includes information about financial benefits, care at home services and about equipment/aids (such as grab rails, bath or shower aids, perch stools and mobility aids).

It is worth noting that the organisation responsible for providing equipment and other

services may vary in different places, but **your facilitator should be able to advise you about who to contact locally to have your support needs assessed.**

In most places in the UK, you can apply to your local Social Services for a "needs assessment" (England and Wales) or 'community care assessment' (in Scotland) or a 'Health and social care assessment' in Northern Ireland. This can include home care help with things like cleaning and shopping and access to day centres to give you or the person who cares for you a break, access to

other activities or groups in your community. Where appropriate, your home can be assessed for appropriate aids and equipment. This could include, for example, grab rails, bath/shower aids, perch stools and mobility aids.

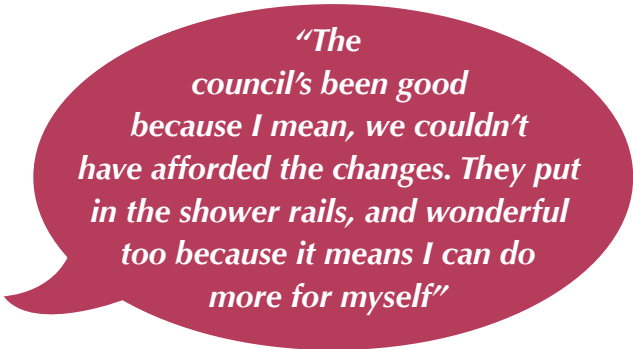
You can also ask for an 'advocate' to assist with completing forms for any benefits that you may be entitled to. Ask your facilitator for websites or contact numbers to find out more.

Carer Assessment

If someone helps with your care, like a partner or family member, they have a legal right to have a 'carer's assessment'. This is also available through the Social Services

department of your local authority or council. This involves assessing what help they might need. The Social Services assessment is used to decide what help to provide. The assessment usually involves a visit at home from a social worker or sometimes another health professional or trained volunteer.

During the carer's assessment you can also ask for care at home services or a sitting service that will allow your caregiver to have some time for him/herself. The assessor will know what is available in your area. Sometimes these are provided by voluntary agencies or charities.



"The council's been good because I mean, we couldn't have afforded the changes. They put in the shower rails, and wonderful too because it means I can do more for myself"

Common Sources of Stress in Heart Failure

It is all too much to think about. I wasn't expecting this

Even if you have had symptoms for some time, the diagnosis of heart failure may have come as a shock. For many people the term heart failure is frightening as they think this means there is nothing you can do about it. This can be misleading as there are many things that can be done to manage the condition. You may be blaming yourself and be feeling angry. You may have concerns about your work, family, or finances and what this all means for the future. All these concerns are common and frequent sources of stress. This is where finding out about heart failure and what you can do about it can help with stress. Some people try to cope with all the uncertainty by putting the problem to the back of their minds. However, this can be unhelpful, as you might overlook ideas that can be helpful in managing your symptoms or that might give you more energy. The clearer the picture you have of your own condition and circumstances the more you will feel in control and able to cope.

I feel I need to try and make sense of it all

Most people feel the same. They want to know what heart failure is and what it means to them personally. You might also question how this will affect your relationships and social life and of course your future health and life expectancy also come to mind. Having heart failure can cause great disruption to the way you live and the way you think about yourself – it can present a number of challenges. For most people, making sense of all this takes

time but as you understand more about what heart failure is and how you can manage it, the easier it will be for you to deal with. This manual and your facilitator will help you with these things, as well as helping to manage your stress levels.

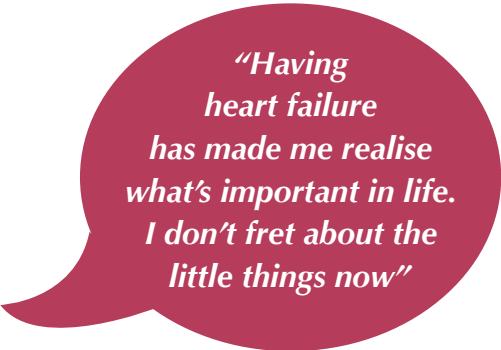
I don't feel as if I am the same person anymore

Having a long-term condition can mean that you are no longer able to fulfil the roles you previously performed as part of your daily life. For example, not being able to do your job, look after children or grandchildren or go out with friends and family. This can make you feel as though you have lost your sense of purpose – as if it has changed who you are. This can be challenging and stressful and it is a common experience. If you have been used to always looking out for others before your own needs you may find it difficult to let go at times when you need to, or to let other people help you. These changes can be difficult to accept. However, many people find that making small changes and re-thinking what is important to them in life allows them to achieve a greater sense of control and feel more at ease with themselves.

People are trying to take over

The first reaction of people who you know may be to try and protect you. This is often because they are concerned and also perhaps because they are not sure about what heart failure is and what it means in terms of daily life. If they are being over protective and trying to 'wrap you up in cotton wool' they may not understand that keeping up the

activities that you are happy and comfortable with is actually good for you, both physically and mentally. However, it is also important that you accept help when you need it. Communication is the key to getting the balance right – talk to the people around you to let them know what you need, how they can help and what you want to do for yourself. By sharing work such as household chores you will find you can do more of the things you need and want to do. Don't try to be 'Superman' or 'Wonder Woman'. Try and resist being too proud and accept help when it is offered such as carrying heavy washing so you can save your energy for when it is needed.



"Having heart failure has made me realise what's important in life. I don't fret about the little things now"

Encourage your partner, or anyone who you think might be able to help, to read the REACH Family and Friends Resource as this will explain more about the condition and its effects on daily life. For instance, it will help them understand your need for doing physical activity to keep your heart healthy.

I find it stressful coping with the changing symptoms

Many people find it difficult to cope especially

when symptoms may be unpredictable with good days and bad days. This manual has a number of tips and ideas that might be helpful here. For instance, if you keep track of how your symptoms change over time, this will help and give you a better understanding of what makes your symptoms better or worse. Pacing your activities and having a plan for coping with days when your symptoms are worse will also help. Feeling anxious or stressed about your symptoms may make things feel worse. If this happens, then using some stress management exercises, such as the breathing and relaxation exercises in the 'Managing Stress' section may be helpful.

I'm scared to do anything in case my symptoms get worse

People are often afraid of doing anything that might make their condition worse, or cause damage to their heart. This can lead them to avoid the very activities that may help symptoms and improve things for their hearts. This particularly applies to physical activity. The fact is that following a paced (slowly building up) walking or exercise programme in combination with your daily living activities will help to build your strength and stamina and is actually good for your heart.

Having a fear of activities is very common with people with heart problems. As well as losing fitness, avoiding activities that you used to enjoy may lead to feeling stressed and depressed, or feeling isolated. Your facilitator will be able to help, so it is important to discuss these feelings with him or her. Please see the 'Keeping Active' section earlier in the

manual and ask your facilitator to show you how to safely pace your activities and to avoid doing too much.

I don't know what to do now I have stopped work

Stopping work when it is not your choice can be stressful. You may feel that your work gave you purpose in life or a reason to get up in the morning. This change may also leave you with financial worries. At the time, these changes may cause a great deal of stress. Discussing your concerns with your facilitator can help you deal with this. They may be able to offer you some practical advice and direct you to other types of support such as local community groups and occupational health services, as well as suggesting some of the financial benefits you may be able to apply for.

I don't want to be a burden on others

People often have concerns that their family, or other people who provide care will have to take on too much. It may be that you do not want those close to you to take on personal care such as helping with going to the toilet or washing. You may be able to get other professional help with this so that you don't need to ask as much from your caregiver. If possible discuss this with your caregiver, then let your GP or facilitator know that you would like to seek additional outside help.

I find the symptoms frightening and stressful

Symptoms such as shortness of breath or dizziness can be stressful and sometimes frightening. Often becoming anxious can

make you even shorter of breath. Learning to relax and practising breathing exercises when you are feeling OK can help you to control your breathing when you experience these symptoms. Please read and practise the exercises for relaxation in the Managing Stress section and see the Managing Breathlessness section.

Other methods can be useful in coping with these episodes such as positive thinking and visualisation. You may want to try the positive thinking exercises and the visualisation exercises on the relaxation audio.

I get annoyed when I get so tired from simple daily living tasks

It can be very frustrating when you feel that you cannot complete all that you want to in a day. Pacing and energy-saving steps will help you here. For instance, sitting using a high stool to do tasks like washing up or preparing food may help. Having heart failure can mean that you can get really tired if you over-do things. One way to cope with this is to manage how much energy you use so that you don't get too tired (for example by going at a steady pace, or breaking big jobs down into smaller tasks). We have some tips and ideas about how to do this in the 'Managing Changes in Your Symptoms' section. Many of these have come from other people with heart failure.

All the concerns above are very common and your facilitator can also help to discuss which ideas might work best for you.

Managing Your Finances

When you developed heart failure, you, or someone who helps to care for you, may have had to reduce your paid hours or give up your job. You may also be faced with additional costs, such as transport, heating and laundry. There may be benefits and/or tax credits available to you, such as a reduction in council tax.

Whether you can claim benefits depends on several factors including your income, savings, your age, what National Insurance contributions you have made, whether you are disabled and if you have children. Each particular benefit has its own rules and the benefits available may vary depending which country you live in.

Filling out forms can be a difficult and complicated process. However, you can contact your local council and ask for someone called an 'advocate' to assist with completing forms for benefits. This can also be set up as part of the carer's assessment (see the section on 'Getting Support' above). Alternatively organisations such as the Citizens Advice Bureau, Carers UK or Age UK can also help. If you live in social housing, you can contact your local council's housing officer for support.

If you are entitled to one benefit you may find that it can lead to others. This may include free dental care, an eye test or prescriptions for example. You may also be able to claim for transport to and from hospital appointments.

Apply for benefits as soon as possible because many benefits cannot be backdated. If your claim is turned down or if you receive less benefit than you feel you are entitled to, you can appeal.

Other sources of financial help

Benefits are not the only way of getting financial help. You may be able to apply for:

- Help with health costs, for example to pay for dental treatment, spectacles or prescriptions.
- A grant to help adapt your home.
- Charity grants or benevolent funds such as the Royal British Legion if you have served any time with the armed forces.

Managing Your Home and Work Environment

This section is about how you can balance the hassles of home or work with managing your heart failure.

Home

Top Tips

- To make planning and organising a bit easier, you could use a calendar or diary to show when activities should be completed.
- Consider prioritising tasks. What do you feel are the most essential or important tasks that need doing? What things are less important and can wait? It may help if you discuss this with any person you live with, or who comes in to help you.
- It is sometimes easier to break tasks down into easier steps instead of trying to do everything in one go. Be patient as some tasks might take longer.
- If you have a computer, you could consider online shopping or alternatively some shops offer a delivery service.
- Involve other people in household. For example, ask for help from a friend, neighbour or relative to help you with preparing meals, or shopping.
- Consider making use of new technologies such as voice activated or automated control of lighting, heating, door answering systems, and calls for help.
- Try to be as flexible as possible and 'go with the flow', making the most of 'good' days.

"That calendar up there is our bible... If he wants to know anything he looks on there. I write everything on there. If I'm at work, if I'm not at work, when I'll be home, when we're going to the hospital, the doctor's appointment, everything goes on there"

You may be able to get help with household tasks such as putting out the dustbins and recycling from your local council.

Work

If you are working, you should inform employers as soon as possible about your situation as they may be able to help you in several ways – for instance, they may be able to change your work pattern or set things up for you to work at home. Often people with heart failure who go back to work can arrange a gradual re-start. Start by talking it over with your manager or occupational health. If you belong to a union they will be able to offer you advice too.

Many large organisations also have a policy on the rights of caregivers. If you have someone who helps to care for you, they may be entitled to special leave for caregiving activities.

Travel

It is really important to make sure that you have enough medications (and some spare medication) to last for the duration of your trip. So plan your trip well.

If you are flying, keep your medication in your hand luggage in case you are delayed or your luggage is lost. You might also wish to arrange airport assistance to help you get to where you need to be on time.

When travelling, carry a written medical history and a list of your current medications – you may need it if you have to seek any medical help during your trip.

If you are going abroad, you should check in advance what rules apply to taking your medications into any countries that you are passing through, as some medicines used in the UK may be controlled in other countries.

You should also check with your GP if you are OK to fly – flying does put some stresses on the heart and the rest of your body and may not be advisable if your symptoms are not well controlled.

Similarly, travel to high altitudes (1500 m or more) and travel to very hot and humid destinations are not advised unless you are in a good, stable condition.

You may also need some vaccinations depending on where you are planning to go. Your GP surgery will have a nurse or other health care professional who is an expert on travel vaccinations. You should start to organise this at least 8 weeks before you are due to travel, because some vaccinations need to be given well in advance to allow your body to develop immunity and some involve multiple doses spread over several weeks.

Remember to keep monitoring your weight and any swelling to make sure you can spot any fluid build-up early and take action (using your Traffic Light Action Plan). Make sure that you drink plenty of water, especially if the weather is hot or humid. Finally, pace yourself – don't take on too much in each day, but do have a great time!

Check with your GP surgery to check that you are fit to fly and if you need to make any special arrangements to travel abroad. Allow plenty of time (at least 8 weeks) before travel for the GP to get your vaccinations organised.

Sex

For most people with heart failure it is quite safe to enjoy sexual relations as long as your symptoms such as shortness of breath and chest pain are controlled and not bothersome. After a diagnosis of heart failure, you may feel anxious and depressed, making you less interested in sex. It is not uncommon in men with heart disease to suffer from erectile dysfunction (impotence). Some drugs such as beta blockers can also have side effects which can worsen these symptoms.

Don't be embarrassed to discuss any concerns you have about sex with your doctor or nurse - they are used to discussing this kind of thing.

The following suggestions have been adapted (with permission) from the Heart Failure Matters website which is endorsed by the European Society of Cardiology:

- Choose a time for sex when you are rested and relaxed.

- Avoid having sex immediately after eating a heavy meal or binge drinking.
- Have sex in a comfortable and familiar room.
- You can use foreplay to help your body get used to the increased level of activity.
- Use positions that you find less strenuous such as lying on your back or side.

It is also safe for people with heart failure who have had a device such as an ICD (implantable cardioverter defibrillator) fitted to have sex. You can get more information about ICDs and sex and heart disease from the British Heart Foundation website.

If you have any concerns mention it to your doctor or nurse. It may be possible to help you enjoy a rewarding sex life.

Sleep

Many people living with heart failure experience poor sleep. However, there are things that you can do to improve the quality of your sleep.

Getting a good night's sleep is important for your mental well-being, for your energy levels and to keep the symptoms of heart failure under control. Most people sleep for around 7-9 hours per night. Sleep patterns may change as you age; older people frequently sleep less at night but may have daytime naps.

Poor sleep is often a sign that something is not quite right. Poor sleep can make you feel irritable, depressed or sleepy during the day. You may also experience headaches or an

upset stomach. You may also find it difficult to concentrate.

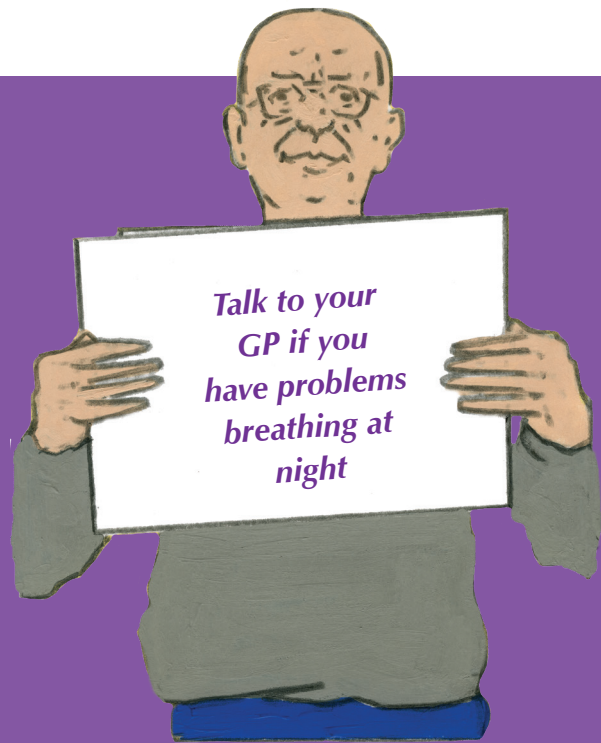
Poor sleep can have a number of causes including:

- worry, anxiety or stress
- breathlessness due to heart failure
- pain from other health conditions such as arthritis
- side effects of medication, including disturbed dreams or cough.
- smoking
- alcohol use

Suggestions for sleeping well

- Establish a good bedtime routine. Try to do the same thing every night before going to sleep, as this will help train your body to sleep at night. For example, read a chapter of a book before going to bed.
- Get up at the same time of day.
- Keep the temperature of the room at a constant level, not too hot or too cold.
- Keep a sleep diary to see if there is a pattern or reason why you are not sleeping well and to help plan your new routine. You can use the Progress Tracker to rate your sleep quality.
- Try eating a light snack before going to bed, unless you suffer from indigestion. A glass of warm milk or a bowl of cereal for instance.
- Use relaxation or visualisation techniques or listen to relaxation music. The relaxation audio which comes with this manual is ideal for this.
- Listen to some music on the radio.
- If you have trouble getting to sleep, don't go to bed too early. This makes the night much longer.
- Avoid napping during the day. Or, if you need to nap, keep it short.

- Avoid alcohol and caffeine in the two hours before bedtime. These make you wake up earlier. Alcohol may also affect your safety if you go to the bathroom at night.
- Avoid smoking as nicotine also affects the quality of sleep.
- If you wake in the night feeling worried, try to avoid 'clock watching' and tossing and turning. Instead, sit up in bed reading or spend time out of bed doing a mundane chore or watching television. Write down everything that's on your mind and what you might be able to do about it. Go back to bed when you feel tired.
- Ensure that your bedroom is dark and quiet. Use a blackout blind if affected by light.
- If you have trouble getting to sleep, get up and go to another room and sit quietly for about 20 minutes before going back to bed. Consider separate beds or a separate room if you are disturbing your spouse or partner and the home situation allows.
- Exercise as directed in the 'Keeping Active' section earlier in the manual – this can really help to improve sleep. However, avoid exercise in the two hours before bedtime.
- If you are breathless at night you may sleep better propped up with some pillows. If this is a new symptom then you should report it to your GP or heart failure nurse specialist.



- If you have pain at night, either from your heart or due to another condition such as arthritis, discuss this with your GP. There may be other treatment options for you.
- Any side effects of medication that affect your sleep should also be reported to your GP who may be able to change your medications to prevent this.
- If you frequently stop breathing whilst asleep, discuss this with your facilitator or GP or heart failure specialist, as this can often be treated.
- If poor sleep continues then you should consult your GP who may be able to assess your sleep and recommend further tests or treatments. Sleeping tablets may not be an option because of your heart condition.

Implanted Devices

Not all people with heart failure need a cardiac device; they are for specific reasons such as correcting the rate at which the heart is beating. Devices include:

- Pacemakers – to monitor the heart and control your heart rate if it drops too low.
- Implantable cardioverter defibrillators (ICDs) – to monitor the heart rhythm and shock the heart if the rhythm is abnormal or the heart stops.
- Cardiac re-synchronisation therapy – a specific type of pacemaker that ensures the different parts of the heart contract in the right order. It may also have a defibrillator.

Many people with heart failure and their families or supporters are fearful that their device may ‘go off’ causing them to worry about going out alone. Some pacemakers make a ‘ticking sound’.

“I have gone out with her each time. She’s going to do some shopping today, which is a major step forward. That will be the first time that she’s done it.”

Tips for coping with devices

- If you are concerned about going out, start with short trips out with another person. Start with half an hour and build up the time. You will gain confidence over time. Reflect on how you feel after a trip out.

- If going out alone, take a mobile phone for peace of mind. You might like to use text messaging to keep in touch.
- Remember that if the device does go off, this is a good thing – it means that the device is working and doing the job it is meant to do.
- Follow the recommendations of your health care team about what to do when it fires.
- Add the contact details for the specialist or technician for your device to your list of contacts in your mobile phone or telephone address book. You can contact them for non-emergency advice.
- Consider wearing a medic alert bracelet or tag necklace. Ask your nurse or GP if you don’t know where to get these items.

For further online information about devices you can search the internet for:

- **NHS Choices (known as ‘NHS Inform’ in Scotland).** Heart failure treatment.
- **British Heart Foundation.** Pacemakers or defibrillators.

Your facilitator can also give you additional information. **Please do ask questions if you have any concerns.**

Alcohol

Alcohol can affect your heart, and drinking too much can worsen your heart failure.

For most people it is safe to consume up to 1 to 2 drinks per day. It is recommended (for both men and women) not to regularly drink more than 14 units a week, with at least 2 alcohol free days in the week advised.

People with more severe symptoms may be advised to keep off alcohol altogether.

If you have been diagnosed with alcohol induced cardiomyopathy, (i.e. your heart failure is as a result of excess alcohol intake), you are advised to abstain completely.

All people with heart failure are strongly

advised to avoid binge drinking and to stick within the recommended limits.

Drinking more than the recommended limits may:

- cause high blood pressure
- damage your heart muscle
- cause problems with your heart rhythm
- worsen ankle swelling
- make you put on weight

Discuss your alcohol intake with your facilitator or GP or nurse to find out what is safe for you.

1.5 units  Small glass red/white/rosé wine (125ml, ABV 12%)	2.1 units  Standard glass red/white/rosé wine (175ml, ABV 12%)	3 units  Large glass red/white/rosé wine (250ml, ABV 12%)	2 units  Pint of lower-strength lager/beer/cider (ABV 3.6%)
3 units  Pint of higher-strength lager/beer/cider (ABV 5.2%)	1.7 units  Bottle of lager/beer/cider (330ml, ABV 5%)	2 units  Can of lager/beer/cider (440ml, ABV 4.5%)	1 unit  Single small shot of spirits* (25ml, ABV 40%)

Many people get confused when trying to calculate units. Have a look at the guide below to see the alcohol content of your favourite drinks.

A single unit is 10ml of pure alcohol. ABV (alcohol by volume) is the percentage of the liquid that is pure alcohol.

So, a small glass of wine (125ml) at a strength of 12% has 15ml of pure alcohol. This is the

equivalent of 1.5 units. A large glass (250ml) contains 30ml of alcohol (3 units).

A Unit Calculator and Drinks Tracker are available to download from the NHS Choices (or NHS Inform in Scotland) website.

The information above has been adapted with permission from 'An everyday guide to living with heart failure' published by the British Heart Foundation.

Smoking

Why should I stop smoking?

You might think that now you have heart disease it is not worth quitting smoking, or that it is too late to get any benefits. However, nothing could be less true. When you smoke, your heart has to work harder to supply your body with sufficient oxygen. This is because smoking increases the carbon monoxide in your blood, which reduces the amount of oxygen carried by your red blood cells. It also makes your blood sticky, which reduces the benefits of aspirin or other blood-thinning medications you are taking. Smoking also leads to a narrowing of your arteries, which can increase your blood pressure and means your heart has to work harder. Indeed, many of your organs will have to work harder to remove all the toxins that smoking puts into your body.

So, the best thing you can do to help your heart condition is to stop smoking. You may have tried before. Don't be put off by previous unsuccessful attempts – most people have to try a number of times before they successfully quit smoking.

Tips for stopping smoking

Set a quit date and stick to it. Make sure family and friends know you are planning to stop and ask for their support. If household members smoke, encourage them to quit at the same time, or to consider smoking outside. Remove tobacco products from your home and places where you spend a lot of time.

Some people find using nicotine replacement,

like nicotine gum or patches or e-cigarettes, for a while helps them to quit smoking. There are also tablets (Bupropion and Varenicline) which can help decrease cravings. You can ask your doctor or the NHS Stop Smoking Service for advice about what might be most suitable for you. Using nicotine replacement or stop smoking medication really increases the chance that you will successfully quit smoking.

Keep a jar and put the money you would have spent on cigarettes or tobacco into it. Buy yourself a treat with the money you've saved.

Try to be more active, as exercise helps to reduce nicotine withdrawal symptoms.

Some people find that listening to relaxation audio, or using mindfulness techniques, can also be helpful in reducing their stress levels and reducing cravings.

You may find that chewing sugar-free gum helps, as it gives you something to do with your hands and mouth.

Cravings come and go – often they will go away after a few minutes – so find things that you can do to distract yourself if you have a craving. People who have successfully stopped smoking say that the cravings do go away completely over a few months, so don't think that you will have to live the rest of your life feeling desperate for a cigarette.

Remember, cutting down is harder than quitting smoking and you won't get the full benefits unless you completely stop smoking.

If you slip up, don't leave it a while, get straight back on track. It often takes several attempts to be successful. Figure out what led to you start again and think about how you could stop that from happening if you were to try again.

The NHS has a free smoking cessation service that can help you quit – this usually includes counselling as well as nicotine replacement or drug-based therapies. High quality research shows that using these services trebles your chances of quitting compared to doing it yourself. Ask your GP or nurse for contact details or look them up online.

A word on e-cigarettes: These are less harmful than smoking tobacco, as these only contain nicotine, without the hundreds of cancer-causing chemicals in cigarette smoke. Using these as a way to gradually cut down and quit is similar to using nicotine patches.

Finally, if you do decide to stop smoking, keep an eye on your diet and the amount you eat. Stopping smoking slows down your metabolism (as your body no longer has to clean out all the toxins from cigarette smoke every day). This means you can put on weight. Indeed, the average weight gain, one year after stopping smoking, is around 1 stone (6-7kg). So you may want to adjust your eating patterns to keep this under control.

What next?

Make plans to quit smoking, consider attending a stop smoking service.

Set a quit date and stick to it.

Once you've stopped, take it one day at a time and congratulate yourself on your achievement.

Healthy Eating

Eating healthily is important if you have heart failure because:

- what you eat can affect your blood pressure and your symptoms
- it can affect how much energy you have over the course of the day
- it can affect your risk of heart attacks and other health problems
- eating too much salt can make you retain more fluids

What is a healthy diet?

The recommendations for healthy eating for people with heart failure are the same as for the general population. This means eating a wide variety of foods and having the right amount of food and drink to maintain a healthy body weight.

A healthy diet is one that is low in fat (especially saturated fat) and includes plenty of fruit and vegetables, balanced with carbohydrates (starchy foods such as potatoes, rice and pasta) and protein (from things like meat, fish and lentils). Along with some milk and dairy foods, this will give you all the nutrients you need. You should also limit the amount of drinks and snacks you have that are high in sugar or salt or both. If you have heart failure you should be particularly careful not to eat too much salt (see the next section on 'Managing Your Salt'). So a healthy diet should include ...

- Fruit and vegetables – this should account for around a third of what you eat.

- Starchy foods, such as bread, rice, potatoes and pasta. Choose wholegrain varieties whenever you can, or eat potatoes with their skin on for more fibre. Again, this should account for around a quarter to a third of what you eat.
- Protein. This can come from meat (ideally lean meat or without skin), fish, eggs and beans, and from milk and dairy foods and soya products.
- Only small amounts of foods that are high in fat or sugar.

Most people in the UK eat and drink too many calories. They eat too much fat, sugar and salt, and not enough fruit, vegetables and fibre.

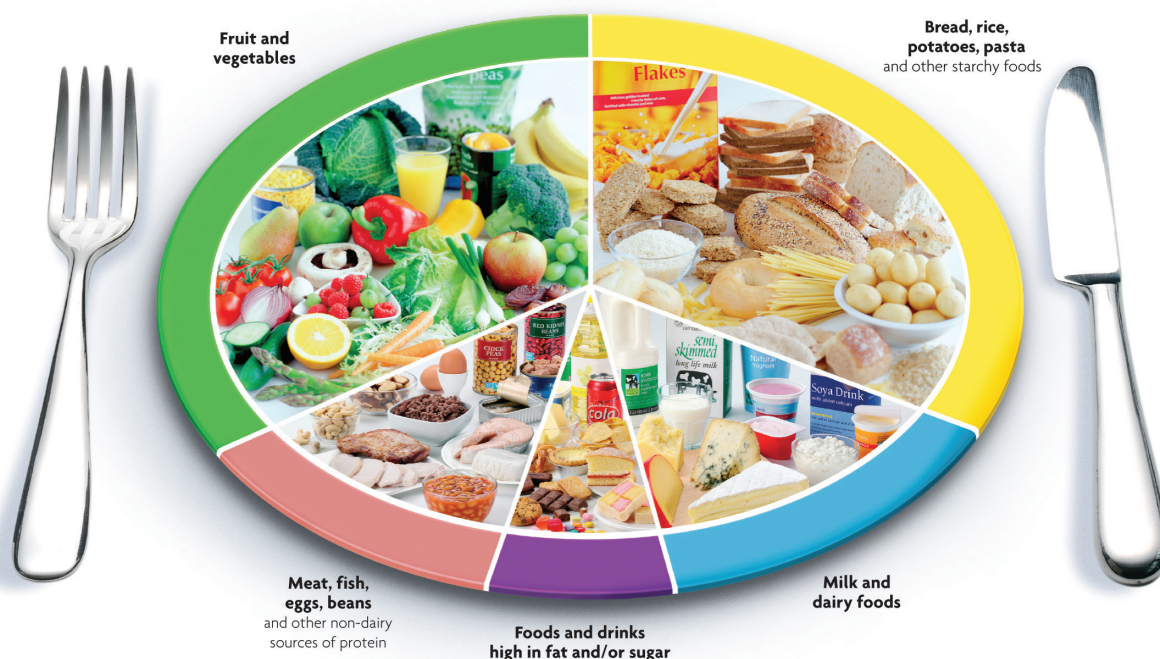
Some fats are more unhealthy than others – these are called **saturated fat** and **trans fat**. Saturated fats are found in meat (especially red meat), butter, cheese and other animal products. Trans fat is present in processed foods including many snack foods like crisps, biscuits, cookies, cakes, pies, pizzas, and deep fried foods, such as chips or chicken nuggets, as well as in some 'ready meals'. This type of fat is so dangerous and unhealthy that a number of companies are starting to ban it. It is worth checking out the labels of foods or snacks that you eat frequently to see if they contain trans fat.

You can get guides to food labels from a number of places on the internet including the British Heart Foundation.

The eatwell plate



Use the eatwell plate to help you get the balance right. It shows how much of what you eat should come from each food group.



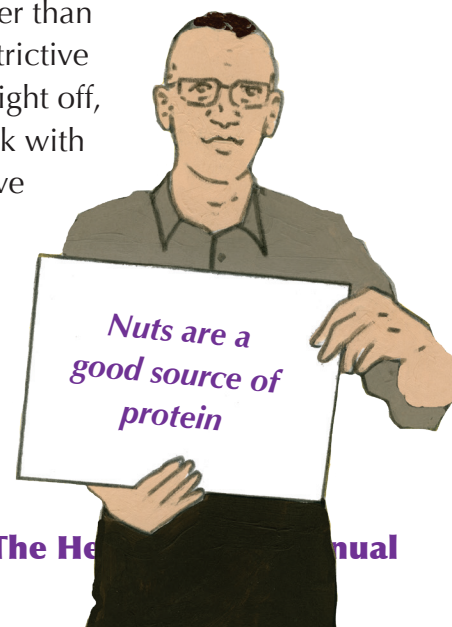
The Eat Well Plate is reproduced from the NHS Livewell website. © Crown copyright material is reproduced with the permission of the Controller of HMSO and Queen's Printer for Scotland, Public Health England in association with the Welsh government, the Scottish government and the Food Standards Agency in Northern Ireland.

If you have heart failure, then being a little overweight is not necessarily a problem. However, if your body mass index (your weight in kilograms divided by your height in metres, then divided by your height in metres again) is more than 35, this can cause some problems in terms of:

- putting extra strain on your heart and lungs
- putting pressure on your lungs when you lie down, making it harder to breathe and to get a good night's sleep
- making it harder to do exercise or to stay active
- increasing your blood pressure

- putting pressure on your knee and hip joints, which may make any joint pain a lot worse

If you are interested in losing weight, it is better to make a number of small changes that are sustainable, rather than trying to go on a restrictive diet. To keep the weight off, you will need to stick with any changes you have made – and if you want to lose more weight, you will need to add more changes.



Healthy eating tips

1. Limit the amount of fat you eat, especially 'saturated' fat.
2. Include plenty of 'starchy' foods in your diet – bread, rice, potatoes, pasta and choose wholegrain when you can.
3. Include food that is high in fibre such as pulses (beans and lentils) and whole-grain options. High fibre foods keep you feeling full for longer as they release their nutrients slowly.
4. Have at least five portions a day of fruit and vegetables.
5. Limit sugary drinks and unhealthy snacks (high fat or high sugar snacks).
6. Limit the amount of salt in your diet (see section on 'Managing Your Salt' later on).
7. If you drink alcohol, drink in moderation.
8. Fish is a good source of protein with low saturated fat. Aim to eat at least two portions a week, including at least one portion of oily fish such as salmon, mackerel, trout, herring, fresh tuna, or sardines.
9. Changing the balance of things on your plate, changing the type of foods you choose (e.g. lower fat or wholegrain options), changing the way you cook food (using less fat, grilling rather than frying etc) and eating less overall are four ways to make your diet healthier.

10. Aim for a diet that is both healthy and enjoyable – only make changes that you can live with!

Drinks that are high in calories include fizzy drinks containing sugar and also fruit juices. **Recent research shows that fruit juices are as unhealthy as many fizzy drinks** and so you should not count fruit juice as one of your 'five a day' (this contradicts previous advice about fruit juice). Whole fruit however is very good for you, as it has fibre and vitamins and the sugars are not as concentrated as they are in the juice.

Recent evidence suggests that eating protein also keeps you feeling fuller for longer and so having more protein (but not more fat) could be helpful. You can get protein without fat by eating more pulses (lentils, beans). Lean meat, fish, eggs and fat-free dairy products can also be a good source of low-fat protein.

Nuts are a good source of protein and are high in fibre, so they can be a good alternative to snacks that are high in saturated fat, but they do still contain high levels of fat, so eat them in moderation.

Good sources of advice, with healthy recipes (including recipes for South Asian diets and eating healthily on a budget), are the NHS LiveWell and British Heart Foundation websites.

Keeping it off

Losing weight is one thing, but the real challenge is keeping it off! The following tips might help:

- Instead of making drastic changes that are hard to keep up, try making a series of small changes that will reduce calorie intake (e.g. less sugar in tea, low fat options, 10% less of everything). Only make changes that you can live with!
- Build up your 'resilience' by keeping track of what situations lead you to go off track with your eating. Think about ways that you can solve the problem when these situations occur.
- Weigh yourself regularly and try to figure out what makes your weight go up and down over time.
- It can be worth thinking about why you were eating more food than your body needs in the first place – what does eating that extra food do for you? For some people it is about managing stress or boredom, for others it is just going along with what other people want to do and for others it is just about the sheer pleasure of eating. The problem is that if you make some changes to your lifestyle, you will then be missing out on something (whatever the food was doing for you previously). **So, ask yourself “how else could I fulfil that need?”** – is there another way to get what you need without resorting to eating? If you can figure this out, then you may find that the 'tension' or pressure that you feel from 'being on a diet' goes away and it becomes a lot easier to manage your weight.
- Stress or boredom are often triggers for overeating, so think about other ways that you can manage stress or boredom (using the Stress Management techniques in this manual for instance).
- If you do have a lapse, don't beat yourself up about it – just make sure that you get back on track the next day.
- Support from others really helps, so if you live with someone else, have a serious conversation about how you want to change your lifestyle and how they can support you in this.

Managing Your Salt

Many people believe they eat a diet which is relatively low in salt because they don't add it at the table or when they cook. These are good habits, but you need to remember that most of the salt that we eat is in the food itself! Many processed, smoked and tinned foods are already high in salt. Foods that are often high in salt include:

- cheese
- bread (unless you make your own)
- stock cubes and stock based drinks
- tomato sauce
- soups (unless you make your own)
- cold meats
- bacon and sausages
- ready meals
- crisps and snacks

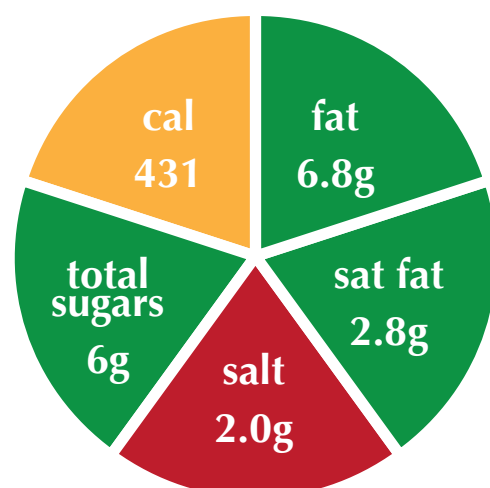
Salt is made up of two minerals, sodium and chloride. Although our bodies need these minerals they only need very small amounts. Sodium is particularly important as it is directly linked to the amount of fluid we store. The more sodium in our body the more fluid it holds. This is particularly important for people with heart failure as it can increase the pressure placed on the heart, leading to increased breathlessness and swelling.

What do the recommendations say? General healthy eating advice states that we should avoid eating more than 6 grams of salt each day. As salt is made up of both sodium and chloride this can be calculated as 2.5 grams of sodium. Knowing what foods are high, medium and low in salt is important when: shopping, preparing food and eating out.

ACTION

Go to your kitchen cupboard and take out a tin, packet or jar of food. Take a look at the nutrition label, similar to the one shown below. Now look at the sodium or salt contained. The label below shows that the food contains 0.25 grams of sodium per 100grams. **Foods which are considered high in sodium have more than 0.6grams per 100 grams.** Foods which are low in sodium contain less than 0.1grams per 100grams. If only salt is shown, a high salt level is more than 1.5 grams per 100 grams and **a low salt level is less than 0.3grams per 100 grams.** This food label therefore indicates a food which has a medium salt content. Did the food you selected have a high, medium or low amount of salt?

This pack provides...



Avoiding foods which are high in salt has become easier since the introduction of colour-coded labels. Next time you're at the supermarket, compare the salt content in your usual brands. If there is an alternative which is lower, perhaps it's time to make a change?

Reducing the salt you eat can be difficult particularly if you have become used to salty foods, but you don't need to lose flavour just because you are reducing your salt. Firstly, if you use herbs and spices instead of salt, this can keep food tasting good and may spur you on to try something new. Secondly, if you reduce the amount of salt gradually, your taste buds will soon adjust (and the same is true for sugar in tea or fat in milk) – your taste buds get used to what you give them!

Don't use "salt substitutes": Some people try to reduce the salt they eat by using salt substitutes (low-sodium table salt alternatives). However, these can have high amounts of potassium – this is another mineral which must be kept in balance when living with heart failure. It is important that you avoid these products.

ACTION

For your next few shopping trips, try to find the lowest-salt options for:

Bread, cereals, snacks, sauces and any other foods you eat regularly. Processed foods (such as ready meals, cook-in sauces) are often high in salt.

NUTRITION INFORMATION

TYPICAL VALUES	PER 100G	PER SERVING
ENERGY	241kJ/58kcal	964kJ/232kcal
PROTEIN	3.0g	12.0g
CARBOHYDRATE	6.8g	27.2g
of which sugars	2.9g	11.6g

Other Conditions That Might Affect Your Heart Failure

Managing heart failure can be a challenge in itself. However, it is extremely important that any other conditions are also well managed as this can have an impact on how well your heart failure is controlled. Other conditions might include:

- diabetes
- high blood pressure
- lung conditions (asthma, COPD)
- kidney problems
- sleep apnoea
- anaemia
- thyroid conditions
- angina
- abnormal heart rhythm (particularly atrial fibrillation)
- other heart conditions
- gout
- low mood, anxiety or depression
- dementia
- arthritis
- osteoporosis
- excess weight (a Body Mass Index of 35 or more may be a concern if you have heart failure)

Like with heart failure, it is important to have a good 'self-care' plan. This might include a lot of the same kinds of things you do to manage your heart failure:

1. Taking medications: Some of these conditions need to be managed by taking medications and these can sometimes affect the way your heart failure medications work. **You may see several specialists for various health-related issues. In this case it is essential that they are all aware of all your conditions and how they are being treated.** This will help to reduce the risk of problems.

Remember to take your 'Progress Tracker' with you to all medical appointments. The medication section will help you to explain how your heart failure is being managed and keep track of any changes. If you need guidance or support to manage any other conditions, or if you notice any changes in the symptoms associated with them, it is important that you speak to your doctor.

2. Managing stress: An increase in stress due to other illnesses can affect your heart, as we have described in the earlier 'Managing Stress' section. Also, just like with heart failure, the effects of many these conditions can be reduced by managing stress. So, use your relaxation techniques and other stress-busting ideas to keep any stress and anxiety under control.

3. Keeping active: Just like with heart failure, the effects of many these conditions can be reduced by maintaining your basic fitness and physical activity levels. For instance there is good evidence that regular physical activity reduces arthritis pain and helps to control your blood sugar if you have diabetes. If you have diabetes, please read the advice on avoiding

low blood sugar (hypos) in the problem solving table in the 'Keeping Active' section earlier in the manual.

4. Eating a healthy diet: Just like with heart failure, eating healthily affects a number of other conditions. For instance there is good evidence that a healthy diet helps to control your blood pressure and blood sugar and is important if you have diabetes or heart or kidney problems. Eating healthily and

avoiding over-eating is also essential for managing your weight. Your weight also has a big effect on joint pain if you have arthritis. There is more information about eating a healthy diet and how to manage your weight in the 'Healthy Eating' section above.

Avoiding smoking and excess alcohol can also make a big difference to most of the above health problems – there's a good reason that it's called a healthy lifestyle!

Living With Uncertainty

Many people have long fulfilling lives when living with heart failure. However, there are no guarantees and sometimes, even when things seem to be going well, people can go into a decline, sometimes quite suddenly. This section provides some information on what you can do to prepare for the future.

Life is often unpredictable and it may be appropriate, whether you are living with heart failure or not, to prepare for end of life. You may feel that now is the right time for both of you to review your plans about what will happen when either of you pass on, including making a will. Having a plan in place can help to reduce anxiety about how others will cope if you are not around. Organisations such as Age UK, Carers Direct, or a solicitor will be able to help. Think about any friends and family who you might like to discuss your plans with, this will help them know what to do when you need them. You might like to prepare an 'Advance Decision' document, which is also known as a 'Living Will'. This might include things like expressing a wish to refuse certain types of treatment, such as a blood transfusion. Letting your family know about your wishes could help them if they are ever in the situation where they have to

make decisions about your care. The Age UK website has useful information about this and advice on how to obtain Power of Attorney. You may like to make other practical plans for how you will cope if you become increasingly unwell.

Your GP will be able to help you prepare for the end of life. You may also wish to discuss your wishes with people who are close to you. This might include where you wish to be or who you want to see. There are other services that can work with you including specialist cardiac nurses, district nurses, hospice care and 'palliative care' nurses.

If you are concerned about this sensitive topic, then talk to your facilitator who will be able to support you and let you know what help is available. You may also find help with your local faith groups – you do not have to be a member to contact them. Other end-of-life services such as Cruse Bereavement Care can also support you as well as your friends and family. More information is also available from the NHS Choices (or NHS Inform in Scotland) website.

Fluid Record

Sometimes you may be advised to limit your fluid intake for a period of time (for instance to 1.5 or 2 litres per day). This is not only the fluid which you drink but additional fluid contained within foods such as soup or ice cream. If you have been advised to limit your fluids it may be helpful to use the space below to record the fluids you take in each day. **The limit will be set by the health care professional who has advised you to limit your fluid intake.**

You may also find the following tips helpful:

- measure out the fluid you can have each day and store it in the fridge
- know what volume of fluid is contained in your usual glass, cup or mug
- if you feel thirsty, try sucking a piece of frozen fruit or ice cube, or rinsing your mouth with water or non-alcohol based mouthwash

Target amount (per day):

Week	Mon	Tues	Wed	Thurs	Fri	Sat	Sun
Morning							
Afternoon							
Evening							
Daily Total							
If you are thinking about increasing or decreasing the amount of fluid you have, how might you be able to do this?							

ACTION

Not everyone needs to restrict their fluid intake. You should restrict your fluid intake **only if you have been advised to do so** by your heart failure specialist nurse or GP. They will also be able to advise on what quantity you can take.

Target amount (per day):

Week	Mon	Tues	Wed	Thurs	Fri	Sat	Sun
Morning							
Afternoon							
Evening							
Daily Total							
If you are thinking about increasing or decreasing the amount of fluid you have, how might you be able to do this?							

ACTION

Not everyone needs to restrict their fluid intake. You should restrict your fluid intake **only if you have been advised to do so** by your heart failure specialist nurse or GP. They will also be able to advise on what quantity you can take.

About Your Medications

Managing heart failure will almost always involve taking several different kinds of medication. The doctor or nurse probably explained to you what they are for and why you have to take them, but it can be difficult to remember all this information particularly if you have just been told you have heart failure or are recovering from recent illness.

Living with the condition will require regular monitoring of your medication and often a number of adjustments are needed to the type of pills you take or the amount of the medication you are taking.

This is called 'medicines optimisation' and is done to make sure you are on the best dose of each drug for your condition, at that point in time. Over time you may be asked to reduce or take additional drugs if your symptoms change or if research shows a new drug will help. Your doctor or nurse will explain about any new medicines.

We have included in this section some more detailed information about the medications which are most frequently prescribed to treat heart failure.

Prescription medicines come with an information sheet. Always be sure to read the information right through when you start taking the pills. If you have any questions speak to your doctor, facilitator or pharmacist.

Doctors prescribe medicines to manage heart failure for three main reasons:

- to reduce or manage symptoms
- to slow down the progression of the condition
- to reduce the risk of developing other problems

An essential part of your 'self care' is continuing to take the medication prescribed. Although you may feel no changes to your well-being on a day-to-day basis, you can be reassured that by taking your medication you are not only helping yourself now but supporting your heart for the future.

Frequently Prescribed Medications

Angiotensin converting enzyme inhibitors (ACE inhibitors)

The name of ACE inhibitor medications frequently end in ‘-pril’.

Commonly prescribed ACE inhibitors include:

Lisinopril, Perindopril, Ramipril and Trandolapril.

In Part 1 we discussed how the body tries to improve the blood flow when heart failure occurs. This is done in several ways. One of these ways involves sending chemical messages to the body to control fluid balance and blood pressure. One of these chemical messengers is called angiotensin. Angiotensin can cause blood vessels to become narrower, making it harder for the heart to push blood around the body. ACE inhibitors help by allowing the blood vessels to relax and widen, this lowers the blood pressure and improves blood flow to the heart. ACE inhibitors make the work of the heart easier. ACE inhibitors protect the heart in other ways, helping you to live longer and feel better. These medications have also been shown to reduce the need for admission to hospital.

Possible side-effects

- light headedness/dizziness or even fainting
- a dry, persistent cough
- reduced kidney function
- mineral imbalance (high potassium)
- swelling of the throat and/or face

Things to remember

As ACE inhibitors play a key role in slowing down the progression of heart failure, they are likely to play an important part in the management of your condition. It may however take several weeks to reach the recommended dose, this is due to the effect that ACE inhibitors can have on your blood pressure and how your kidneys adjust to the medication. It is normal to start ACE inhibitors at a low dose and gradually increase them every 2 weeks until they reach the best dose for you. At this time your nurse or doctor will check your blood pressure and take your blood to check that your kidneys are coping, as well as monitoring the minerals (potassium particularly) in your blood. After you have reached a stable dose you will probably have to have your blood checked every 6 months.

ACE inhibitors reduce your blood pressure and this can lead to possible symptoms of dizziness and even fainting. It is important to be aware of the affect that your ACE inhibitor has on you. This may mean taking extra care when getting up in the morning and avoiding getting up too quickly. If you experience dizziness first thing in the morning, avoid standing up straight away on waking. Instead sit on the side of the bed for a few minutes to allow your body to adjust before standing. It is important that you report any troublesome side effects to your doctor or nurse as they can often help you manage any side effects that may occur.

Angiotensin-Receptor Neprilysin Inhibitors (ARNIs)

ARNI medication is a combination of valsartan and Sacubitril known as “Entresto”.

ARNIs help to improve the heart’s ability to pump blood around the body. This medication is not suitable for everyone. Your doctor or nurse practitioner will advise you regarding this, and will also inform you of the suitable starting dose for you should you be needing to use this medication. It generally has to be taken twice a day, approximately 12-hours apart. This tablet cannot be taken if you are currently taking Angiotensin converting enzyme inhibitors or Angiotensin-II receptor blockers. For more information on this medication, go to the Heart Failure Matters website.

Possible side effects

- Light headedness/dizzy or even fainting (low blood pressure)
- Reduced or worsening kidney function
- Dry cough
- Swelling of the face and lips

Things to remember

It is very rare to develop swelling of the tongue or throat. If this occurs you must immediately stop taking the medication and seek expert help or call the emergency services. You will be advised to have your blood pressure and kidney function checked prior to starting this medication and to have it monitored shortly after starting, and following each increase. Once you have reached a stable dose you should have your blood pressure and kidney function checked every six months.

Angiotensin-II receptor blockers (ARB’s)

The name of ARB medications frequently end in ‘-sartan’.

Commonly prescribed ARB’s include: Candesartan, Losartan and Valsartan.

Similar to ACE inhibitors, ARB’s help to relax and widen blood vessels, easing the flow of blood around your body. They are often used if you are unable to tolerate an ACE inhibitor or if you develop side effects from your ACE inhibitor such as a persistent cough. ARB’s lower blood pressure, reducing the workload of the heart.

These medications tend to be started at a low dose and are increased gradually by your doctor or heart failure nurse specialist over a number of weeks. This helps your body to adjust to the changes which may occur. Like ACE inhibitors they can affect the function of the kidneys and increase the amount of potassium in the blood.

Possible side effects

- Light-headedness/dizziness or even fainting.
- Reduced kidney function.
- Diarrhoea.
- Swelling of the throat and/or face.

Things to remember

As ARB medications reduce blood pressure, light-headedness or dizziness may occur until your body becomes used to the medication. If you are affected by these symptoms it is important that you take particular care first thing in the morning when rising from bed

or after a prolonged period of sitting. It is also important to inform your doctor or nurse of any symptoms you may have been experiencing.

It is normal to have the dose of your ARB increased. This normally takes place every two weeks but depends on how your body is adjusting to the therapy. Prior to each dose change, you will be asked to have your blood pressure checked and your blood taken; this is to ensure you get the best dose of medication for you. Once you have reached an optimum dose you will need to continue to have your blood pressure and bloods monitored. This normally takes place every 6 months.

Beta-blockers

The name of beta-blocker medications frequently end in ‘-lol’.

Commonly prescribed beta-blockers include: Bisoprolol, Nebivolol and Carvedilol.

As the heart struggles to supply an adequate blood flow, so the body tries to find ways to increase the function of the heart. One of the body's coping mechanisms is to speed up the heartbeat. Unfortunately over time the result is an increase in pressure on a system which is already struggling to meet the body's normal demands. Beta-blockers are prescribed to counteract this reaction by blocking the signals sent to intensify the force of each beat and number of times the heart pumps. Beta-blockers may also expand the blood vessels reducing the workload of the heart as well as stabilising the heart's rhythm. All of these

actions help to improve the function of the heart.

Beta-blockers not only help people with heart failure to feel better by reducing symptoms, but can help them live longer with less need for hospital stays.

Possible side effects

- Cold fingers and toes.
- Sleep disturbance.
- Light headedness/dizziness or even fainting.
- Changes in blood sugar in those with diabetes.
- Reduced sexual function.

Things to remember

Some people may notice a slight increase in symptoms such as increased tiredness or shortness of breath when beta-blockers are started or when the dose is increased. Beta-blockers can take time to adjust to but symptoms should improve within a week or two. Beta-blockers are started at a low dose and gradually increased to help reduce the risk of side effects. This allows the body to adapt to the effects that the medication may have. This can mean that it takes several weeks or even months to feel the full benefit of the medication. Once started it is important to persevere with the treatment as beta-blockers have been shown to improve symptoms and help people with heart failure to live longer.

If you are finding any side effects troublesome, **it is essential that you do not stop your beta-blocker suddenly as this can cause a rapid worsening of symptoms.** Always speak to your

doctor or nurse who will be able to review your medications and offer advice about the best course of action. If your symptoms worsen suddenly, you should contact your doctor as soon as possible.

If you have a lung condition such as asthma you may not be able to take a beta-blocker as this may make your symptoms worse. Always discuss other medical conditions with your doctor if you are unsure.

Taking a beta-blocker will slow your heartbeat and can reduce your blood pressure. Your doctor or nurse will monitor your pulse and blood pressure regularly especially when you have the dose of your beta-blocker adjusted until you have reached the correct dose. Once you have had your beta-blocker stabilised you will need to continue having your blood pressure and pulse checked. If you have diabetes your blood sugars may also need to be monitored more frequently as it can affect them. Your doctor or nurse will tell you when all of these tests should be done.

Other heartbeat regulators

Digoxin

Digoxin strengthens the heartbeat and slows it down. Digoxin has been used for many years particularly for older people who have a heart rhythm known as 'atrial fibrillation'. Today digoxin is used for people who have heart failure who may or may not have atrial fibrillation. It can help reduce the need for hospital admission, improve the way the heart works and make people feel better.

Possible side effects

- Heart rate or rhythm changes.
- Light headedness/dizziness or even fainting.
- Stomach upset (nausea and vomiting).
- Bowel changes (diarrhoea or constipation).
- Confusion.
- Visual changes.

Things to remember

Digoxin needs to be carefully monitored as the medication can build up in the body, particularly in older people, causing some unpleasant side effects. Regular blood tests will be required to monitor the minerals within the body, particularly potassium, and occasionally your doctor or nurse specialist may want to check the amount of digoxin in your blood. Your doctor or nurse specialist will let you know if and when these tests should be done. Digoxin is also known to interact with a number of other medications and some herbal treatments. It is important to let new doctors or nurses know that you are taking digoxin and speak to the pharmacist before taking any other medications.

Ivabradine

Ivabradine works by slowing down the electrical signals of the heart. It is prescribed by heart failure specialists. Ivabradine is offered to people who have a specific heart rate and normal heart rhythm.. It has been found to be particularly useful in those who are unable to take beta-blockers.

Possible side effects

- Dizziness.
- Headache.

- Blurred vision or difficulties with changes in light.

Things to remember

As with many of the medications used to manage heart failure, ivabradine requires careful monitoring. This will include checking your pulse and blood pressure to assess your progress. If you have been prescribed ivabradine, it is important that you avoid grapefruit juice as a chemical within the juice can increase the amount of medicine in your blood stream. Ivabradine may also interact with other medications including some antibiotics and the herbal remedy St Johns Wort. It is important to remind your doctor or nurse that you are taking ivabradine and speak to the pharmacist before taking any other medications.

Diuretics (water tablets)

The name of diuretics frequently end in ‘-ide’.

Commonly prescribed diuretics include:

Furosemide, Bumetanide, Torasemide, Bendroflumethiazide, Indapamide and Metolazone.

Diuretics, or water tablets as they are commonly known, are used to help remove extra fluid which may build up in your body. Some of the main symptoms experienced by people with heart failure are due to the body's reduced ability to keep fluid in balance. This can cause breathlessness and swelling of the ankles, legs and waist.

Fluid not only refers to the water you take in from eating and drinking but the natural

fluids that are within your body. Around 60% of your body is made up of fluid. Fluid is within your organs, tissues and bones and is carried around in the blood. The heart sends blood all over the body but the main organs which help to balance fluid within the body are the kidneys. The kidneys not only balance the body's fluid needs but manage some of the body's minerals, in particular sodium (a mineral found in table salt) and potassium.

Diuretics make the kidneys work harder.

This causes you to produce more water

(urine), so you will need to visit the toilet

more frequently. Diuretic medicines are an important part of controlling symptoms. Some people may have also been given a diuretic to help control their blood pressure.

Possible side effects

- Light headedness/dizziness or even fainting
- Mineral (sodium and/or potassium) imbalance
- Gout (a build up of substances in the joints that cause pain)
- Reduced kidney function

Things to remember

As diuretics make you produce urine, you will need to visit the toilet more frequently. Most people take their water pills in the morning, but occasionally this can interfere with their plans for the day due to the need to visit a toilet. It is however essential that these medicines are taken as prescribed. This may mean that your doctor or nurse advises you to occasionally take your diuretics later in the day to help you manage your weekly routine.

Taking them late in the evening or at night is likely to cause you to have a disturbed night's sleep.

Some people may be prescribed a regular dose of diuretic, while others may have their diuretic routine changed to help manage their symptoms and/or give their kidneys a rest. When taking a diuretic it is important to check how your kidneys are working, and this will require regular blood tests. Your doctor or nurse specialist will let you know when this should be done.

Occasionally diuretics can make the kidneys work too hard. This can cause less urine to be produced and mineral levels to become unbalanced. If you are passing less urine than normal or are having difficulty passing water it is important that you let your doctor know as soon as possible.

Sometimes our bodies require more fluid than others, such as when the weather is hot, if we develop a temperature due to illness, or if we are troubled by a stomach upset which causes vomiting and/or diarrhoea. It is important that you know what to do if this happens, so speak to your facilitator or GP or nurse specialist about this.

Other diuretics – Aldosterone Antagonists

Aldosterone Antagonists are also referred to as Mineralocorticoid Receptor Antagonists (MRAs). The name of these medications frequently end in '-one'. Commonly

prescribed Aldosterone Antagonists/MRAs include:

Spironolactone and Eplerenone.

Aldosterone Antagonists also cause the kidneys to work harder and produce more urine. Aldosterone Antagonists are also used to manage blood pressure. These medications are not prescribed for all people with heart failure but may be offered to those who need more support to control the condition. In these people Aldosterone Antagonists help to control the symptoms they are experiencing, slow down the condition and live longer.

Possible side effects

- Light headedness/dizziness or even fainting.
- Mineral (potassium) imbalance.
- Reduced kidney function.
- Upset stomach (nausea and/or diarrhoea).
- Breast pain or chest (breast) enlargement in men.

Things to remember

Although these medications can make people feel much better, it can take several weeks or even months before the full benefits are felt. As with many other heart medications, blood tests will have to be done regularly to monitor kidney activity and mineral levels. Your doctor or nurse will tell you when this should be done.

Breast enlargement and pain can be distressing, so speak to your doctor or nurse if this is a problem. Don't be embarrassed to seek help as there is often something which can be done.

Potassium chloride

Commonly prescribed potassium chloride supplements include:

Slow-K, Kay-Cee-L and Sando K.

Potassium is an essential mineral used within the body to keep the body's fluid in balance and maintain the heart's steady and stable rhythm. The correct amount of potassium, not too high and not too low, is an important part of keeping the body working well. A side effect of some medications is the higher than normal amount of potassium passed in urine. To keep the body's potassium level within normal limits you may be prescribed potassium chloride as a potassium supplement.

Possible side effects

- Stomach upset (nausea, vomiting and/or diarrhoea).
- Stomach pain.
- Changes in heart rhythm.
- Numbness or tingling in hands and/or feet.

Things to remember

Keeping your body's minerals (sodium and potassium) within normal limits is an important part of keeping your fluids in balance. Your doctor and nurse will monitor the minerals within your blood by taking regular blood tests. This information will help guide them to the best treatment for you.

Managing the minerals in your body is also your responsibility. Check the amount of salt (sodium) you take and sticking to the

healthy recommendations and try to avoid too much potassium by avoiding things that are known to be high in potassium such as salt substitutes. The section on 'Managing Your Salt' covers this.

Antiplatelets

Commonly prescribed antiplatelets include:

Aspirin, Clopidogrel, Prasugrel and Dipyridamole.

Antiplatelet medications are used to reduce the risk of clots forming by making the blood less sticky. This can help prevent serious health problems such as a heart attack or stroke. Antiplatelet treatments are normally only prescribed for those who have cardiovascular disease (the main cause of heart attack and stroke) or are at risk of developing this condition in the future.

Possible side effects

- Bruising.
- Prolonged bleeding.
- Bleeding of the gums.
- Indigestion and stomach pain.

Some of the side effects of antiplatelet medication can be very serious. If you notice any abnormal bleeding it is important to seek medical advice as soon as possible.

Things to remember

Antiplatelet medications such as aspirin can irritate the stomach, causing a feeling of indigestion. In some cases this can lead to bleeding, causing darker bowel movements

These medicines may not apply if you have HFpEF, although you might have been prescribed some. The medicines you are given will depend on your own specific symptoms and type of heart failure. It is important that you ask your doctor, facilitator or pharmacist what your medicines are for and if there are any specific instructions for each.

or vomiting of blood. If you experience any of these symptoms it is important that you speak to your doctor as soon as possible.

Occasionally, in those with more severe heart failure, antiplatelet treatments can make the symptoms of heart failure worse, and this may mean that you will need to have this treatment reviewed.

Anticoagulants

Commonly prescribed anticoagulants include:

Warfarin, Acenocoumarol, Phenindione, Dabigatran etexilate, Apixaban, Rivaroxaban, and Edoxaban.

Anticoagulant medications may be referred to as 'blood thinners'; they also reduce the risk of clots forming, although they work differently to antiplatelet medications. If you have an abnormal heart rhythm or have had some types of heart valve replacement, you may have been prescribed an anticoagulant. If you have a history of a clot forming in either of your legs or lungs you may also have been advised to take an anticoagulant.

Possible side effects

- Bruising.
- Prolonged bleeding.
- Bleeding of the gums.

Some of the side-effects of blood thinning medication can be very serious, particularly if you are taking an anticoagulant such as warfarin. If you notice red or darker stools, pink or darker urine or any abnormal bleeding it is important to seek medical

advice as soon as possible. Extra care should also be taken if you fall or develop a nose bleed.

Things to remember

Anticoagulants such as warfarin require regular blood tests. These tests check how quickly your blood clots. This is to ensure that your blood is both thin enough to prevent clots forming but not so thin that it causes you to bleed too easily. You will be asked to keep a record of these test results and perhaps change the dose of your medication to get the best treatment. Remember to write down any specific advice that the doctor, nurse or pharmacist gives you.

You should normally avoid aspirin if you are on warfarin, unless advised otherwise by your doctor. You should also check with your doctor or pharmacist before taking any other over-the-counter medicines (including herbal remedies). Many of these can also affect the dose of warfarin that you require.

Avoid drinking too much alcohol, cranberry juice or eating cranberries if you are on warfarin, as the combination can make your blood too thin.

Nitrates

Commonly prescribed nitrates include:

Isosorbide mono-nitrate, GTN spray, Nitroglycerin tablets

Nitrates mainly work by relaxing the veins so that they widen and ease the blood flow to the heart. Some people may be given a short-

acting nitrate such as a glyceryl trinitrate spray (GTN) – this is often prescribed to help to relieve chest pain (angina). If you have been given one of these it is important that you follow the advice given to you. There are other long-acting forms of nitrate such as Isosorbide mono-nitrate which may be prescribed in combination with another medication which widens the arteries called hydralazine. These medications work together to help lower the blood pressure and are particularly useful if you are unable to take an ACE inhibitor or ARB or if you are of black African or Caribbean origin.

Possible side effects

- Dizziness.
- Headache.
- Rapid heartbeat.
- Flushing.

Things to remember

As nitrates can lower the blood pressure it is important to take particular care when these medications are first started or when a short-acting dose is taken. Dizziness can cause a loss of balance so remember to take extra care when moving from lying down to standing up.

Some medications used to improve sexual function such as Viagra can interact with nitrates. Always check with your doctor before taking any medications which have not been prescribed for you.

Statins

The name of these medications frequently end in “-statin”

Commonly prescribed Statins include: Simvastatin, Atorvastatin and Pravastatin.

Statins are an important group of drugs which reduce the risk of cardiac events for people with coronary artery disease (which is a common cause of heart failure). These medications work by reducing the amount of cholesterol made in your liver and work quite quickly. They are usually taken at night.

High cholesterol is one of the risk factors which can contribute to the process of atherosclerosis (furring up of the arteries) and for many people lowering their cholesterol will reduce their likelihood of developing further problems.

Possible side effects

These can include tiredness, muscle pains, stomach upsets and headache. Occasionally these medicines can cause inflammation in the muscles or the liver. This can be confirmed with a blood test, and your GP will inform you if the drug needs to be changed or stopped. If you have mild side-effects, do discuss this with your doctor. A reduction in dose or a change to a different statin can often solve the problem.

Things to remember

Report any new muscle pains immediately to your doctor, as these may be the first sign of a serious side-effect. Avoid eating grapefruit or drinking grapefruit juice if you take

Simvastatin. Grapefruit can interact with the medication increasing the risk of serious side-effects.

Dapagliflozin

This is a new type of drug called a Sodium-glucose cotransporter-2 inhibitor (SGLT2i)

Dapagliflozin works by helping the kidneys increase the amount of glucose (sugar) passed out of the body in the urine. This helps control diabetes, but it is also thought to help in heart failure by increasing the amount of fluid removed from the body and by lowering blood pressure, reducing the workload on the heart. Research has shown that dapagliflozin can help you live longer, feel better and reduce the need for admissions to hospital.

You may be offered dapagliflozin in addition to your other heart failure medication if you continue to have symptoms, or your symptoms come back. Dapagliflozin is not suitable for everyone and a Heart Failure Specialist will decide when and if it is right for you.

Possible side effects

- Low blood sugar in those with diabetes
- Diabetic ketoacidosis (DKA): If you experience rapid weight loss, sleepiness, feeling sick, pain in the tummy, sweet smell to your breath, a sweet or metallic taste in the mouth, different smell to your urine or sweat, then this may be a sign of too much of a chemical called ketones in your blood and you should contact the doctor immediately
- Increased risk of infections (urine/genital)

- Back pain
- Pain when passing urine
- Passing urine more often
- Dizziness or lightheaded when standing up

Things to remember

Dapagliflozin is also used to help people who have diabetes control their blood sugar levels. If you are diabetic the doses of any medication you take to treat your diabetes may be changed when you start dapagliflozin. As with many of your other heart failure medicines, taking dapagliflozin means you will need to have blood tests to monitor how your blood sugar levels are and how your kidneys are coping with the treatment. Your doctor or nurse will let you know if and when these tests should be done.

If you are sick with nausea and vomiting, you will need to stop dapagliflozin when you are sick. Please remember to restart the dapagliflozin when you are (usually after 24 to 48 hours after eating and drinking normally). When you restart, just take the dapagliflozin as normal.

Dapagliflozin is taken as a tablet once a day and you can take it with or without food. Try to take it at the same time each day.

There isn't enough space in this manual to list all of the medications which are available, and new ones are being developed all the time. If you have been prescribed medications which are not listed on these pages, ask your doctor, facilitator or pharmacist what they are for and any specific actions you should take.

What Happens Next

Now you have completed the Heart Failure manual, you may be thinking “I’m glad that’s over” and looking forward to getting back to your normal routine. However, your journey is far from over – completing the course is just the first step! We hope that you will take the learning from the last few months and keep applying it. If you keep your exercise levels up, keep managing your fluids, get control over any psychological stresses or worries and keep taking your medications, you should be able to keep yourself healthy and fit and able to engage in life and do all the things that you enjoy doing. Of course, you may have some limitations due to having heart failure that you have to adjust to, but the trick is to make the most of what you have. The key things to remember are:

1. Keep Exercising

The deal with exercise and fitness is of course “use it or lose it”. It is very important to treat maintaining your fitness as a key goal for the rest of your life – not just as a 12 week programme. Keeping active helps to maximise your physical abilities, energy levels and mood. So, please keep going at whatever level you have managed to reach with your walking programme or your chair based exercises – and feel free to step it up a level if you and your facilitator think that is appropriate.

2. Keep Tracking

Your Progress Tracker has a spare (double-sided) tracker at the end, so you can copy these and keep tracking your progress and monitoring your symptoms if you think that is appropriate. You may also find this helpful if you have a setback and have to go back a few steps to build up your fitness again (see the next section).

What to do if I have a setback?

Life doesn’t always run smoothly, despite our best efforts. If you have to go into hospital or have an injury or other illness, you may need to take some time away from exercising and this can lead to you losing your fitness. Effectively, you can end up back at square one (or even lower). In this case, the only thing to do is to start again with the exercise programme. Start with whatever level you can easily manage from the videos or the walking programme and then gradually build it up over 2-3 months to get back to a higher level of fitness. The good news is that you have done it before – and you still have your manual to guide you. You may also be able to ask your heart failure care team or GP if they can get you referred into another rehabilitation programme, so you will be able to have someone to discuss your progress with.



Feedback Form

Your feedback is considered invaluable and will help users in the future. We regularly use this feedback for audit and evaluation purposes. These findings may be published. No one will be able to identify you from them. The questionnaire will only take a few minutes to complete.

Please circle the most appropriate answer for each question

Did your family & friends read your manual?	Yes	No
How many times did your REACH-HF facilitator either visit you at home or speak to you on the telephone Home visit..... Telephone..... Other (please state)		

Please circle the most appropriate rating for each statement	 Strongly Disagree	 Disagree	 Unsure	 Agree	 Strongly Agree
This programme has helped me to...					
understand more about my condition	1	2	3	4	5
understand more about my medication	1	2	3	4	5
manage breathlessness	1	2	3	4	5
manage stress or anxiety	1	2	3	4	5
manage low spirits	1	2	3	4	5
increase my daily activity levels	1	2	3	4	5
practise relaxation	1	2	3	4	5
stop smoking (if applicable)	1	2	3	4	5
eat a more healthy diet	1	2	3	4	5
recognise if my fluids are building up	1	2	3	4	5
take appropriate action if my fluids are building up	1	2	3	4	5
get support from others	1	2	3	4	5
adapt to living with heart failure	1	2	3	4	5
feel better	1	2	3	4	5

Feedback Form

Please circle the most appropriate rating for each statement	 Strongly Disagree	 Disagree	 Unsure	 Agree	 Strongly Agree
The Manual and Progress Tracker are very clear about when to call a doctor	1	2	3	4	5
Overall, the information in the Manual was easy to understand	1	2	3	4	5
The resources were easy to use	1	2	3	4	5
Using the manual and the Progress Tracker has motivated me to change my lifestyle	1	2	3	4	5
Overall the REACH-HF Manual was useful to me	1	2	3	4	5
Overall the relaxation audio was useful to me	1	2	3	4	5
Overall the chair based exercise videos were useful to me	1	2	3	4	5
Overall the Progress Tracker was useful to me	1	2	3	4	5
Having contact with my facilitator helped me	1	2	3	4	5
I would have liked more contact with my facilitator	1	2	3	4	5
Overall, the REACH-HF programme has helped me to manage my condition	1	2	3	4	5

What do you think has changed in your day to day life since completing the programme?

Is there anything that you would like to see added to the REACH-HF Manual or other resources?

What do you think has been most useful?

Any other comments about the REACH-HF materials?

Thank you. Please return this sheet to:

Heart Manual Department, NHS Lothian, Astley Ainslie Hospital, Edinburgh EH9 2HL.